ONGO WEIGHT LOSS · BOOK SIX

Ongo Weight Loss

Write It Right.

Writing for a health brand · from first draft to independent YMYL writer

For every Ongo Weight Loss teammate who writes a page, a post, a button or an email: clear, honest, safe and genuinely helpful.

PREPARED BY

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Edition: October 2026 · Book Six

ONGO WEIGHT LOSS · ONBOARDING SERIES

Writing for a Health Brand

How to write YMYL content for Ongo Weight Loss: clear, compliant and helpful
Book Six of the Ongo Weight Loss onboarding series

Prepared by	Ashish Osten, Team Lead, Ongo Weight Loss
Edition	October 2026 (first edition)
Written for	New content writers (no prior web or marketing writing needed), with SEO, reviewers, designers and team leads
Read first	Book One: The Foundation (Books Three and Four explain the content map and on-page SEO)
Inside	44 chapters · 440 MCQs · 38 content types · 20+ writing models · voice guide · competitor review · 70-point audit sheet · annotated sample article · practice assignments · glossary · dated sources

Important

This book is internal orientation material. It is not medical, legal or regulatory advice for you or for any patient. Real medical questions go to Ongo Weight Loss's licensed providers; legal and regulatory questions go to compliance and counsel.

Search guidance, drug approvals, regulations, prices and tools change often. Every claim carries a dated source and an evidence label so you can recheck it. When this book and a newer official source disagree, the official source wins.

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ABOUT THIS BOOK

What's inside

> **Before you start:** read **Book One: The Foundation** (what Ongo Weight Loss is and what the rules allow). Book Three (site architecture and the content map) tells you which pages exist and how they link; Book Four (on-page SEO) explains titles, headings, alt text and events; Book Five (social media) shows how your articles become posts. Book Six teaches you to write every page and post: clearly, honestly and safely.

How every chapter is built

Part of chapter	What it gives you
Trivia hook	One surprising, sourced fact that shows why the topic matters
Plain explanation	The concept in very easy English, with many "before → after" rewrites
Desi analogy (Hinglish)	Rotates between restaurant, IT office, cricket, wedding and mohalla
Ongo Weight Loss example	Real findings from our homepage and blog review (30 October 2026)
Tables and templates	Rules, formulas, skeletons and checklists at a glance
Evidence labels	Official, Google spokesperson, industry, or expert/internal
Do's and Don'ts	Quick rules
Pros and Cons	Honest trade-offs
Where freshers go wrong	Common mistake, why it happens, how to avoid it, how to fix it
Thumb rules + Notes to remember	One-line rules to keep
Glossary	Every new term in the chapter
10 MCQs	Answer key at the foot of the chapter, flipped upside down
Sources	Every claim linked to where it came from, with dates

The four levels

Level	Chapters	What you'll learn
Level 1 · Foundation	1–8	What a writer does here, YMYL, E-E-A-T and helpful content, compliance and claim types, our voice and tone guide, our readers and reading level, people-first language, medical basics
Level 2 · Preparing to Write	9–17	Purpose first, briefs and the 38 content types, search intent and the funnel, keyword and question research, the evidence ladder, how competitors write, outlines and headings, claim logs, using AI safely
Level 3 · Writing Techniques	18–32	Plain language, rhythm, inverted pyramid and BLUF, hypophora, breaking the fourth wall, clinical neutrality, persuasion and explanation models, information gain, storytelling, headlines, intros and conclusions, scannability, AI answers and FAQs, CTAs and microcopy
Level 4 · Page Types, Review and Growth	33–44	Money pages, blog types, trust pages, short formats, self-editing, fact-checking and dates, medical and legal review, SEO handoff and internal links, accessibility, tools, measuring and refreshing, ZETA for writers

Your questions: where to find the answers

You asked about	Chapter(s)	You asked about	Chapter(s)
How to write for YMYL health	2, 3, 4, 23	Inverted pyramid	20
How to prepare before writing	9–17	Hypophora	21
How a writer should write	18–32	Breaking the fourth wall	22
What to check before submitting	37, 38, 44, Appendix H	Other writing models (AIDA, PAS, BAB, FAB, PEEL, SCQA, Feynman…)	24, 25, Appendix A
All content types (20+ → 38)	10, 33–36, Appendix B	Brand voice and tone guide	5, Appendix K

You asked about	Chapter(s)	You asked about	Chapter(s)
How competitors write; best pages and why	14	Tools: free, freemium, paid	42
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What writers deliver in Google Docs	16, 40	Image briefs for designers	30

What's new compared with Books One to Five

Topic	Book One	Book Three	Book Four	Book Five	Book Six
Compliance rules	✅ Defined	Applied to pages	Applied to tags	Applied to posts	✅ Applied to every sentence (claim types)
Content types	—	Page list	Page specs	Social formats	✅ 38 types with skeletons
Voice and tone	Overview	—	—	Social tone	✅ Full guide
Writing techniques	—	—	Headings, meta, alt	Hooks, captions	✅ 20+ models and techniques
Review workflow	—	—	QA	Social approvals	✅ Claim logs, VERIFY, approval log, 70-point audit sheet

BEFORE YOU BEGIN

Front Matter

A Letter to the Reader

Welcome to Book Six.

If you know English but have never written for a website, a brand or a health company, this book is for you. By the last chapter you should be able to take a brief, research a health topic properly, write a clear and honest page, check it yourself, get it through medical and legal review, and improve it after it goes live, on your own.

Writing for Ongo Weight Loss is different from writing a school essay or a lifestyle blog. Our readers are adults who may be worried, embarrassed or tired of failed diets. They are making decisions about their health and their money. Google holds pages like ours to its highest standard. Regulators read our claims. So this book teaches two things at once: **how to write well**, and **how to write safely.**

Here is what we did before writing:

- We read our team's earlier writing guidelines and kept what works: purpose statements, Why → What → How, clinical neutrality, the inverted pyramid, hypophora, information gain and the checklist tests. We then updated anything that didn't fit Ongo Weight Loss or wasn't supported by current evidence.
- We reviewed the Ongo Weight Loss homepage and blog on **30 October 2026**, so the examples are real.
- We studied how leading weight-care brands write (Ro, Hims, Found, WeightWatchers, Noom, Mochi, Fridays, Calibrate, IVIM, Henry Meds) and a medical benchmark (Mayo Clinic): what works, what doesn't, and why.
- We read Google's own guidance on helpful content and its quality rater guidelines, and official plain-language guidance from NIH, CDC and the U.S. government.

Every important claim has a source, a date and a label. When something might change (prices, tools, rules), it says so.

Let's write pages that a worried reader, a doctor, a regulator and Google would all be glad exist.

How to Read This Book

The four levels

Level	Who it's for	What you can do after
Level 1 – Foundation	Every writer on day one	Explain YMYL, E-E-A-T, our rules and our voice
Level 2 – Preparing to Write	Writers, with SEO	Turn a brief into a researched, approved outline
Level 3 – Writing Techniques	Writers	Draft clear, honest, engaging content with the right model
Level 4 – Page Types, Review and Growth	Writers, reviewers, team leads	Write every page type, pass review, measure and refresh

The boxes you'll see

> 💡 **Trivia** – a surprising fact that shows why the topic matters.
>
> 🍛 **Desi analogy** – the idea explained in Hinglish with an everyday example.
>
> 🟧 **Ongo Weight Loss example** – what we found on our site, and what to do.
>
> ⚠️ **Where freshers go wrong** – the common mistake and how to avoid or fix it.
>
> 👍 **Thumb rule** – one line to remember forever.
>
> 📝 **Note to remember** – a key fact to keep.

If you've never written before

Start with Chapters 1, 9, 18 and 20. Then read in order. Keep Appendix A (writing models), Appendix C (word swaps), Appendix H (checklists) and Appendix L (audit sheet) open while you write. Study the annotated sample article in Appendix M, and do the practice assignments in Appendix N at the end of each level.

Working with your team

You'll work with the **team lead** (briefs and approvals), the **SEO team** (keywords, links, meta), **medical and legal reviewers** (every health, price and testimonial claim), **designers** and **developers** (layout and build), and the **social team** (repurposing). Ask early; ask specifically.

Using the MCQs for onboarding

- Each chapter ends with 10 multiple-choice questions.
- Team leads can use them after each level; 8/10 or better means ready to move on.

The Evidence Label Legend

Source labels: where the claim comes from

Label	Meaning
🟢 **Official**	Google's own documentation, regulators and agencies (FDA, CDC, NIH, FTC), peer-reviewed trials, official research (KFF, Pew), established medical references (Mayo Clinic)
🟡 **Google spokesperson**	A public statement by a Google employee
🟠 **Industry**	Competitor pages we reviewed, law-firm summaries, journals' secondary analyses, trade press
🔵 **Expert / Ongo Weight Loss internal**	Expert judgment, our site review, our writing guidelines and leadership decisions

How much to trust each label

Label	Trust	Before you rely on it
🟢	Highest	Check it's still current (dates are given)
🟡	High	Remember it's one person's statement
🟠	Medium	Prefer the original source when possible
🔵	Varies	Confirm with the team lead or reviewer

Changeable facts

Tool plans, competitor pages, prices and regulations change. Treat them as **changeable** and re-check before relying on them.

Risk level: what can go wrong

Most writing risk comes from five places: **"FDA-approved", "generic" or "same as" claims about compounded medications**, **brand drug names in commercial copy**, **outcome promises and borrowed trial results**, **fake or unconsented stories and reviews**, and **publishing without medical and legal review**. Chapter 44 (ZETA) lists them all.

How the Answer Keys Work

Every chapter's answer key sits at the foot of the chapter, printed **upside down**. Try the quiz first, then turn the page around to check. All 44 keys are also collected in the back matter.

01

LEVEL 1

Foundation

What a writer does here, YMYL, E-E-A-T, our compliance rules and claim types, our voice, our readers, people-first language and medical basics.

1 · What a Writer Does at Ongo Weight Loss

2 · YMYL: Why Google Judges Health Pages Harder

3 · E-E-A-T and Helpful Content

4 · Compliance Rules and the Four Types of Claims

5 · The Ongo Weight Loss Voice and Tone Guide

6 · Who We Write For: Readers, Personas and Reading Level

7 · People-First, Weight-Respectful Language

8 · Medical Basics Every Writer Must Know

LEVEL 1 · CHAPTER 1

Chapter 1: What a Writer Does at Ongo Weight Loss

You are not "making content". You are explaining health decisions to real people who are worried, hopeful and short on time. This chapter shows what the job really is.

> 💡 **Trivia**
>
> The CDC reports that **41.9% of U.S. adults** had obesity in 2017–March 2020, and **9.2%** had severe obesity. [1] ● Official (CDC)
>
> That is roughly two in every five adults. Many of them will search for answers before they ever talk to a doctor. Your page might be the first thing they read.

1.1 The writer's job in one sentence

> **Help a real person understand a health decision clearly and honestly, then show them a safe next step.**

Part of the sentence	What it means for you
A real person	Not Google, not a keyword: an adult who may be anxious, embarrassed or tired of failed diets
Understand	Plain words, short sentences, the answer first
A health decision	Treatment, cost, side effects, eligibility: things that affect bodies and money
Clearly and honestly	No hype, no hidden selling, no promises
A safe next step	Read more, talk to a provider, check eligibility; never "buy now or miss out"

1.2 What you will write

You will write many kinds of content. Every one of them has the same job, but a different shape.

Group	Examples	Chapter
Money pages	Service, medication, cost, how it works, landing pages	33
Blog content	Guides, comparisons, myth vs fact, checklists, recipes, glossaries	34
Trust pages	Doctor and author profiles, editorial policy, testimonials, policies	35
Short copy	Meta titles, alt text, buttons, form errors, emails, captions	32, 36

The full list of 35+ content types is in Chapter 10 and Appendix B.

> 🪔 **Desi analogy: dawai ki dukaan ka pharmacist**
>
> Achha pharmacist dawai bechne se pehle samjhata hai: "Khaane ke baad lena, chakkar aaye toh doctor ko batana." Woh yeh nahi kehta "yeh lo, sab theek ho jaayega." **Writer bhi wahi pharmacist hai:** samjhao, sach batao, aur sahi agla kadam dikhao.

1.3 Why health writing is different from other writing

Normal marketing writing	Health writing at Ongo Weight Loss
"Best", "#1", "amazing results"	Specific, sourced, calm statements
Bold promises sell	Promises can mislead and break the law (Chapter 4)
Opinions are fine	Facts must match medical evidence
Anyone can review it	Medical and legal reviewers must check claims (Chapter 39)
A mistake is embarrassing	A mistake can harm someone's health or money

Google calls topics like ours **"Your Money or Your Life" (YMYL)** and holds them to the highest standard (Chapter 2).

1.4 Who you work with

Person	How they help you
Team lead	Assigns briefs, approves final pages
On-page SEO	Gives keywords, headings, internal links (Books Three and Four)
Medical reviewer	Checks every health claim
Legal reviewer	Checks compliance wording, prices, testimonials
UI/UX designer and graphic designer	Turn your words into layouts and visuals
Front-end developer	Puts your copy on the page
Social team	Turns your articles into posts and videos (Book Five)

1.5 A day in the life

Time	Task
Morning	Read the brief; write the purpose statement; research from approved sources
Midday	Outline and draft
Afternoon	Self-edit with the checklists; fill the claim log; submit for review
Daily	Fix reviewer comments; update one older page if assigned

> 🟧 **Ongo Weight Loss example**
>
> Our homepage already does some things well. "Your Weight Loss Shouldn't Need Insurance" is clear and speaks to a real worry. But it also has lines like "Real Transformations, Backed by Science" that promise more than the page proves. By the end of this book, you will be able to spot both kinds of lines and fix the second kind. 🔵

1.6 Do's and Don'ts

✅ Do	❌ Don't
Write for one anxious, real reader	Write for "the algorithm"
Answer the question first	Make readers dig for the answer
Ask the reviewer when unsure	Guess a medical fact
Keep every claim provable	Copy confident lines from competitors

1.7 Pros and cons of health writing as a career

✅ Pros	❌ Cons
Your work genuinely helps people	Slower process (reviews)
Skills transfer to any regulated industry	Less creative freedom with claims
Clear rules make decisions easier	Constant learning as science and rules change

1.8 Where freshers go wrong in Chapter 1 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing like an ad ("Lose weight fast!")	Copying marketing they've seen	The job sentence (1.1)	Rewriting with facts and a safe next step
Writing for word count	Thinking longer ranks better	Purpose first (Chapter 9)	Cutting filler
Skipping the reviewer	Wanting speed	Workflow (Chapter 39)	Submitting before publishing

1.9 Thumb rules

👍 Explain, don't sell. The selling happens when trust is earned.

👍 Every sentence should help the reader or be cut.

👍 If you can't prove it, don't write it.

1.10 Notes to remember

📝 Health content is YMYL; mistakes can affect health and money.

📝 Every health claim goes to the medical reviewer.

Chapter 1 Glossary

Term	Simple meaning
Content writer	The person who writes the words on pages, posts and emails
Copy	The words on a page, especially marketing words
Money page	A page that leads directly to a service or sign-up
Brief	Instructions for a piece of content
Reviewer	An expert who checks content before it goes live

Chapter 1 Quiz

1. What is the writer's job at Ongo Weight Loss?

- A) Help a real person understand a health decision clearly and honestly, then show a safe next step
- B) Copy competitors
- C) Write the longest pages

- D) Use as many keywords as possible

2. About what share of U.S. adults had obesity in 2017–March 2020 (CDC)?

- A) 90%
- B) 41.9%
- C) 19%
- D) 4.9%

3. What does YMYL stand for?

- A) Yearly Medical Yield List
- B) You Must Your Link
- C) Your Money or Your Life
- D) Your Marketing, Your Leads

4. Who checks every health claim?

- A) The medical reviewer
- B) The graphic designer
- C) The social media junior
- D) Nobody

5. Which line is most appropriate for a health page?

- A) "The #1 miracle shot"
- B) "Guaranteed results"
- C) "Lose 30 lbs fast!"
- D) "Results take time and vary; a provider decides if treatment is right for you."

6. What is a money page?

- A) A page about banking
- B) A page that leads directly to a service or sign-up
- C) A recipe page only
- D) A page with no links

7. Which chapter lists all content types?

- A) Chapter 1
- B) Chapter 10
- C) Chapter 44

- D) Chapter 99

8. What should you do if unsure about a medical fact?

- A) Ask the reviewer
- B) Guess
- C) Copy a competitor
- D) Leave it vague and publish

9. Why is health writing different?

- A) It doesn't need editing
- B) It never needs sources
- C) It uses bigger words
- D) Mistakes can harm someone's health or money

10. In the pharmacist analogy, a good writer...

- A) Explains, tells the truth and shows the right next step
- B) Promises a cure
- C) Says nothing
- D) Sells the medicine quickly

Sources

1. CDC, Adult Obesity Facts
2. Ongo Weight Loss, homepage copy review, 30 October 2026 (internal)

Answer key, Chapter 1: 1-A · 2-B · 3-C · 4-A · 5-D · 6-B · 7-B · 8-A · 9-D · 10-A

LEVEL 1 · CHAPTER 2

Chapter 2: YMYL: Why Google Judges Health Pages Harder

Google uses thousands of human raters to judge search results. Their rulebook treats health pages like ours as high-risk. Here is what that rulebook says, in plain words.

> 💡 **Trivia**
>
> Google's Search Quality Rater Guidelines (September 11, 2025 version) say: "If a page on YMYL topics is highly inexpert, it should be considered Untrustworthy and rated Lowest." [1] ● Official (Google)
>
> "Lowest" is the worst rating a page can get.

2.1 What YMYL means

YMYL = "Your Money or Your Life". Google uses it for topics that could seriously affect a person's:

Area	Example at Ongo Weight Loss
Health	Side effects, dosing, who should not take GLP-1 medications
Money	Prices, what's included, refunds, insurance (we are self-pay)
Safety	When to call a doctor or 911
Society's welfare	Accurate public health information

Almost every page we write is YMYL. [1] ●

2.2 Who are quality raters?

Fact	Detail
Who	People Google hires to rate sample search results
What they use	The Search Quality Rater Guidelines (a public PDF)
What they do NOT do	They don't directly change rankings of a single page
Why it matters	Their ratings help Google test and improve its systems, so the guidelines show what Google wants

2.3 The two ratings you must know

Rating	Question it answers	Scale
Page Quality (PQ)	Is this page trustworthy, expert and well made for its purpose?	Lowest → Low → Medium → High → Highest
Needs Met (NM)	Did this page answer what the searcher actually wanted?	Fails to Meet → Slightly → Moderately → Highly → Fully Meets

A page can be **high quality but fail Needs Met** if it answers the wrong question. Example: someone searches "semaglutide nausea how long" and lands on a page about how semaglutide was discovered.

> 🍛 **Desi analogy: shaadi ka khaana**
>
> Khaana kitna bhi swaadisht ho (Page Quality), agar mehmaan ne paani maanga aur aapne biryani de di, toh "Needs Met" fail. **Sahi cheez, sahi samay pe, achhi quality mein:** dono chahiye.

2.4 What makes a YMYL page "Lowest"

Red flag	Example
No information about who made it	No author, no reviewer, no company info

Red flag	Example
Inexpert or inaccurate health content	Wrong dose, invented study
Harmful advice	"Double your dose if you're not losing weight"
Deceptive purpose	Looks like education, but only pushes a sale
Misleading claims	"Same as Ozempic", "guaranteed results"

2.5 What makes a YMYL page "High"

Green flag	Example
Clear author and medical reviewer with real credentials	"Written by [name]; medically reviewed by [name], MD; updated [date]"
Accurate, sourced, current facts	Trial named, FDA label linked
Answers the question directly and fully	Answer in the first two sentences
Honest about limits	"Results vary", "may", "not for everyone"
Clear company information	About, contact, policies, LegitScript certification

> 🟧 **Ongo Weight Loss example**
>
> Our blog listing (checked 30 October 2026) shows a doctor byline on each post, but no named medical reviewer and no dates. For YMYL pages, raters look for "who made this, who checked it, and when". Adding reviewer names and honest "Last medically reviewed" dates is one of the easiest trust wins. 🔵

2.6 Do's and Don'ts

✅ Do	❌ Don't
Show author, reviewer and dates	Publish anonymous health content
Answer the searcher's real question	Answer a different, easier question

✅ Do	❌ Don't
State limits and risks	List only benefits
Link primary sources	Rely on "studies show" with no link

2.7 Pros and cons of the YMYL standard

✅ Pros	❌ Cons
Rewards honest, careful brands	More work per page
Pushes out low-quality competitors over time	Slower to publish
Builds long-term trust	Needs experts' time

2.8 Where freshers go wrong in Chapter 2 topics

Mistake	Why it happens	Avoid it by	Fix it by
Treating a blog like a lifestyle post	Not knowing YMYL	Section 2.1	Adding sources and review
Answering a broader question than asked	Wanting to cover everything	Needs Met thinking	Refocusing the intro
No author/reviewer box	Thinking it's design's job	Pre-submit checklist	Requesting the box

2.9 Thumb rules

👍 Health and money pages are judged hardest; write like it.

👍 Quality plus the right answer: you need both.

👍 Who made it, who checked it, when: always visible.

2.10 Notes to remember

📝 Rater guidelines version: September 11, 2025.

📝 Highly inexpert YMYL pages are rated Lowest.

Chapter 2 Glossary

Term	Simple meaning
YMYL	Topics that can affect health, money, safety or society
Quality rater	A person hired by Google to rate search results
Page Quality (PQ)	How trustworthy and well made a page is
Needs Met (NM)	How well a page answers the searcher's need
Search Quality Rater Guidelines	Google's public rulebook for raters

Chapter 2 Quiz

1. What does YMYL cover?

- A) Topics that can affect health, money, safety or society
- B) Only recipes
- C) Only celebrity gossip
- D) Only finance news

2. What happens to highly inexpert YMYL pages in the rater guidelines?

- A) Rated Highest
- B) Ignored
- C) Rated Lowest
- D) Promoted

3. What does Needs Met measure?

- A) Font size
- B) Number of images
- C) Whether the page answers what the searcher wanted
- D) Page speed

4. Can a high-quality page fail Needs Met?

- A) No, never
- B) Yes, if it answers the wrong question
- C) Only on weekends
- D) Only on mobile

5. Which is a red flag for YMYL?

- A) Named reviewer with credentials
- B) No author or reviewer information
- C) Honest limits
- D) Linked FDA label

6. What do quality raters do?

- A) Sell ads
- B) Write content
- C) Rate sample results to help Google improve its systems
- D) Directly rank each page

7. Which line is a green flag?

- A) "Medically reviewed by [name], MD; updated [date]"
- B) "Same as Ozempic"
- C) "Studies show" with no link
- D) "Guaranteed results"

8. Which rater guidelines version is cited here?

- A) None
- B) 2010
- C) September 11, 2025
- D) 1999

9. What was missing from Ongo Weight Loss blog listings?

- A) Titles
- B) URLs
- C) Named medical reviewer and dates
- D) Words

10. In the shaadi-ka-khaana analogy, giving biryani when someone asked for water fails…

- A) Needs Met
- B) Keyword density
- C) Page speed
- D) Grammar

Sources

1. Google, Search Quality Rater Guidelines (September 11, 2025) 🟢
2. Ongo Weight Loss, blog listing review, 30 October 2026 🔵

Answer key, Chapter 2: 1-A · 2-C · 3-C · 4-B · 5-B · 6-C · 7-A · 8-C · 9-C · 10-A

LEVEL 1 · CHAPTER 3

Chapter 3: E-E-A-T and Helpful Content

Google asks one big question about every page: was this made to help people, or to rank? E-E-A-T is how a page shows the answer.

> 💡 **Trivia**
>
> Google's guide to helpful content says of Experience, Expertise, Authoritativeness and Trust: "Of these aspects, trust is most important." It also says E-E-A-T "itself isn't a specific ranking factor." [1] ● Official (Google)
>
> E-E-A-T is not a score you can buy. It is what a reader (and Google's systems) can see on the page.

3.1 The four letters

Letter	Meaning	How a writer shows it
E · Experience	First-hand familiarity	Real details: what the first weeks of treatment can feel like (from providers and consented patients), practical tips, our own process
E · Expertise	Knowledge of the subject	Accurate facts, correct terms, nuance, named trials, a qualified reviewer
A · Authoritativeness	Being a recognised source	Citing primary sources; staying within our topic; consistent quality
T · Trust	Honest, accurate, safe	No exaggeration, clear disclaimers, visible company info, honest prices

Trust is the multiplier. If trust fails, the other three don't matter. [1] ●

> 🪔 **Desi analogy: mohalle ka doctor**
>
> Mohalle mein sab us doctor ke paas jaate hain jo **sach batata hai**, bhale hi woh bada naam na ho. Degree (expertise) aur naam (authority) achhe hain, lekin agar woh ek baar jhooth bole, toh log wapas nahi aate. **Trust sab se upar.**

3.2 Google's "Who, How, Why"

Question	What Google means	What you do
Who created it?	Authorship should be clear	Author and reviewer bylines with bios
How was it created?	Explain process, including AI use when readers would expect it	Editorial and medical review policy pages; honest notes
Why was it created?	"Perhaps the most important question": to help people, not mainly to attract search visits	Purpose statement (Chapter 9)

[1] 🟢

3.3 People-first vs search-engine-first

People-first (✅)	Search-engine-first warning signs (❌)
Written for an existing audience who would find it useful	Lots of content on many topics "hoping some performs"
Shows first-hand expertise and depth	Summarising others without adding value
Reader leaves satisfied	Reader has to search again
Updated when facts change	Changing dates to seem fresh without real changes
As long as it needs to be	Writing to a word count because "Google prefers it"

All of these warning signs come from Google's own guide. [1] 🟢

3.4 The three reader reactions to aim for

After reading your page, a real person should be able to say at least one of:

1. "That answered my question."
2. "I understand this better now."
3. "I know what to do next."

If none of these is true, the page failed, no matter how well optimised it is. 🔵

3.5 Spam lines you must never cross

Policy	What it means for writers
Scaled content abuse	"Many pages are generated for the primary purpose of manipulating search rankings and not helping users", **however they are created** (by AI or humans) [2] 🟢
Fake authors	Invented profiles are deceptive [1] 🟢
AI mainly to manipulate rankings	A spam-policy violation [1] 🟢

> 🟧 **Ongo Weight Loss example**
>
> Competitor Found publishes an "Our editorial standards" note: articles are written by health journalists, edited for people-first language, fact-checked against peer-reviewed literature and reviewed by obesity medicine doctors. That is "How" made visible. Ongo Weight Loss should have an editorial policy and a medical review policy page and link to them from every article (Chapter 35). 🔵

3.6 Do's and Don'ts

✅ Do	❌ Don't
Make authorship and review visible	Hide who wrote it
Explain your process honestly	Pretend AI content is hand-written by a doctor

✅ Do	❌ Don't
Write for a real audience	Write pages "just to rank"
Update facts, then update dates	Change dates without changes

3.7 Pros and cons of E-E-A-T work

✅ Pros	❌ Cons
Builds trust with readers and Google	Needs real experts' time
Protects against algorithm swings	Can't be faked or rushed
Supports conversions	More page elements to maintain

3.8 Where freshers go wrong in Chapter 3 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing "doctor approved" with no name	Sounds trustworthy	Named reviewer rule	Adding the real reviewer
Updating the date after fixing a typo	Wanting freshness	Honest-date rule	Reverting the date
Rewriting competitors	Easy	Information gain (Chapter 26)	Adding original value

3.9 Thumb rules

👍 Trust first; expertise shown, not declared.

👍 Who, How, Why: visible on every page.

👍 Write for people; rankings follow.

3.10 Notes to remember

📝 E-E-A-T isn't a single ranking factor, but trust is "most important".

📝 Scaled content abuse applies however the pages are made.

Chapter 3 Glossary

Term	Simple meaning
E-E-A-T	Experience, Expertise, Authoritativeness, Trust
People-first content	Content made mainly to help readers
Search-engine-first content	Content made mainly to attract search traffic
Scaled content abuse	Mass-producing pages to manipulate rankings
Byline	The line naming who wrote or reviewed a page

Chapter 3 Quiz

1. Which E-E-A-T element does Google call most important?

- A) None
- B) Experience
- C) Authoritativeness
- D) Trust

2. Is E-E-A-T itself a specific ranking factor?

- A) No
- B) Only for news
- C) Only on mobile
- D) Yes, a single score

3. Which Google question is "perhaps the most important"?

- A) Why was it created?
- B) Who designed the logo?
- C) What font is used?
- D) How long is it?

4. Which is a search-engine-first warning sign?

- A) Changing dates without substantial changes
- B) Answering the question first
- C) Citing the FDA label
- D) Naming the medical reviewer

5. Does scaled content abuse depend on how pages are made?

- A) Only videos
- B) Only AI pages
- C) No; it applies however they're created
- D) Only human pages

6. How do you show expertise?

- A) Accurate facts, correct terms, named sources and a qualified reviewer
- B) Adding more keywords
- C) Writing "expert approved"
- D) Using long words

7. What should a reader be able to say after reading?

- A) "That answered my question" (or understand better / know what's next)
- B) "I need to search again"
- C) "I'm confused"
- D) "That was long"

8. What did Found publish that shows "How"?

- A) A video only
- B) A logo
- C) An editorial standards note
- D) A price list

9. Are fake author profiles acceptable?

- A) If the photo is nice
- B) For blogs only
- C) Yes
- D) No; they're deceptive

10. In the mohalle-ka-doctor analogy, people return because the doctor…

- A) Is cheapest
- B) Tells the truth
- C) Has the biggest sign
- D) Talks the most

Sources

1. Google Search Central, Creating helpful, reliable, people-first content
2. Google Search Central, Spam policies for Google web search
3. Found, Weight loss news blog (editorial standards), viewed 30 October 2026

Answer key, Chapter 3: 1-D · 2-A · 3-A · 4-A · 5-C · 6-A · 7-A · 8-C · 9-D · 10-B

LEVEL 1 · CHAPTER 4

Chapter 4: Compliance Rules and the Four Types of Claims

Every sentence you write is a claim of some kind. This chapter gives you the Ongo Weight Loss rules, and a simple way to sort every sentence into "safe", "needs a qualifier" or "never".

> 💡 **Trivia**
>
> FDA letters to telehealth companies in September 2025 flagged phrases such as "same ingredient as Wegovy & Ozempic", "generic Ozempic/Wegovy/Mounjaro" and "clinically proven ingredients". [1] ● Industry (Holland & Knight)
>
> Some competitors still use lines like "Same ingredient as" on product cards. We don't.

4.1 The standing rules (from Book One)

#	Rule	Example
1	Our brand is **"Ongo Weight Loss"**, never "Ongo" alone in copy	✅ "Ongo Weight Loss providers..."
2	We offer **compounded semaglutide and compounded tirzepatide** only	✅ "compounded semaglutide" ❌ "semaglutide injections like Wegovy"
3	Compounded medications are **not FDA-approved**	✅ "Compounded medications are not FDA-approved."
4	**No brand drug names** (Ozempic, Wegovy, Mounjaro, Zepbound, etc.) in commercial copy, titles, CTAs or meta during the launch phase; education only after review, with "Ongo Weight Loss does not offer [brand]"	❌ "Get Wegovy-level results"
5	**Never name the partner pharmacy**	✅ "a licensed U.S. compounding pharmacy"

#	Rule	Example
6	**No outcome promises**: no pounds, speed or guarantees	❌ "Lose 20 lbs in a month"
7	**A licensed provider decides** if treatment is appropriate	✅ "A provider decides whether treatment is right for you."
8	**Prices must match the website today**, with what's included	✅ "[price exactly as on the website]"
9	**Self-pay**: no insurance claims	✅ "No insurance needed" ❌ "Covered by insurance"
10	**No patient information** anywhere without signed consent	—
11	**Testimonials**: real, consented, typical, disclosed (FTC)	✅ "Shared with permission. Individual experiences vary."
12	**Trial results** belong to brand products in those trials; name the trial and say compounded versions weren't studied	✅ "In the STEP 1 trial of brand semaglutide..."

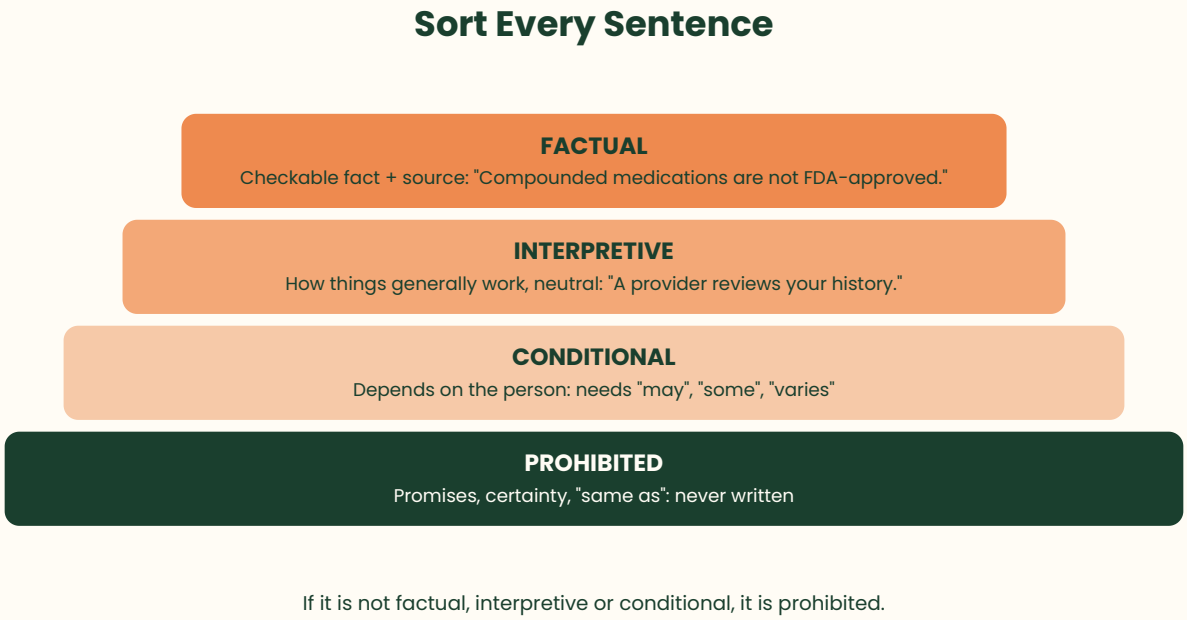
4.2 The four claim types (sort every sentence)

Type	What it is	Language	✅ OK	❌ Not OK
FACTUAL	Can be checked against a source	Plain statement + source	"Compounded medications are not FDA-approved."	"Compounded semaglutide is FDA-approved."
INTERPRETIVE	Explains how things generally work	Neutral, reflects variation	"A provider reviews your health history before deciding on treatment."	"Doctors approve almost everyone."
CONDITIONAL	Depends on the person or situation	Must have a qualifier	"Some people may feel nausea in the first weeks."	"You'll feel nauseous for two weeks."

Type	What it is	Language	✅ OK	❌ Not OK
PROHIBITED	Promises outcomes, certainty or things we can't prove	Never written	—	"Guaranteed weight loss", "No side effects", "Same as Ozempic"

Rule: if a sentence is not clearly FACTUAL, INTERPRETIVE or CONDITIONAL, it is PROHIBITED. 🔵

> 🪔 **Desi analogy: cricket umpire**
>
> Umpire ke paas teen hi faisle hote hain: out, not out, ya third umpire ko bhejo. "Shayad out, shayad not out, chalo chhakka de dete hain" nahi hota. **Har sentence ka bhi faisla karo:** fact, explanation, condition, ya bahar.



4.3 The qualifier library

Use these in CONDITIONAL sentences:

Qualifier	Example
may / can	"Semaglutide may reduce appetite."

Qualifier	Example
some people / in some cases	"Some people notice nausea when the dose increases."
depends on	"Your plan depends on your health history."
varies / results vary	"Weight loss varies from person to person."
if appropriate / when appropriate	"A provider prescribes treatment when appropriate."
not guaranteed	"A prescription is not guaranteed."
talk to your provider	"Talk to your provider before changing your dose."

Warning: qualifiers can't rescue a PROHIBITED claim. "May cure obesity" is still prohibited. 🔵

4.4 Disclaimers you'll use most

Where	Line
Treatment mention	"Compounded medications are not FDA-approved. A licensed provider decides whether treatment is appropriate."
Brand trial results	"Results are from studies of [brand product]; compounded versions were not studied in these trials."
Educational article	"This article is for education and is not medical advice. Talk to your provider about your health."
Testimonial	"Shared with permission. Individual experiences vary."
Brand name in education	"Ongo Weight Loss does not offer [brand name]."

🟧 **Ongo Weight Loss example**

On our homepage (30 October 2026), the key line "Compounded semaglutide and compounded tirzepatide are not FDA-approved medications" sits in the FAQ and footer, away from the product cards. Competitor Mochi places "Compounded medications are not approved or evaluated by the FDA for safety, effectiveness, or quality" near its compounded products. Writers should place the disclaimer **next to** the treatment copy, not only in the footer. 🔵

4.5 Do's and Don'ts

✅ Do	❌ Don't
Sort sentences into the four types	Write "confident" lines from memory
Put disclaimers next to treatment copy	Hide disclaimers in footers only
Name trials when citing results	Borrow brand results for our product
Ask legal about any price or offer copy	Invent savings claims

4.6 Pros and cons of strict claim rules

✅ Pros	❌ Cons
Legal and regulatory safety	Less "exciting" copy
Trust with readers	Takes practice
Faster reviews over time	Must update when rules change

4.7 Where freshers go wrong in Chapter 4 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Same active ingredient as Wegovy" in a CTA	Copying competitors	Rule 4	Removing; reviewed education only
"May guarantee results"	Thinking "may" fixes everything	4.3 warning	Deleting the claim
Old price in a blog	Prices changed	Rule 8	Checking the site before submitting

4.8 Thumb rules

👍 Compounded, not FDA-approved, provider decides.

👍 Fact, explanation, condition, or cut.

👍 A qualifier softens a condition; it can't save a promise.

4.9 Notes to remember

📝 Never name the partner pharmacy or use brand drug names commercially.

📝 Disclaimers belong next to the claim they qualify.

Chapter 4 Glossary

Term	Simple meaning
Claim	A statement about what something is or does
Qualifier	A word that limits a claim ("may", "some")
Disclaimer	A sentence that clarifies limits or status
Compounded medication	A medication prepared by a licensed pharmacy for a patient; not FDA-approved
Substantiation	Evidence that supports a claim

Chapter 4 Quiz

1. How must compounded medications be described?

- A) Generic Wegovy
- B) Not FDA-approved
- C) Identical to brands

- D) FDA-approved

2. Which claim type needs a qualifier?

- A) Conditional
- B) Prohibited
- C) Factual
- D) None

3. Can a qualifier rescue "cures obesity"?

- A) No; it's prohibited
- B) Yes, add "may"
- C) Only in FAQs
- D) Yes, add "some"

4. What is the pharmacy line?

- A) Nothing at all, ever
- B) "A licensed U.S. compounding pharmacy"
- C) The pharmacy's brand name
- D) "Our secret lab"

5. Which is FACTUAL and allowed?

- A) "You'll lose 10 lbs."
- B) "Doctors approve almost everyone."
- C) "Compounded medications are not FDA-approved."
- D) "No side effects."

6. Where should treatment disclaimers go?

- A) In image alt text only
- B) Only in the footer
- C) Next to the treatment copy
- D) Nowhere

7. What is our insurance line?

- A) "Covered by insurance"
- B) "Bill your insurer"
- C) "Insurance guaranteed"

- D) "No insurance needed" (self-pay)

8. How are trial results cited?

- A) Name the trial and brand product; say compounded versions weren't studied
- B) Hide the source
- C) Claim them for our product
- D) Round them up

9. Which phrase did FDA letters flag?

- A) "Educational only"
- B) "Generic Ozempic"
- C) "Results vary"
- D) "Talk to your provider"

10. In the umpire analogy, every sentence…

- A) Is always out
- B) Gets a six
- C) Gets a clear decision: fact, explanation, condition or out
- D) Is ignored

Sources

1. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025) 🟠
2. Mochi Health, homepage, viewed 30 October 2026 🟠
3. Ongo Weight Loss, Book One (standing rules) and homepage review, 30 October 2026 (internal) 🔵

Answer key, Chapter 4: 1-B · 2-A · 3-A · 4-B · 5-C · 6-C · 7-D · 8-A · 9-B · 10-C

LEVEL 1 · CHAPTER 5

Chapter 5: The Ongo Weight Loss Voice and Tone Guide

Voice is who we are on every page. Tone is how we adjust to the moment. This is the first version of our guide, built from what works for trusted health brands and what our readers need.

> 💡 **Trivia**
>
> Hims' weight-loss page leads with "It's not you, it's biology." Calibrate's says "It's About Biology Not Willpower." Ro's says "It's not just a matter of willpower." [1][2][3] 🟠 Industry (competitor pages, October 2026)
>
> Three big brands, one shared message: **remove shame first.** That is now the expected tone in this category.

5.1 Our voice in four words

Voice trait	We are…	We are not…	Sounds like
Warm	Kind, human, on the reader's side	Cold, clinical, preachy	"You're not alone in this, and it's not about willpower alone."
Clear	Plain words, answer first	Jargon, long build-ups	"Semaglutide is a weekly injection that can reduce appetite."
Honest	Specific, sourced, open about limits	Hype, vague promises	"Results vary. Some people lose more, some less."
Steady	Calm, confident, medical-grade	Alarmist, salesy, jokey about bodies	"Here's what to expect in the first month."

One-line summary: *A knowledgeable, kind provider explaining things clearly over a cup of tea, never a salesperson, never a lecturer.* 🔵

> 🪔 **Desi analogy: ghar ka family doctor**
>
> Family doctor aapse "aap" bolta hai, aapki baat poori sunta hai, sach batata hai, aur kabhi sharminda nahi karta. Na woh aapko dawai bechne ki jaldi mein hota hai, na lambi lecture deta hai. **Ongo Weight Loss ki awaaz wahi family doctor hai.**

5.2 Tone changes by situation

Situation	Tone dial	Example
Education (blog)	Warm 3 · Clinical 3 · Sales 1	"GLP-1 medications work by..."
Service / medication page	Warm 3 · Clinical 3 · Sales 2	"If a provider decides treatment is right for you, here's what's included..."
Side effects / safety	Warm 3 · Clinical 4 · Sales 0	"Call your provider right away if you have severe stomach pain."
Pricing	Clear 4 · Sales 2	"[Price as on site]. This includes..."
Error messages / forms	Clear 4 · Warm 2	"Please enter your date of birth so we can check eligibility."
Social captions	Warm 4 · Casual 3 · Sales 1	Book Five

(0 = none, 4 = strong) 🔵

5.3 Point of view

Use	When
"You" (second person)	Most pages: speak to the reader directly
"We" / "Ongo Weight Loss"	When describing what our team or providers do
"People" / "patients" (third person)	When describing research or general experiences ("Patients in the trial...")
"I"	Only in a named provider's or author's own quote or byline content

5.4 Words we use and avoid

We say	We avoid	Why
"weight care", "weight management"	"battle the bulge", "fight fat"	Respectful, medical
"person with obesity"	"obese person", "the obese"	People-first (Chapter 7)
"your goals", "your health"	"your dream body", "bikini body"	Health, not appearance pressure
"provider", "licensed provider"	"our doctors will approve you"	Accuracy; no approval promise
"compounded semaglutide"	"generic Ozempic", "semaglutide (same as Wegovy)"	Compliance
"may help", "some people"	"melts fat", "works for everyone"	Honesty
"start with a free eligibility check"	"Buy now!", "Limited time only!"	No pressure

Full list: Chapter 7 and Appendix C.

5.5 Style rules (house style)

Item	Rule
Spelling	U.S. English (color, center, program)
Headings	Sentence case for blog H2/H3 ("How long does nausea last?"); title case allowed for page H1s per design
Numbers	Numerals for 10 and above and for all doses, prices and percentages ("2.4 mg", "15%")
Units	"mg", "lbs" (with a space: "20 lbs"); "kg" in parentheses if useful
Dates	Month Day, Year ("October 30, 2026")
Drug names	Lowercase generic names (semaglutide); brand names only in reviewed education, with ® on first use
Contractions	Allowed ("you're", "don't") for warmth; avoid in legal and policy pages

Item	Rule
Exclamation marks	Rare; never in medical or safety copy
Emojis	Not on web pages (social only)
Oxford comma	Use it ("nausea, constipation, and fatigue")

5.6 What competitors teach us about voice

Brand	Voice we observed	Learn	Avoid
Hims	Warm, shame-free ("It's not you, it's biology.")	Removing blame	Vague medication wording
Ro	Confident, direct, benefit-led	Clear process language, labelled paid testimonials	Heavy CTA repetition
Mochi	Simple, direct ("Weight loss medication, prescribed online.")	Short, honest headlines; clear compounded labels	Promo urgency banners
IVIM	Punchy, personal ("Not a bot. Not a one-time consult.")	Contrast lines that show care	Missing compounding disclaimer on homepage
Henry Meds	Hype ("The Solution That Finally Works!")	—	Overclaiming, "same ingredient as" cards
Mayo Clinic	Neutral, plain, structured	Clinical clarity	Too dry for marketing pages

Details in Chapter 14. [1–6] 🟠

> 🟧 **Ongo Weight Loss example**
>
> Our homepage line "Your Weight Loss Shouldn't Need Insurance" is on-voice: clear, warm, honest. "Guided. Tailored. Care." is off-voice: it sounds like a slogan and says nothing specific. A better version: "A licensed provider guides your plan and checks in as you go." 🔵

5.7 Do's and Don'ts

✅ Do	❌ Don't
Sound like a kind, knowledgeable provider	Sound like an infomercial
Remove shame and blame	Imply weight is a character flaw
Adjust tone to the situation	Use jokes in safety copy
Follow house style	Mix UK and U.S. spelling

5.8 Pros and cons of a written voice guide

✅ Pros	❌ Cons
Every writer sounds like one brand	Needs updates as the brand grows
Faster reviews	Can feel limiting at first
Easier onboarding	Must be enforced by editors

5.9 Where freshers go wrong in Chapter 5 topics

Mistake	Why it happens	Avoid it by	Fix it by
Slogan headings ("Guided. Tailored. Care.")	Copying ad style	Clear-first rule	Rewriting as a plain benefit
"Crush your goals!!"	Fitness-blog habit	Tone dial	Calm wording
Switching between "you" and "patients" randomly	No POV plan	5.3 table	Choosing one per section

5.10 Thumb rules

👍 Warm, clear, honest, steady.

👍 Kind provider, not salesperson.

👍 Remove shame before you explain science.

5.11 Notes to remember

📝 U.S. English, sentence-case blog headings, numerals for doses and prices.

📝 No emojis or exclamation marks in medical copy.

Chapter 5 Glossary

Term	Simple meaning
Voice	The brand's consistent personality in writing
Tone	How the voice adjusts to a situation
House style	Agreed rules for spelling, numbers and formatting
Point of view (POV)	Who the text speaks as or to (I, we, you, they)
Sentence case	Only the first word and names capitalised

Chapter 5 Quiz

1. What are our four voice traits?

- A) Cold, technical, formal, long
- B) Salesy, urgent, exciting, vague
- C) Loud, fast, funny, bold
- D) Warm, clear, honest, steady

2. What message did Hims, Calibrate and Ro share?

- A) Weight is about biology, not just willpower (remove shame)
- B) Diet harder
- C) No side effects
- D) Buy now

3. What's the tone for safety copy?

- A) Clinical and warm, zero sales
- B) Casual with emojis
- C) Very salesy
- D) Jokey

4. Which phrase is on-voice?

- A) "Bikini body in 30 days"
- B) "A licensed provider guides your plan and checks in as you go."
- C) "Guided. Tailored. Care."
- D) "Melt fat fast!"

5. Which spelling do we use?

- A) UK English
- B) Mixed
- C) U.S. English
- D) Any

6. How are doses written?

- A) Roman numerals
- B) Words ("two point four milligrams")
- C) Emojis
- D) Numerals ("2.4 mg")

7. Which competitor voice is a warning example?

- A) Mayo Clinic's plain sections
- B) Henry Meds: "The Solution That Finally Works!"
- C) Hims' shame-free line
- D) Mochi's simple headline

8. When do we use "I"?

- A) Always
- B) Never in any form
- C) In error messages
- D) Only in a named provider's or author's own quote

9. Are emojis used on web pages?

- A) No (social only)
- B) In safety copy
- C) In prices
- D) Yes, everywhere

10. In the family-doctor analogy, our voice…

- A) Rushes to sell
- B) Uses jokes about weight
- C) Lectures for an hour
- D) Listens, tells the truth and never shames

Sources

1. Hims, Weight loss, viewed 30 October 2026
2. Calibrate, homepage, viewed 30 October 2026
3. Ro, Weight loss, viewed 30 October 2026
4. Mochi Health, homepage, viewed 30 October 2026
5. IVIM Health, homepage, viewed 30 October 2026
6. Henry Meds, Weight management, viewed 30 October 2026
7. Ongo Weight Loss, voice and tone guide v1, October 2026 (internal)

Answer key, Chapter 5: 1-D · 2-A · 3-A · 4-B · 5-C · 6-D · 7-B · 8-D · 9-A · 10-D

LEVEL 1 · CHAPTER 6

Chapter 6: Who We Write For: Readers, Personas and Reading Level

You can't write clearly for "everyone". You write for a specific person. This chapter introduces our readers and how simple your writing needs to be.

> 💡 **Trivia**
>
> The AMA and NIH recommend that patient education materials be written at a **sixth- to eighth-grade reading level or lower**. [1] 🟠 Industry (JMIR, 2023)
>
> Most of our readers are smart adults. Simple writing isn't "dumbing down"; it's respect for a tired, worried reader on a phone.

6.1 What research says about our readers

Fact	Source	What it means for writing
12% of U.S. adults currently take a GLP-1; **18%** ever have	KFF, Nov 2025 [2] 🟢	Many readers already know the basics; skip the obvious
Use is highest among adults **50–64 (22%)**	KFF [2] 🟢	Write clearly for older readers; avoid slang
Women **15%** vs men **9%**	KFF [2] 🟢	Address women's concerns (hormones, menopause) without excluding men
56% of users say GLP-1s are hard to afford	KFF [2] 🟢	Be clear and honest about cost
17% got the medication from an online provider/website	KFF [2] 🟢	Explain how telehealth works and why it's safe

6.2 Four personas (use them in every brief)

Persona	Who	Main worry	What they need from your page
Linda, 58, first-timer	Tried many diets; heard about GLP-1s from a friend	"Is this safe for someone my age? Is it a scam?"	Calm safety facts, who the providers are, honest cost
Marcus, 44, comparer	Researching prices and options	"Why compounded? Why not the brand?"	Clear compounded vs brand facts, what's included, no hype
Priya, 36, busy professional	Wants online care that fits her schedule	"How fast, how easy, how private?"	Simple steps, timelines, privacy
Dana, 51, current patient	On treatment; side effects or a plateau	"Is this normal? What do I do?"	Practical tips, red-flag symptoms, "contact your provider"

Personas are composites for planning, not real patients. 🔵

> 🍛 **Desi analogy: restaurant ka menu**
>
> Achha restaurant menu dekhta hai ki kaun aa raha hai: office waale ko jaldi khaana, family ko bada thaal, buzurgon ko kam teekha. **Writer bhi pehle poochta hai: "Aaj kaun padh raha hai?"**

6.3 Health literacy in plain terms

Health literacy is how easily someone can find, understand and use health information. It's affected by stress, illness, language and time, not only education.

Readers find it hard when…	So you…
Words are medical ("gastrointestinal adverse events")	Say "stomach side effects like nausea"
Numbers are complex ("HR 0.80, 95% CI")	Say "about 20% lower risk in the study" (and check with the reviewer)
Sentences are long and passive	Use short, active sentences

Readers find it hard when…	So you…
The answer is buried	Put it first (Chapter 20)
Pages are walls of text	Use headings, lists, tables (Chapter 30)

The CDC Clear Communication Index (4 introductory questions + 20 scored items) covers exactly these areas: main message, calls to action, language, numbers and risk. [3] 🟢

6.4 Reading-level targets

Content	Target grade level
Service, medication, cost, how-it-works pages	6–8
Blog guides and FAQs	7–8
Comparisons and research summaries	8–9 (define every term)
Policies (legal)	As simple as legal allows

Caution: NIH warns that readability formulas "only measure the difficulty of the vocabulary used and average sentence length." A low score doesn't guarantee clarity. Read it aloud too. [4] 🟢

> 🟧 **Ongo Weight Loss example**
>
> "Personalized, pharmacy-compounded formula" (homepage product card) is hard for Linda. Clearer: "Compounded semaglutide, prepared by a licensed U.S. pharmacy for you. Not FDA-approved. A provider decides if it's right for you." 🔵

6.5 Do's and Don'ts

✅ Do	❌ Don't
Pick one persona per piece	Write for "everyone"
Aim for grade 6–8 on money pages	Show off vocabulary
Explain numbers in everyday terms	Paste statistics without context

✅ Do	❌ Don't
Read your draft aloud	Trust a readability score alone

6.6 Pros and cons of simple writing

✅ Pros	❌ Cons
More people understand and act	Takes more effort to write
Fewer support questions	Experts may find it basic (link to detail)
Better for mobile and AI answers	Some terms can't be simplified; define them

6.7 Where freshers go wrong in Chapter 6 topics

Mistake	Why it happens	Avoid it by	Fix it by
Medical jargon everywhere	Copying journal language	6.3 table	Plain-word swaps
Writing for 25-year-olds only	Assumption	KFF data	Persona check
Long, complex sentences	Trying to sound smart	Grade targets	Splitting sentences

6.8 Thumb rules

👍 Write for Linda: smart, worried, on a phone.

👍 Grade 6–8 for money pages.

👍 Formula scores help; your ears decide.

6.9 Notes to remember

📝 GLP-1 use peaks at ages 50–64 (KFF, 2025).

📝 AMA/NIH: sixth- to eighth-grade reading level or lower.

Chapter 6 Glossary

Term	Simple meaning
Persona	A made-up but research-based example reader
Health literacy	How easily someone can find, understand and use health information
Reading level	How many years of school a text roughly needs
Readability formula	A score based on word and sentence length (Flesch, SMOG)
Clear Communication Index	CDC's checklist for clear health materials

Chapter 6 Quiz

1. What reading level do AMA and NIH recommend for patient materials?

- A) College level
- B) Sixth to eighth grade or lower
- C) PhD level
- D) No recommendation

2. Which age group has the highest GLP-1 use (KFF)?

- A) 65+
- B) 50–64
- C) 18–24
- D) Under 18

3. What share of users find GLP-1s hard to afford (KFF)?

- A) 56%
- B) 0%
- C) 99%
- D) 5%

4. Which persona worries "Is this normal?" during treatment?

- A) Priya, professional
- B) Linda, first-timer
- C) Dana, current patient
- D) Marcus, comparer

5. What is health literacy?

- A) Reading speed
- B) How easily someone can find, understand and use health information
- C) Typing skill
- D) A medical degree

6. What does NIH say about readability formulas?

- A) They measure truth
- B) They're perfect
- C) They replace reviewers
- D) They only measure vocabulary difficulty and sentence length

7. Which is the plainer phrase?

- A) "Gastrointestinal adverse events"
- B) "GI AEs"
- C) "Stomach side effects like nausea"
- D) "Emetogenic sequelae"

8. How many scored items does the CDC Clear Communication Index have?

- A) 0
- B) 200
- C) 2
- D) 20

9. What's the target grade level for service pages?

- A) 12–14
- B) 6–8
- C) None
- D) 16+

10. In the restaurant-menu analogy, the writer first asks…

- A) "Who is reading today?"
- B) "Which keyword?"
- C) "What font?"
- D) "How long can I make it?"

Sources

1. JMIR, Assessing the readability of online patient education materials in obstetrics and gynecology (2023) 🟠
2. KFF, Poll: 1 in 8 adults say they are currently taking a GLP-1 drug (November 14, 2025) 🟢
3. CDC, Clear Communication Index 🟢
4. NIH, Clear & Simple 🟢

Answer key, Chapter 6: 1-B · 2-B · 3-A · 4-C · 5-B · 6-D · 7-C · 8-D · 9-B · 10-A

LEVEL 1 · CHAPTER 7

Chapter 7: People-First, Weight-Respectful Language

The words you choose can make a reader feel judged and leave, or feel respected and keep reading. This chapter gives you the words to use and the words to drop.

> 💡 **Trivia**
>
> The Obesity Action Coalition promotes **people-first language**: "The man **with obesity** was on the bus," not "The man on the bus was very obese." Eleven organisations have signed on, including the Obesity Medicine Association, The Obesity Society, the World Obesity Federation and the Academy of Nutrition and Dietetics. [1] ● Official (OAC)
>
> Leading obesity medicine groups write this way. So do we.

7.1 What people-first language is

People-first language puts the person before the condition. A person *has* obesity; they are not "an obese". It avoids defining someone by a disease, the same way we say "a person with diabetes", not "a diabetic". [1] ●

✅ Use	❌ Avoid
person with obesity / people living with obesity	obese person, the obese, obese patients
person with higher weight / larger body (casual contexts)	fat, heavy, overweight people (as identity)
weight-related health conditions	weight problems, weight issues
weight management, weight care	weight battle, war on fat
person with type 2 diabetes	diabetic

7.2 Words that shame or blame

❌ Avoid	Why	✅ Instead
"lack of willpower", "just eat less"	Obesity is a complex, chronic condition; blame drives people away	"Weight is influenced by biology, environment and habits"
"cheat day", "guilty pleasure", "bad food"	Moralises eating	"a treat", "foods you enjoy in balance"
"letting yourself go"	Shames	— (cut)
"fix your body", "problem areas"	Body shame	"reach your health goals"
"bikini body", "summer body", "revenge body"	Appearance pressure	"feel stronger", "move more comfortably"
"finally be happy" (after weight loss)	Implies worth depends on weight	"support your health"

> 🍛 **Desi analogy: shaadi mein rishtedaar**
>
> Har shaadi mein ek aunty hoti hai: "Arre, kitna mota ho gaya!" Aur ek hoti hai: "Kitne dino baad mile, kaise ho?" Dusri aunty ke paas sab baithte hain. **Hamare shabd dusri aunty jaise hone chahiye.**

7.3 Hype words to drop (health-specific)

❌ Risky	Why	✅ Safer
guaranteed, guarantee	Absolute promise	"results vary", "not guaranteed"
melts fat, burns fat fast	Misleading mechanism and speed	"may reduce appetite and help with weight loss"
miracle, magic shot, wonder drug	Hype	"a prescription medication"
fast, rapid, overnight, in 30 days	Speed claims	"over time", "gradually"
no side effects, 100% safe, risk-free	False	"side effects can occur; talk to your provider"

❌ Risky	Why	✅ Safer
works for everyone	Universal claim	"may not be right for everyone"
best, #1, top-rated (unsourced)	Unverifiable superlative	"licensed", "doctor-led", or the sourced rating
clinically proven (for our compounded products)	FDA-flagged	Name the brand trial and its limits
generic Ozempic / same as Wegovy	FDA-flagged; untrue	"compounded semaglutide"
easy approval, instant prescription	Implies no medical judgment	"a provider reviews your health history"
cure, cures obesity	Medical misinformation	"helps manage", "treatment option"

7.4 Filler phrases to cut

❌ Cut	Why
"In today's world..."	Says nothing
"It is important to note that..."	Just say the thing
"When it comes to..."	Wordy
"In this article, we will discuss..."	Show, don't announce
"At the end of the day..."	Cliché
"Many people believe..." (without a source)	Vague
"Look no further!"	Salesy

7.5 Inclusive language beyond weight

Area	Guidance
Gender	Use "people" or "patients" where gender doesn't matter; say "women" when research is about women (e.g., menopause)

Area	Guidance
Age	Avoid "elderly"; say "older adults" or specific ages
Ability	"person with a disability" or identity-first if the community prefers; avoid "suffers from"
Culture and food	Don't label cuisines as "unhealthy"; give swaps within the food tradition

> 🟧 **Ongo Weight Loss example**
>
> The homepage says "Losing stubborn weight is possible with the right medical support." "Stubborn weight" is common marketing language, but it can feel blaming. A respectful version: "With medical support, weight loss can feel more manageable." 🔵

7.6 Do's and Don'ts

✅ Do	❌ Don't
Put the person first	Label people by their weight
Talk about health and how people feel	Focus only on appearance
Use calm, accurate words	Use hype or fear
Search your draft for the risky list	Trust your memory

7.7 Pros and cons of people-first language

✅ Pros	❌ Cons
Readers feel respected, stay and trust	Slightly longer phrasing
Matches medical organisations' standards	Some searchers type "obese"; use it only where needed in search terms, not to describe people
Reduces complaints and risk	Needs habit change

7.8 Where freshers go wrong in Chapter 7 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Obese patients may benefit..."	Copying research papers	People-first table	"Patients with obesity may benefit..."
"Melt stubborn belly fat"	Ad habit	Hype list	Health-focused wording
"Guilt-free dessert"	Common blog phrase	Shame list	"A lighter dessert"

7.9 Thumb rules

👍 Person first, condition second.

👍 Health and feelings, not shame and looks.

👍 If it sounds like an infomercial, rewrite it.

7.10 Notes to remember

📝 OAC: 11 organisations use people-first language.

📝 Run the risky-word search (Appendix C) before every submission.

Chapter 7 Glossary

Term	Simple meaning
People-first language	Putting the person before the condition ("person with obesity")
Weight bias	Negative attitudes toward people because of their weight
Weight stigma	Shame or discrimination linked to weight

Term	Simple meaning
Hype word	An exaggerated word that overpromises
Filler	Words that add length but no meaning

Chapter 7 Quiz

1. Which is people-first language?

- A) "The obese"
- B) "Person with obesity"
- C) "Fatty"
- D) "Obese person"

2. Which organisation promotes people-first language for obesity?

- A) A social media app
- B) Obesity Action Coalition
- C) A fast-food chain
- D) A gym brand

3. Which phrase should be cut as filler?

- A) "Compounded medications are not FDA-approved."
- B) "Talk to your provider."
- C) "Results vary."
- D) "It is important to note that..."

4. What should replace "melts fat fast"?

- A) "May reduce appetite and help with weight loss"
- B) "Miracle shot"
- C) "Burns fat overnight"
- D) "Guaranteed"

5. Why avoid "lack of willpower"?

- A) It's a brand name
- B) It blames people for a complex condition

- C) It's a keyword
- D) It's too short

6. Which is a safer word than "elderly"?

- A) "Seniors citizens only"
- B) "Aged"
- C) "Old people"
- D) "Older adults"

7. Is "clinically proven" OK for our compounded products?

- A) Only in titles
- B) Yes
- C) No; it's FDA-flagged
- D) Only in emails

8. How many organisations signed on to OAC's approach?

- A) None
- B) Eleven
- C) One
- D) Fifty

9. What's a respectful rewrite of "stubborn weight"?

- A) "Lazy weight"
- B) "Problem areas"
- C) "Destroy stubborn fat"
- D) "With medical support, weight loss can feel more manageable."

10. In the shaadi-aunty analogy, our words should be like the aunty who…

- A) Says "kitna mota ho gaya"
- B) Ignores you
- C) Gives diet advice
- D) Asks how you are, not comments on your weight

Sources

1. Obesity Action Coalition, People-First Language
2. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025)
3. Ongo Weight Loss, homepage copy review, 30 October 2026 (internal)

Answer key, Chapter 7: 1-B · 2-B · 3-D · 4-A · 5-B · 6-D · 7-C · 8-B · 9-D · 10-D

LEVEL 1 · CHAPTER 8

Chapter 8: Medical Basics Every Writer Must Know

You don't need to be a doctor. You do need to understand the basics well enough to explain them correctly, spot errors and ask the reviewer the right questions.

> 💡 **Trivia**
>
> In the STEP 1 trial of brand semaglutide 2.4 mg, adults lost an average of **14.9%** of body weight over 68 weeks, vs **2.4%** with placebo. In SURMOUNT-1, brand tirzepatide 15 mg led to **20.9%** average loss over 72 weeks. [1][2] ● Official (NEJM trials)
>
> These are brand products in controlled trials. Compounded versions were not studied in these trials, and you must say so whenever you cite them.

8.1 Key terms in plain English

Term	Plain meaning
GLP-1	A natural gut hormone that helps control blood sugar and appetite
GLP-1 receptor agonist	A medication that acts like GLP-1 (semaglutide is one)
GIP	Another gut hormone; tirzepatide acts on **both** GIP and GLP-1 receptors
Semaglutide	The active ingredient name; brand products are made by Novo Nordisk
Tirzepatide	The active ingredient name; brand products are made by Eli Lilly
Compounded medication	Prepared by a licensed pharmacy for a patient's prescription; **not FDA-approved**
Titration	Starting at a low dose and increasing slowly to reduce side effects
Subcutaneous	Injected under the skin

Term	Plain meaning
Placebo	An inactive treatment used for comparison in trials
Contraindication	A reason someone should not use a medication

8.2 How GLP-1 medications work (explain it like this)

> GLP-1 medications act like a natural hormone your gut releases after eating. They can help you feel full sooner, slow how quickly your stomach empties, and reduce appetite. A provider decides whether they're right for you.

This short version is reviewer-friendly. Avoid "melts fat" or "resets metabolism". 🔵

> 🍛 **Desi analogy: shaadi ka buffet**
>
> GLP-1 aisa hai jaise shaadi mein koi aapko pyaar se yaad dilaye "pet bhar gaya hai, ab mithai thodi der baad." Khaana band nahi karta; **bhookh aur pet bharne ka signal** thoda jaldi aur saaf mehsoos hota hai.

8.3 BMI basics (CDC adult categories)

BMI (kg/m²)	Category
Less than 18.5	Underweight
18.5 to less than 25	Healthy weight
25 to less than 30	Overweight
30 or greater	Obesity (Class 1: 30–<35, Class 2: 35–<40, Class 3: 40+)

The CDC calls BMI "a screening measure" that "should be considered with other factors". Write "BMI is one screening tool", never "BMI decides your health". [3] 🟢

8.4 Side effects and safety: how to write them

Rule	Example
Name common side effects plainly	"The most common side effects are stomach-related, such as nausea, diarrhea, vomiting and constipation."
Say when they tend to happen	"They're often more noticeable when starting or increasing a dose."
Give red flags and actions	"Call your provider right away for severe stomach pain that won't go away."
Never minimise	❌ "Side effects are mild and rare."
Never diagnose	❌ "If you feel X, it's just Y."
Point to the prescriber	"Talk to your provider before changing or stopping your dose."

Every side-effect sentence goes to the medical reviewer. Brand labels carry serious warnings (for example, about thyroid C-cell tumours); the reviewer decides how we present safety information for compounded products. 🔵

8.5 Reading a study without misreading it

Check	Question	Why
Type	Randomised controlled trial (RCT), observational, review?	RCTs give the strongest evidence of cause
Who	Who was studied (age, BMI, diabetes)?	Results may not apply to everyone
What	Brand or compounded? What dose?	Our products may differ
How long	Weeks? Years?	Short studies can't show long-term effects
Compared with	Placebo? Another drug?	Changes what the result means
Average vs individual	Mean loss vs range	"Average 14.9%" doesn't mean everyone lost that
Relative vs absolute	"50% lower risk" of what baseline?	Relative numbers can sound bigger

Check	Question	Why
Funding	Who paid?	Not disqualifying, but note it

🟧 **Ongo Weight Loss example**

Competitor IVIM labels its outcome stats: "Not randomized controlled trials. Individual results may vary." That's a good habit. If we ever publish our own patient data, say what kind of data it is, how many people, over what time, and that results vary. [4] 🟠

8.6 Do's and Don'ts

✅ Do	❌ Don't
Name the trial, product, dose and time	Write "studies show" with no detail
Say compounded versions weren't studied	Apply brand trial results to our product
Explain averages and ranges	Turn averages into promises
Send every medical sentence to review	Simplify until it's wrong

8.7 Pros and cons of explaining science to readers

✅ Pros	❌ Cons
Builds expertise and trust	Easy to oversimplify
Helps readers talk to providers	Needs careful review
Earns links and citations	Science changes; needs updates

8.8 Where freshers go wrong in Chapter 8 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Semaglutide helps you lose 15%"	Rounding the trial into a promise	Study checklist	"In STEP 1, brand semaglutide...; results vary"
Calling tirzepatide "a GLP-1" only	Simplifying	8.1 table	"acts on GIP and GLP-1 receptors"
"BMI over 30 means unhealthy"	Over-reading BMI	CDC wording	"BMI is one screening tool"

8.9 Thumb rules

👍 Brand trial, brand product, brand result.

👍 Average is not a promise.

👍 Side effects: plain, honest, with an action.

8.10 Notes to remember

📝 STEP 1: 14.9% vs 2.4% at 68 weeks (brand semaglutide 2.4 mg).

📝 SURMOUNT-1: up to 20.9% at 72 weeks (brand tirzepatide 15 mg).

Chapter 8 Glossary

Term	Simple meaning
GLP-1 receptor agonist	A medicine that acts like the GLP-1 hormone
Titration	Slowly increasing a dose
RCT	Randomised controlled trial: strongest study type for cause and effect

Term	Simple meaning
Placebo	Inactive comparison treatment
BMI	Body mass index: a screening measure of weight for height

Chapter 8 Quiz

1. What did STEP 1 show for brand semaglutide 2.4 mg?

- A) No effect
- B) 14.9% average weight loss over 68 weeks vs 2.4% placebo
- C) 50% in 2 weeks
- D) 100% for everyone

2. Which hormones does tirzepatide act on?

- A) Cortisol
- B) GIP and GLP-1
- C) Thyroid
- D) Insulin only

3. Were compounded versions studied in these trials?

- A) Yes
- B) Unknown, so assume yes
- C) Only tirzepatide
- D) No

4. What does the CDC call BMI?

- A) A diagnosis
- B) A perfect health score
- C) A medication
- D) A screening measure

5. What is titration?

- A) A side effect
- B) Starting low and increasing the dose slowly

- C) Stopping treatment
- D) A blood test

6. Which side-effect sentence is acceptable?

- A) "Ignore nausea."
- B) "Side effects are always mild."
- C) "There are no side effects."
- D) "Common side effects include nausea and diarrhea; contact your provider if severe."

7. What does "average 14.9%" mean?

- A) The maximum loss
- B) A group average, not a promise for each person
- C) Everyone lost 14.9%
- D) The minimum loss

8. Which study type gives the strongest evidence of cause?

- A) Social media poll
- B) Testimonial
- C) Anonymous blog post
- D) Randomised controlled trial

9. What BMI range is "overweight" (CDC)?

- A) 18.5 to 25
- B) 25 to less than 30
- C) 40+
- D) 30+

10. In the buffet analogy, GLP-1 is like...

- A) A gentle signal that you're full
- B) A diet pill that melts fat
- C) Locking the kitchen
- D) Extra dessert

Sources

1. Wilding et al., Once-Weekly Semaglutide in Adults with Overweight or Obesity (STEP 1), *New England Journal of Medicine* (2021) ●
2. Jastreboff et al., Tirzepatide Once Weekly for the Treatment of Obesity (SURMOUNT-1), *New England Journal of Medicine* (2022) ●
3. CDC, Adult BMI Categories ●
4. IVIM Health, homepage, viewed 30 October 2026 ●

Answer key, Chapter 8: 1-B · 2-B · 3-D · 4-D · 5-B · 6-D · 7-B · 8-D · 9-B · 10-A

02

LEVEL 2

Preparing to Write

Purpose first, briefs and content types, intent and funnel, keyword and question research, sources, competitors, outlines, claim logs and AI.

9 · The Writer's Mindset: Purpose First

10 · Content Briefs and the 38 Content Types

11 · Search Intent and the Funnel

12 · Keyword and Question Research for Writers

13 · Researching a Health Topic: The Evidence Ladder

14 · How Competitors Write: Best Pages, Blogs and Why They Work

15 · Building the Outline and Writing Headings

16 · Fact Sheets and Claim Logs

17 · Using AI Safely: Consultant, Not Co-Writer

LEVEL 2 · CHAPTER 9

Chapter 9: The Writer's Mindset: Purpose First

Before keywords, length or headings comes one question: why would a real person be glad this page exists? Answer it in one sentence, and the rest of the page gets easier.

> 💡 **Trivia**
>
> Google lists "Are you writing to a particular word count because you've heard or read that Google has a preferred word count?" as a warning sign of search-engine-first content. [1] 🟢 Official (Google)
>
> There is no magic length. There is only "enough to fully help".

9.1 The purpose statement

Before you write anything, write one sentence that starts with **"This page exists to..."** Keep it at the top of your draft (not published).

✅ Strong purpose statement	❌ Weak purpose statement
"This page exists to explain what nausea on semaglutide feels like, how long it usually lasts, what helps, and when to call a provider."	"This page exists to rank for 'semaglutide nausea'."
"This page exists to show exactly what's included in the Ongo Weight Loss price, so people can compare honestly."	"This page exists to get more sign-ups."

Test: Why would a real human be glad this page exists? If your answer is vague or only about sales, rethink the page. 🔵

9.2 WHY → WHAT → HOW

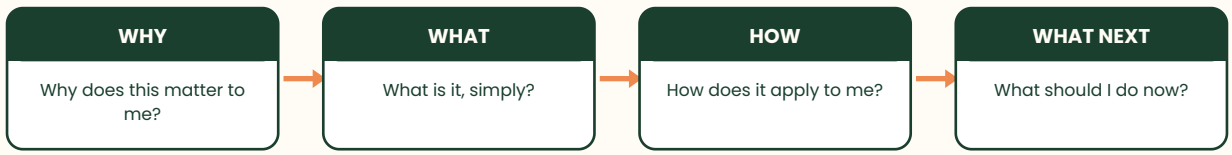
People learn in this order:

Step	Reader's inner question	Example (semaglutide nausea)
WHY	Why does this matter to me?	"Nausea is the most common reason people feel unsure in the first weeks."
WHAT	What is it, simply?	"Semaglutide slows how quickly your stomach empties, which can cause nausea."
HOW	How does it apply to me / what do I do?	"Eat smaller meals, avoid greasy foods, sip water; call your provider if..."

Sketch these three as rough notes before writing sentences.

> **Desi analogy: IT office ka requirement doc**
>
> Developer pehle code nahi likhta; pehle samajhta hai "client ko ye feature kyun chahiye, kya chahiye, kaise kaam karega." Bina requirement ke code bekaar hota hai. **Purpose statement hamara requirement doc hai.**

How People Learn



Sketch these four before writing a sentence.

9.3 The three helpful reactions

Your page succeeds if the reader can say:

1. "That answered my question."
2. "I understand this better now."
3. "I know what to do next."

9.4 The four purpose tests (from our checklist set)

Test	Letters	Use when
PURPOSE	Beneficial · Clear · Honest · Focused · Achieved	Before writing
MASK	Match intent · Avoid deception · Serve one goal · Knowledge-first	Planning a money page
HUMAN	Helpful · Understandable · Meaningful depth · Accurate · Not manipulative	After drafting
RIGHT	Relevant · Intent-matched · (Right audience) · Helpful next step · Timely	Checking search intent

All checklists are collected in Appendix H. 🔵

9.5 The Goldilocks rule for length

Too short	Just right	Too long
Reader goes back to Google	Reader stops searching	Reader gets overwhelmed and leaves
Missing steps, limits, safety	Answers fully, then stops	Padding, repetition, history lessons

If 300 words answer it fully, don't write 1,500. If it honestly needs 2,000, don't cut to 800. 🔵

> 🟧 **Ongo Weight Loss example**
>
> Our blog has two near-identical dosing guides (semaglutide and tirzepatide) with long, keyword-packed titles. Purpose statements would sharpen them: "This page exists to help a current patient understand their compounded semaglutide dose schedule and measure it safely, with their provider's instructions as the final word." 🔵

9.6 Do's and Don'ts

✅ Do	❌ Don't
Write the purpose statement first	Start with keywords

✅ Do	❌ Don't
Sketch WHY → WHAT → HOW	Jump straight into facts
Stop when the reader is helped	Pad to a word count
Cut paragraphs that don't serve the purpose	Keep "nice" but off-topic sections

9.7 Pros and cons of purpose-first writing

✅ Pros	❌ Cons
Faster drafting, less rewriting	Takes 5 minutes before you "start"
Clear pages that rank and convert	Forces you to cut favourite paragraphs
Easier reviews	—

9.8 Where freshers go wrong in Chapter 9 topics

Mistake	Why it happens	Avoid it by	Fix it by
History intro ("GLP-1 was discovered in...")	Thinking context = value	WHY → WHAT → HOW	Moving history to the end or cutting
Purpose = "rank for X"	SEO habit	9.1 test	Rewriting as reader benefit
Padding to 1,500 words	Brief said so	Goldilocks rule	Asking the team lead

9.9 Thumb rules

> 👍 "This page exists to..." before anything else.
>
> 👍 Why, what, how: in that order.
>
> 👍 Enough to fully help, then stop.

9.10 Notes to remember

📝 Writing to a word count is a Google warning sign.

📝 Every paragraph must serve the purpose statement.

Chapter 9 Glossary

Term	Simple meaning
Purpose statement	One sentence explaining why the page exists
WHY → WHAT → HOW	The order people learn in
Goldilocks rule	Not too short, not too long: just enough
Padding	Extra words that add length but not value
North Star	The guiding goal for every decision

Chapter 9 Quiz

1. How does a purpose statement start?

- A) "Keywords:"
- B) "This page exists to..."
- C) "Once upon a time"
- D) "Buy now"

2. Which is a strong purpose statement?

- A) "...to get more clicks."
- B) "...to rank for a keyword."
- C) "...to explain what's included in the price so people can compare honestly."
- D) "...to reach 2,000 words."

3. What's the learning order?

- A) How → Why → What
- B) What → How → Why
- C) Random
- D) Why → What → How

4. What does Google say about writing to a word count?

- A) It's required
- B) It's a search-engine-first warning sign
- C) 2,000 words is ideal
- D) Shorter always ranks

5. Which is one of the three helpful reactions?

- A) "That was long."
- B) "I know what to do next."
- C) "Too many ads."
- D) "Where's the answer?"

6. What does the Goldilocks rule say?

- A) Match competitors' length
- B) Always 300 words
- C) Always 3,000 words
- D) Enough to fully help, then stop

7. Which checklist is used after drafting to test helpfulness?

- A) HUMAN
- B) FACTS
- C) READY
- D) None

8. What should happen to history intros?

- A) Make them longer
- B) Move to the end or cut
- C) Bold them
- D) Always lead with them

9. Is the purpose statement published?

- A) Yes, as the H1
- B) Yes, as alt text
- C) Yes, in the footer
- D) No; it stays in the draft notes

10. In the IT-office analogy, the purpose statement is like…

- A) The office party
- B) The requirement document
- C) The final invoice
- D) The coffee machine

Sources

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, writing guidelines and blog review, October 2026 (internal) 🔵

Answer key, Chapter 9: 1-B · 2-C · 3-D · 4-B · 5-B · 6-D · 7-A · 8-B · 9-D · 10-B

LEVEL 2 · CHAPTER 10

Chapter 10: Content Briefs and the 38 Content Types

Every piece of content starts with a brief and belongs to one content type. Pick the right type and half the writing decisions are made for you.

> 💡 **Trivia**
>
> Ro's health guide article on semaglutide nausea (updated August 24, 2026) is a **guide** with two authors, an MD reviewer, a key-takeaways box, ten numbered tips, a "Bottom line", FAQs and 20+ references. [1]
> ● Industry
>
> Its shape tells the reader, "This is a complete, trustworthy answer," before they read a word.

10.1 What a brief contains

Field	Example	If it's missing, ask…
Title / working title	"Semaglutide nausea: how long it lasts and what helps"	"What's the angle?"
Content type	Guide (blog)	"Which type is this?"
Primary keyword (one, unique)	semaglutide nausea	"Is this keyword already used on another page?"
Secondary / related keywords	how long does semaglutide nausea last; foods to avoid	—
Search intent and funnel	Informational · MOFU (current or soon-to-be patients)	"Who's searching and why?"
Persona	Dana, 51, current patient	—
Parent page and internal links	Semaglutide hub; side effects guide	"Where does it sit in the site map (Book Three)?"
Length guide	~1,200–1,600 words (Goldilocks)	—
CTA	Soft: "Questions? Message your provider"	"Which CTA and event?"

Field	Example	If it's missing, ask…
Reviewer needed	Medical + legal	—
Deadline	—	—

The one-keyword rule: every piece of content must have its **own unique primary keyword**. If it doesn't, it shouldn't be created (it would compete with our own page: "cannibalisation"). 🔵

> 🪔 **Desi analogy: restaurant ka order ticket**
>
> Waiter ka ticket saaf ho, "Table 4, 2 dal makhani, kam teekha", toh chef sahi khaana banata hai. Ticket adhoora ho toh guess karna padta hai. **Brief hamara order ticket hai:** adhoora ho toh poochho, guess mat karo.

10.2 The 38 content types

A. Money and conversion pages (BOFU)

#	Type	Purpose	Example for Ongo Weight Loss
1	**Service page**	Explain a service and how to start	Weight loss program
2	**Medication page**	Explain one medication we offer	Compounded semaglutide
3	**Landing page**	One focused goal for a campaign or audience	"Weight care without insurance"
4	**Cost / pricing page**	Price, what's included, what's not	Plans and pricing
5	**How-it-works page**	Steps from sign-up to delivery	4 steps
6	**Eligibility / assessment copy**	Questions, help text, results messages	Eligibility check
7	**Comparison page (us vs options)**	Help evaluate options honestly	Compounded vs brand: what's different

B. Education (TOFU and MOFU)

#	Type	Purpose	Example
8	**Pillar page**	Central hub for a major topic	GLP-1 medications: complete guide
9	**Guide**	Step-by-step education	Starting semaglutide: your first month
10	**Tutorial / how-to**	Practical task	How to measure your dose with a syringe
11	**FAQ page**	Direct answers to common questions	Semaglutide FAQs

#	Type	Purpose	Example
12	**Comparison article**	Compare two things	Semaglutide vs tirzepatide
13	**Listicle**	Numbered tips or ideas	10 high-protein breakfasts
14	**Myth vs fact**	Correct misinformation	7 GLP-1 myths
15	**Checklist**	Actionable list	Questions to ask your provider
16	**Glossary / vocabulary**	Define terms	GLP-1 terms explained
17	**Definition page**	One term in depth	What is titration?
18	**Side-effect guide**	One side effect, causes, relief, red flags	Constipation on GLP-1s
19	**Timeline / what to expect**	Week-by-week expectations	First 12 weeks
20	**Recipe / meal plan**	Food guidance	High-protein meal plan
21	**Lifestyle article**	Movement, sleep, stress	Strength training while losing weight
22	**News article**	Timely updates	FDA update on compounded GLP-1s (reviewed)

C. Authority and trust (E-E-A-T)

#	Type	Purpose	Example
23	**Report**	Research-backed insight	GLP-1 cost survey summary
24	**Whitepaper**	Deep, data-backed document	Compounded GLP-1s explained
25	**Interview**	Expert voice	Q&A with our medical director
26	**Expert opinion**	Provider's view on a topic	When to pause a dose increase
27	**Case study**	Real, consented example of our process	How one patient handled a plateau (with consent)
28	**Success story**	Patient experience (consented)	"What helped me most..."
29	**Testimonials page**	Social proof (FTC-compliant)	Patient reviews with disclosures
30	**Profile page**	Doctor or author bio	Dr. Johnathan Chance Miller, MD
31	**Editorial / medical review policy**	How content is made and checked	Editorial policy

#	Type	Purpose	Example
32	**Policy page**	Legal and practical rules	Privacy, refunds, shipping, telehealth consent

D. Support and operations

#	Type	Purpose	Example
33	**Help article**	Platform guidance	How to message your provider
34	**Troubleshooting**	Fix a problem	My shipment is late: what to do
35	**Technical documentation**	Process or compliance explanation	How prescriptions are reviewed
36	**Video script**	Explain visually	60-second "How it works"

E. Short and supporting copy

#	Type	Purpose	Example
37	**Microcopy**	Buttons, forms, errors, tooltips	"Check my eligibility"
38	**Meta / snippet / alt / email / social**	Short copy around content	Meta title, alt text, newsletter, caption

Skeletons for each type: Chapters 33–36 and Appendix D. Suggested length ranges for every type are in Appendix B. They are **starting estimates**: the right length is whatever fully answers the reader (Chapter 9). 🔵

10.3 Matching type to intent

Searcher types…	Intent	Best type
"what is semaglutide"	Learn	Definition / pillar
"semaglutide vs tirzepatide"	Compare	Comparison article
"how long does nausea last on semaglutide"	Specific answer	Side-effect guide / FAQ
"compounded semaglutide price"	Evaluate cost	Cost page
"ongo weight loss reviews"	Validate brand	Testimonials / reviews page
"online weight loss doctor"	Act	Service / landing page

🟧 **Ongo Weight Loss example**

The live blog has only five posts, three of which are dosing guides. Many high-value types are missing: side-effect guides, timelines, myth vs fact, comparisons and recipes. Each one needs its own unique keyword and a place in the Book Three content map before it's written. 🔵

10.4 Do's and Don'ts

✅ Do	❌ Don't
Confirm type, keyword and intent before writing	Start writing from a title alone
Check the keyword isn't already used	Create two pages for one keyword
Ask for missing brief fields	Guess the CTA or reviewer
Use the type's skeleton	Invent a new structure every time

10.5 Pros and cons of fixed content types

✅ Pros	❌ Cons
Faster writing and reviewing	Can feel formulaic if overused
Readers recognise the shape	Some topics need hybrids
Easier planning and reporting	Needs a maintained list

10.6 Where freshers go wrong in Chapter 10 topics

Mistake	Why it happens	Avoid it by	Fix it by
Writing a sales page for an informational keyword	Wanting conversions	Intent table	Changing the type
Two posts targeting "semaglutide dosage"	No keyword check	One-keyword rule	Merging or re-targeting
No CTA decided	Brief incomplete	Brief fields	Asking the team lead

10.7 Thumb rules

👍 One page, one type, one unique keyword.

👍 The searcher's words tell you the type.

👍 Incomplete brief? Ask, don't guess.

10.8 Notes to remember

📝 38 content types in five groups: money, education, trust, support, short copy.

📝 No unique keyword = don't create the page.

Chapter 10 Glossary

Term	Simple meaning
Content type	The kind of content (guide, FAQ, comparison...)
Cannibalisation	Two of our pages competing for the same keyword
Pillar page	A central hub page for a big topic
Microcopy	Tiny bits of interface text (buttons, errors)
Primary keyword	The main search phrase a page targets

Chapter 10 Quiz

1. What's the one-keyword rule?

- A) Share keywords between pages
- B) Use one keyword 100 times
- C) Every piece needs its own unique primary keyword
- D) Use no keywords

2. What is cannibalisation?

- A) A design style
- B) A recipe type
- C) Our pages competing for the same keyword

- D) A legal term

3. Which type fits "semaglutide vs tirzepatide"?

- A) Troubleshooting
- B) Microcopy
- C) Policy page
- D) Comparison article

4. Which type is a central hub for a major topic?

- A) Alt text
- B) News article
- C) Pillar page
- D) Button text

5. What should you do if the brief has no CTA?

- A) Ask the team lead
- B) Skip it
- C) Copy a competitor's
- D) Invent "Buy now"

6. Which group does "profile page" belong to?

- A) Money pages
- B) Short copy
- C) Support
- D) Authority and trust

7. Which type fits "compounded semaglutide price"?

- A) Cost / pricing page
- B) Recipe
- C) Glossary
- D) Interview

8. How many content types are listed?

- A) 38
- B) 5
- C) 100
- D) 12

9. What did Ro's nausea guide include?

- A) No sources
- B) Only prices

- C) Authors, MD reviewer, key takeaways, tips, FAQs, references
- D) Only a video

10. In the order-ticket analogy, an incomplete brief means…

- A) Ask, don't guess
- B) Close the kitchen
- C) Cook anything
- D) Double the order

Sources

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026)
2. Ongo Weight Loss, blog listing, viewed 30 October 2026
3. Ongo Weight Loss, content-type list (team guidelines), October 2026 (internal)

Answer key, Chapter 10: 1-C · 2-C · 3-D · 4-C · 5-A · 6-D · 7-A · 8-A · 9-C · 10-A

LEVEL 2 · CHAPTER 11

Chapter 11: Search Intent and the Funnel

The same topic can be searched by someone just curious, someone comparing options, or someone ready to start. Each one needs a different page, a different tone and a different CTA.

> 💡 **Trivia**
>
> Google's rater guidelines measure two separate things: **Page Quality** and **Needs Met**. A beautifully written page that answers the wrong intent still fails Needs Met. [1] 🟢 Official (Google)
>
> Intent is the bridge between a good page and the right page.

11.1 The four kinds of search intent

Intent	The searcher wants to…	Example query	Best content
Informational	Learn	"how does semaglutide work"	Guide, definition, pillar
Commercial investigation	Compare before deciding	"compounded semaglutide vs brand"	Comparison, cost page, reviews
Transactional	Do or start something	"online weight loss program"	Service, landing, eligibility
Navigational	Find a specific brand/page	"ongo weight loss login"	Portal/help pages

How to check intent: search the keyword in an incognito window and look at the top 10 results. If they're all guides, Google thinks the intent is informational. 🔵

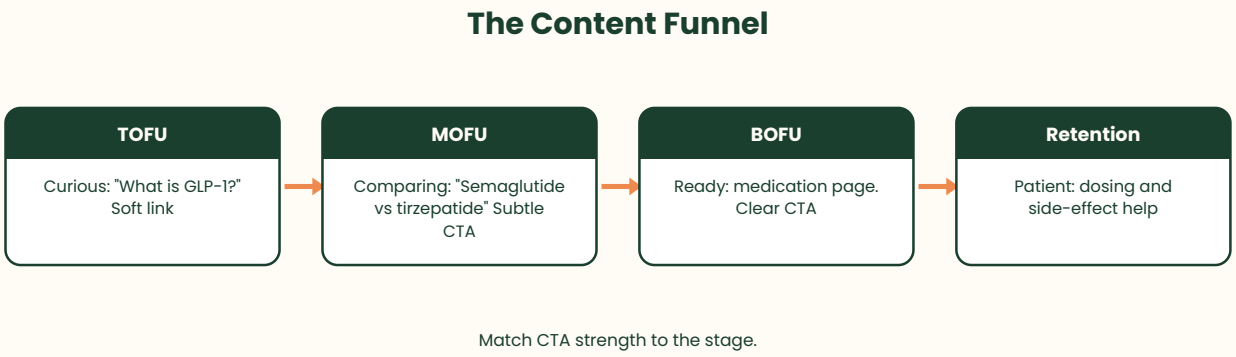
11.2 The funnel: TOFU, MOFU, BOFU

Stage	Reader mindset	Content	CTA strength
TOFU (top)	"I'm curious / I have a problem"	What is GLP-1? Why weight is hard to lose	**Soft**, informational; no brand push. Link to a related deeper article

Stage	Reader mindset	Content	CTA strength
MOFU (middle)	"I'm comparing / thinking"	Semaglutide vs tirzepatide; cost explained; side effects	**Subtle** brand mention + one gentle CTA ("See how our program works")
BOFU (bottom)	"I'm ready to act"	Service, medication, pricing, how it works	**Clear** CTAs (2–3): "Check my eligibility"
Re-MOFU / retention	"I'm a patient"	Dosing help, side-effect tips, plateaus	Support CTA: "Message your provider"

These CTA rules come from our team guidelines. 🔵

> 🪔 **Desi analogy: shaadi ka rishta**
>
> Pehli mulaqat mein koi "shaadi karoge?" nahi poochta (TOFU). Pehle baat-cheet, phir family milna (MOFU), phir rishta pakka (BOFU). **Pehli mulaqat mein proposal = TOFU page pe "Buy now" button.**



Match CTA strength to the stage.

11.3 Funnel path through the site (from Book Three)

```
TOFU: "What is GLP-1?"  →  MOFU: "Semaglutide vs tirzepatide"  →  BOFU: Compounded semaglutide page
→  CTA: Check my eligibility
```

Each step links to the next with a clear, descriptive anchor. Book Four's `internal_link_click` and `cta_[pagegroup]_[action]` events measure these moves. 🔵

11.4 Intent mistakes and fixes

Mismatch	What happens	Fix
Sales copy on an informational query	Reader leaves; low trust	Educate first; soft link
Pure education on a transactional query	Reader can't find how to start	Clear steps, price, CTA

Mismatch	What happens	Fix
Answering a general question for a specific query	Fails Needs Met	Answer the specific question first
Two intents on one page	Neither served well	Split into two pages

> 🟧 **Ongo Weight Loss example**
>
> "How Semaglutide Works for Weight Loss: Benefits, Dosage, Side Effects & More" (our blog) tries to serve several intents in one title. Better: a TOFU explainer ("How semaglutide works"), a MOFU side-effects guide, and a BOFU medication page, each with its own keyword and links between them. 🔵

11.5 Do's and Don'ts

✅ Do	❌ Don't
Check the top 10 results before writing	Assume intent from the keyword alone
Match CTA strength to funnel stage	Put hard CTAs on TOFU pages
Link each stage to the next	Leave readers at a dead end
Split pages with mixed intent	Cram everything into one page

11.6 Pros and cons of funnel planning

✅ Pros	❌ Cons
Right message at the right time	Needs a content map
Measurable journeys	More pages to maintain
Less "salesy" feel	CTA rules need discipline

11.7 Where freshers go wrong in Chapter 11 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Start today!" banner on a TOFU article	Conversion pressure	Funnel CTA table	Softening
BOFU page with no price or steps	Writing like a blog	Intent check	Adding steps and pricing

Mistake	Why it happens	Avoid it by	Fix it by
No internal link to the next stage	Forgetting the path	Funnel diagram	Adding the link

11.8 Thumb rules

👍 Search it first; Google shows you the intent.

👍 Soft at the top, clear at the bottom.

👍 Every page points to the next sensible step.

11.9 Notes to remember

📝 TOFU: no brand push. MOFU: subtle. BOFU: 2–3 clear CTAs.

📝 One page, one dominant intent.

Chapter 11 Glossary

Term	Simple meaning
Search intent	The reason behind a search
TOFU / MOFU / BOFU	Top, middle and bottom of the funnel
Transactional intent	Wanting to do or start something
Commercial investigation	Comparing before deciding
SERP	Search engine results page

Chapter 11 Quiz

1. Which intent is "how does semaglutide work"?

- A) None
- B) Transactional
- C) Navigational
- D) Informational

2. How do you check intent?

- A) Guess
- B) Count letters
- C) Search the keyword and look at the top 10 results
- D) Ask a friend

3. What CTA strength suits TOFU?

- A) Hard "Buy now"
- B) Three pop-ups
- C) None ever
- D) Soft, informational

4. How many clear CTAs can BOFU pages have?

- A) 2–3
- B) 1 hidden
- C) 20
- D) 0

5. What does a mixed-intent page usually need?

- A) Splitting into separate pages
- B) Nothing
- C) Bigger fonts
- D) More keywords

6. Which intent is "compounded semaglutide vs brand"?

- A) Commercial investigation
- B) None
- C) Informational only
- D) Navigational

7. Which Book Four event measures moves between pages?

- A) `scroll`
- B) `internal_link_click`
- C) `page_not_found`
- D) `video_start`

8. Which stage is for current patients?

- A) Re-MOFU / retention
- B) Pre-TOFU
- C) TOFU

- D) None

9. What fails Needs Met?

- A) Answering a general question for a specific query
- B) Adding a source
- C) Answering the exact question
- D) Using headings

10. In the rishta analogy, a hard CTA on TOFU is like...

- A) Proposing at the first meeting
- B) A wedding card
- C) A polite hello
- D) Meeting the family

Sources

1. Google, Search Quality Rater Guidelines (September 11, 2025) 🟢
2. Ongo Weight Loss, Book Three (content map) and Book Four (events), internal 🔵

Answer key, Chapter 11: 1-D · 2-C · 3-D · 4-A · 5-A · 6-A · 7-B · 8-A · 9-A · 10-A

LEVEL 2 · CHAPTER 12

Chapter 12: Keyword and Question Research for Writers

Keywords are the words real people type. Questions are what they really want to know. Your job is to find both, then answer them better than anyone else.

> 💡 **Trivia**
>
> Google's John Mueller has said there's no such thing as "LSI keywords" in how Google works (2019). 🟡 Google spokesperson
>
> Many old guides still say "add LSI keywords". What actually helps is covering the **related questions and terms** a complete answer naturally includes.

12.1 Keyword vocabulary in plain English

Term	Meaning	Example
Primary keyword	The one main phrase the page targets	semaglutide nausea
Secondary keywords	Close variations	how long does semaglutide nausea last
Related terms	Words a full answer naturally uses	gastric emptying, ginger, small meals, dehydration
Long-tail keyword	A longer, specific phrase	foods to avoid on semaglutide for nausea
Search volume (SV)	Average monthly searches	2,400
Keyword difficulty (KD)	How hard it is to rank (tool estimate)	35/100
Question keyword	A query phrased as a question	can you drink coffee on semaglutide

12.2 Where to find keywords and questions (free first)

Source	What you get	Cost
Google autocomplete	What people type	Free
People Also Ask (PAA) boxes	Related questions	Free

Source	What you get	Cost
Google Search Console (Book Four)	Queries already reaching our pages	Free
Google Trends	Rising topics, seasonality	Free
AlsoAsked / AnswerThePublic	Question maps	Free tiers
YouTube and TikTok search bars	How people phrase things in video search	Free
Reddit and forums (read only)	Real worries in real words	Free
Our comment and support logs	Questions our patients actually ask	Free (internal)
Semrush / Ahrefs (via SEO team)	SV, KD, competitor keywords	Paid

12.3 The keyword table (attach to every draft)

Keyword	Type	SV	KD	Intent	Where used
semaglutide nausea	Primary	(from tool)	(from tool)	Info	H1, intro, meta title, slug
how long does semaglutide nausea last	Secondary			Info	H2 + answer block
what helps nausea on semaglutide	Secondary			Info	H2
foods to avoid on semaglutide	Long-tail			Info	H3 / list

Get the primary and secondary keywords from the SEO team; add your own research; note SV and KD from the tool. 🔵

> 🪔 **Desi analogy: mohalle ki chai ki dukaan**
>
> Chai ki dukaan pe baith ke suno: log asli mein kya pooch rahe hain? "Bhai, sasta kahan milega?" "Side effect toh nahi?" **Keyword research yahi sunna hai:** apne shabd nahi, logon ke shabd.

12.4 Where to place keywords (naturally)

Place	Rule
H1 / title	Primary keyword, readable
Meta title and slug	Primary keyword (Book Four, Chapters 5–6)
First 100 words	Primary keyword once, in a natural sentence
H2s	Secondary keywords as **questions** where natural

Place	Rule
Body	Related terms where they help explanation
Image alt text	Only if it describes the image (Chapter 36)
FAQs	Question keywords

Never: repeat the keyword unnaturally, stuff it in bold everywhere, or add keywords "because a tool said so". Read it aloud: would a person say it this way? 🔵

12.5 Turning questions into structure

Question found	Becomes
How long does it last?	H2 + 40–60-word answer block
What helps?	H2 + numbered tips
When should I worry?	H2 + red-flag list + "call your provider"
Can I eat X?	FAQ item

> 🟧 **Ongo Weight Loss example**
>
> Ro ranks with titles built from real questions: "Semaglutide nausea: causes, duration, and tips for relief" covers three questions in one clean title. Our own titles, like "Semaglutide Dosing Guide: Vial Sizes, Weekly Doses, Titration & How to Measure Your Dose", list keywords rather than a reader's question. A cleaner option: "How to measure your compounded semaglutide dose". [1] 🟠

12.6 Do's and Don'ts

✅ Do	❌ Don't
Collect questions from real people	Rely only on tool lists
Use one primary keyword per page	Target five unrelated keywords
Place keywords where they read naturally	Stuff or over-bold keywords
Attach the keyword table	Hide your research

12.7 Pros and cons of question-led research

✅ Pros	❌ Cons
Pages match how people search	Takes longer than copying a list
Good for People Also Ask and AI answers	Some questions are low volume
Content ideas never run out	Needs regular refresh

12.8 Where freshers go wrong in Chapter 12 topics

Mistake	Why it happens	Avoid it by	Fix it by
Keyword in every sentence	Old SEO advice	Read-aloud test	Removing repeats
"LSI keywords" list pasted in	Outdated guides	Related-terms approach	Covering questions instead
Missing GSC queries	Not knowing it exists	Ask SEO for access	Adding real queries

12.9 Thumb rules

👍 Listen first: people's words, not ours.

👍 One primary keyword, many real questions.

👍 If a keyword sounds forced, rewrite the sentence.

12.10 Notes to remember

📝 Keyword table (keyword, type, SV, KD, intent, placement) goes with every draft.

📝 Primary keyword: H1, first 100 words, meta title, slug.

Chapter 12 Glossary

Term	Simple meaning
Search volume (SV)	How many times a phrase is searched per month

Term	Simple meaning
Keyword difficulty (KD)	A tool's estimate of ranking difficulty
Long-tail keyword	A longer, more specific search phrase
People Also Ask (PAA)	Google's box of related questions
Keyword stuffing	Overusing keywords unnaturally

Chapter 12 Quiz

1. What has Google said about "LSI keywords"?

- A) They're required
- B) There's no such thing in how Google works
- C) They're the top ranking factor
- D) Use 50 per page

2. Where should the primary keyword go?

- A) Only in the footer
- B) H1, first 100 words, meta title and slug
- C) Hidden text
- D) Every sentence

3. Which source shows queries already reaching our pages?

- A) A dictionary
- B) Instagram filters
- C) Google Search Console
- D) Canva

4. What is a long-tail keyword?

- A) A longer, more specific phrase
- B) A brand name
- C) A one-word keyword
- D) An image

5. How should secondary keywords appear in H2s?

- A) Repeated three times
- B) As natural questions where possible
- C) In capital letters
- D) Not at all

6. What is keyword stuffing?

- A) Adding sources
- B) Using headings
- C) Overusing keywords unnaturally
- D) Writing FAQs

7. Which free tool shows rising topics?

- A) Google Trends
- B) Photoshop
- C) A calculator
- D) Excel only

8. What goes with every draft?

- A) Nothing
- B) A password list
- C) Competitor's text
- D) The keyword table

9. Which Ro title shows question-led writing?

- A) "Weight loss stuff"
- B) "Semaglutide nausea: causes, duration, and tips for relief"
- C) "Page 1"
- D) "Click here"

10. In the chai-dukaan analogy, keyword research is...

- A) Talking loudly
- B) Closing early
- C) Selling more tea
- D) Listening to what people actually ask

Sources

1. Ro, Health Guide, viewed 30 October 2026 🟠
2. Ongo Weight Loss, blog listing, viewed 30 October 2026 🔵
3. Ongo Weight Loss, writing guidelines (keyword table), internal 🔵

Answer key, Chapter 12: 1-B · 2-B · 3-C · 4-A · 5-B · 6-C · 7-A · 8-D · 9-B · 10-D

LEVEL 2 · CHAPTER 13

Chapter 13: Researching a Health Topic: The Evidence Ladder

Where a fact comes from decides whether you can use it. This chapter gives you a simple ladder: climb as high as you can, and never stand on the bottom rungs.

> 💡 **Trivia**
>
> Ro's semaglutide nausea guide lists 20+ references: mostly peer-reviewed journal articles, plus StatPearls entries, FDA prescribing labels and a MedlinePlus page. [1] 🟠 Industry
>
> The leaders in our category cite medical literature and regulators, not other brands' blogs.

13.1 The evidence ladder

Rung	Source type	Examples	Use for
1 (best)	Regulators and official labels	FDA (drug labels, safety communications), CDC, NIH, HHS	Approval status, safety warnings, official statistics
2	Peer-reviewed research	PubMed-indexed trials and reviews (NEJM, JAMA, Lancet, Obesity)	Effectiveness, side-effect rates
3	Clinical guidelines and medical societies	Obesity Medicine Association, The Obesity Society, ADA, Endocrine Society	Standards of care
4	Trusted medical references	MedlinePlus, Mayo Clinic, Cleveland Clinic, StatPearls	Plain-language background
5	Reputable surveys and data	KFF, Pew, government surveys	Behaviour, cost, use statistics
6 (caution)	Quality news, law-firm analyses	Major newspapers, health news, legal updates	News context; confirm with rung 1–3
❌ **Never**	Competitor blogs, forums, social posts, AI output, content farms	—	Ideas only; never as evidence

Domain myths: ".gov" and ".org" aren't automatically trustworthy, and some great sources are ".com" (NEJM, Mayo Clinic). Judge the **source**, not the ending. 🔵

> **Desi analogy: rishta pakka karne se pehle**
>
> Rishta tay karne se pehle log padosi ki gossip pe nahi, seedha family se, documents se, purane jaan-pehchaan se confirm karte hain. **Facts ke liye bhi gossip (forums) nahi, original source.**

The Evidence Ladder



Climb as high as you can; never stand on the bottom rung.

13.2 How to search like a researcher

Task	Where and how
Find a trial	PubMed: search "semaglutide obesity randomized"; filter "Clinical Trial"
Check a drug fact	FDA label (Drugs@FDA or DailyMed); search the generic name
Get plain background	MedlinePlus drug information
Find a statistic	CDC, KFF, Pew; note the date and sample
Check news	Read the original FDA/agency statement the news cites

13.3 The 30-second source check

Question	Pass if…
Who wrote it?	Named experts or official body
Is it the original?	You're reading the study/agency, not a summary of a summary
When?	Current enough for the topic (drug rules change fast)
About what product?	Brand vs compounded is clear
Does it say what I'm claiming?	Your sentence matches the source exactly

13.4 Citing in the draft

Rule	Example
Link the exact page, not a homepage	`https://doi.org/10.1056/NEJMoa2032183`
Put the claim and source together in the claim log (Chapter 16)	"14.9% (STEP 1, 68 weeks)" → link
Use descriptive anchor text	"the STEP 1 trial", not "click here"
Date-stamp facts that change	"As of October 2026, …"
Never link competitors as sources	Learn from them; cite the original evidence

> 🟧 **Ongo Weight Loss example**
>
> Our homepage cites a CDC obesity statistic in a footnote: a good habit. The CDC page currently reports **41.9%** for 2017–March 2020; when you use a number, copy it from the live source and add the period, so the page and the source always match. [2] 🟢

13.5 Do's and Don'ts

✅ Do	❌ Don't
Climb to the highest rung available	Use the first Google result
Read the original source	Trust a summary's numbers
Note dates and products	Mix brand and compounded facts
Ask the reviewer when sources disagree	Pick the source you prefer

13.6 Pros and cons of strict sourcing

✅ Pros	❌ Cons
Accurate, defensible pages	Slower research
Easier medical review	Some topics have little evidence; say so
Builds expertise signals	Paywalled studies need abstracts or reviewer help

13.7 Where freshers go wrong in Chapter 13 topics

Mistake	Why it happens	Avoid it by	Fix it by
Citing a competitor's blog	It ranked first	Ladder "Never" rung	Finding the original study
Old statistic	First result was outdated	Date check	Replacing with current data
Wrong product in the fact	Brand/compounded confusion	30-second check	Correcting and flagging

13.8 Thumb rules

👍 Highest rung you can reach; never the bottom.

👍 Original source or no claim.

👍 Judge the source, not the domain ending.

13.9 Notes to remember

📝 Rung 1: FDA, CDC, NIH. Rung 2: peer-reviewed trials.

📝 Competitor content is for ideas, never evidence.

Chapter 13 Glossary

Term	Simple meaning
Primary source	The original study, label or agency statement
Secondary source	A summary of a primary source
Peer-reviewed	Checked by independent experts before publishing
PubMed	NIH's free database of medical research
Prescribing label	Official FDA-approved drug information

Chapter 13 Quiz

1. What's on the top rung of the evidence ladder?

- A) AI output
- B) Forums
- C) Competitor blogs
- D) Regulators and official labels (FDA, CDC, NIH)

2. Can competitor blogs be cited as evidence?

- A) If they're long
- B) If they rank first
- C) No; ideas only
- D) Yes

3. Are ".gov" and ".org" automatically trustworthy?

- A) No; judge the source
- B) Only on Fridays
- C) Only ".org"
- D) Yes, always

4. Where do you find clinical trials?

- A) PubMed
- B) Instagram
- C) A recipe site
- D) A shopping site

5. What should anchor text be?

- A) Descriptive, e.g., "the STEP 1 trial"

- B) "Click here"
- C) A URL only
- D) Empty

6. What's checked in the 30-second source check?

- A) Number of ads
- B) Font and colour
- C) Who, original, when, which product, matches claim
- D) Logo size

7. What does "peer-reviewed" mean?

- A) Written by a competitor
- B) Free to read
- C) Checked by independent experts before publishing
- D) Liked by peers on social

8. What should you add to facts that change?

- A) A date ("As of October 2026")
- B) Bold text only
- C) Nothing
- D) An emoji

9. Which source type gives plain-language background?

- A) MedlinePlus or Mayo Clinic
- B) Memes
- C) Competitor ads
- D) Forums

10. In the rishta analogy, facts are confirmed from...

- A) The original source, not gossip
- B) The neighbour's gossip
- C) Social media comments
- D) A rumour

Sources

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026) ●
2. CDC, Adult Obesity Facts ●
3. Wilding et al., STEP 1, *NEJM* (2021) ●

Answer key, Chapter 13: 1-D · 2-C · 3-A · 4-A · 5-A · 6-C · 7-C · 8-A · 9-A · 10-A

LEVEL 2 · CHAPTER 14

Chapter 14: How Competitors Write: Best Pages, Blogs and Why They Work

We read the homepages, product pages and blogs of leading weight-care brands on 30 October 2026. Here is how each one writes, what works, what to avoid, and what Ongo Weight Loss should take from it.

> 💡 **Trivia**
>
> Found describes its blog as "Trusted information about medical weight loss, reviewed by obesity medicine specialists," and publishes editorial standards: health journalists, people-first language, fact-checking against peer-reviewed literature, and doctor review. [1] ● Industry
>
> The strongest competitors make their **process** visible, not just their products.

14.1 Competitor writing at a glance

Brand	Headline example	Voice	Strengths	Weaknesses
Ro	"Get access to prescription weight loss medication online"	Confident, direct	Answer-first guides; MD reviewers; key takeaways; 20+ references; paid testimonials labelled	CTA repetition; brand-name heavy
Hims	"Weight loss treatment for where you are now"	Warm, shame-free	"It's not you, it's biology."; simple steps	Vague medication wording; no prices
Found	Blog: "Zepbound vs Ozempic: Which..."	Friendly, informational	Editorial standards; RN author, fact-checker, editor; peer-reviewed sources	Older article dates; no tables; a reference error found
WeightWatchers	"How often should you weigh yourself?"	Practical, coach-like	Recipes and everyday questions; huge archive	Listing shows no bylines; unfootnoted "#1" claim
Noom	"5 ways to spot a quick fix—and what to do instead"	Supportive, psychology-led	Myth-busting; behaviour focus; fresh dates	Listing shows no reviewer signals

Brand	Headline example	Voice	Strengths	Weaknesses
Mochi	"Weight loss medication, prescribed online."	Simple, clinical	Compounded clearly labelled; FDA disclaimer near products	Promo banners and urgency
Fridays	"A journey that feels like Fridays"	Upbeat, premium	Disclaimers define their stats	Big stat in headline, qualifier only in fine print
Calibrate	"It's About Biology Not Willpower"	Science-confident	Results report linked; metabolic framing	"Safest and most effective" claim; no body disclaimers
IVIM	"Not a bot. Not a one-time consult."	Punchy, personal	Contrast lines; outcome footnotes ("Not randomized controlled trials")	No compounding disclaimer on homepage
Henry Meds	"The Solution That Finally Works!"	Hype	Disclaimers exist in FAQ	"Same ingredient as" cards; unqualified savings claim
Mayo Clinic (benchmark)	"Semaglutide (subcutaneous route)"	Neutral, plain	Clear sections; side effects by frequency; dated	Too dry for marketing pages

[1–11]

14.2 Best-practice page: Ro's semaglutide nausea guide

Element	What they did	Why it works
Title	"Semaglutide nausea: causes, duration, and tips for relief"	Three real questions in one line
Trust box	2 authors + MD reviewer + updated date	E-E-A-T visible at the top
Key takeaways	4 bullets before the article	Skimmers get the answer in 10 seconds
Intro	"Semaglutide... commonly causes nausea. This is likely because semaglutide slows gastric emptying..."	Inverted pyramid: answer, then why
Body	Why it happens → 10 numbered tips → how long it lasts	WHY → HOW → WHAT NEXT
Safety	Red-flag symptom list	Protects readers; YMYL trust
End	Bottom line, FAQs, references, "Why trust our experts?"	Closure + proof

Element	What they did	Why it works
Information gain	Incidence range, titration timelines, symptom diary idea	More useful than a generic list

Take for Ongo Weight Loss: the structure. **Leave:** brand-drug framing and heavy CTAs. [2]

14.3 Best-practice blog: Found and Noom

What	Found	Noom
Title style	Questions and "X vs Y"	Sentence case, "what to do instead"
Topic mix	Medication, metabolism, women's health, insurance, success stories	Recipes, nutrition, well-being, menopause, myths
Trust	Editorial standards note	Fresh dates
Lesson	Show the process	Answer the behaviour questions, not just the drug questions

14.4 What we learn (and won't copy)

✅ Learn	❌ Won't copy
Answer-first intros and key-takeaway boxes (Ro)	"Same ingredient as [brand]" (Henry)
Shame-free framing (Hims, Calibrate, Ro)	Big outcome stats with fine-print qualifiers (Fridays)
Visible editorial process (Found)	"Safest and most effective" style superlatives (Calibrate)
Clear compounded labels next to products (Mochi)	Urgency banners and countdowns (Mochi, promos)
Defining what data is and isn't (IVIM, Fridays)	Missing compounding disclaimers (IVIM homepage)
Everyday, practical questions (WW, Noom)	Unsourced "#1" claims (WW)
Clinical structure for safety info (Mayo Clinic)	Competitor text, ever

> 🪔 **Desi analogy: cricket ka net practice**
>
> Achha batsman doosre teams ke bowlers ki video dekhta hai: kaun kya karta hai, kahan galti hoti hai. Lekin woh unka bat churaa ke nahi khelta. **Competitors ko dekho, seekho, lekin apna khel khelo.**

14.5 How to run your own mini-audit (for any topic)

Step	Action
1	Search your keyword; open the top 5 results in incognito
2	Note: title, intro first two sentences, H2 list, trust box, sources, length, CTAs
3	Mark what's missing in all of them (your information gain, Chapter 26)
4	Note risky claims you must not copy
5	Write your outline, not their text

> 🟧 **Ongo Weight Loss example**
>
> Compared with these competitors, our blog (5 posts, no reviewer names or dates, a noindexed listing page on 30 October 2026) is far behind on volume and trust signals. Our homepage voice is warmer than Henry's and more honest about pricing than many. The fastest wins: add reviewer boxes and dates, write answer-first intros, and publish the missing content types from Chapter 10. 🔵

14.6 Do's and Don'ts

✅ Do	❌ Don't
Study structure and gaps	Copy sentences or headings
Note risky claims as warnings	Assume competitors are compliant
Re-audit every quarter	Rely on one old audit
Keep audit notes with the brief	Link competitors as sources

14.7 Pros and cons of competitor audits

✅ Pros	❌ Cons
Shows what readers already expect	Tempting to copy
Reveals gaps to win	Competitors may be wrong
Speeds up outlining	Pages change often

14.8 Where freshers go wrong in Chapter 14 topics

Mistake	Why it happens	Avoid it by	Fix it by
Rewording a competitor article	Saves time	Outline-not-text rule	Starting from your outline
Copying "same ingredient as"	Looks common	"Won't copy" table	Compliance check
Trusting a competitor's stat	It's on a big site	Evidence ladder	Finding the original

14.9 Thumb rules

👍 Steal the structure, never the sentences.

👍 Their risky claims are your warning list.

👍 Find what all five miss, then write that.

14.10 Notes to remember

📝 Best structure: Ro's guide. Best process: Found's editorial standards. Best tone: Hims.

📝 Re-audit competitors every quarter.

Chapter 14 Glossary

Term	Simple meaning
Competitor audit	A structured review of competitors' content
Key takeaways box	A short summary near the top of an article
Trust box	Author, reviewer and date information
Benchmark	A standard to compare against
Gap	Something useful that no competitor covers well

Chapter 14 Quiz

1. Which competitor's guide shows the best structure?

- A) Henry Meds' homepage
- B) Ro's semaglutide nausea guide
- C) A forum post
- D) None

2. What does Found publish to show its process?

- A) Only prices
- B) Nothing
- C) Editorial standards
- D) Only videos

3. Which line should we never copy?

- A) "Not randomized controlled trials."
- B) "Same ingredient as [brand]"
- C) "It's not you, it's biology."
- D) "Individual results vary."

4. What does a key-takeaways box do?

- A) Gives skimmers the answer quickly
- B) Hides the answer
- C) Replaces sources
- D) Adds ads

5. Which brand's voice is the shame-free example?

- A) A content farm
- B) None
- C) Henry Meds
- D) Hims

6. What's step 3 of a mini-audit?

- A) Copy the best intro
- B) Mark what's missing in all of them
- C) Delete your notes
- D) Link them as sources

7. Which is a weakness seen at Fridays?

- A) No headline
- B) Too many sources
- C) Too many reviewers
- D) A big stat in the headline with its qualifier only in fine print

8. How often should competitors be re-audited?

- A) Every quarter
- B) Hourly
- C) Never
- D) Every 10 years

9. What did Mayo Clinic do well?

- A) Celebrity endorsements
- B) Clear sections; side effects grouped by frequency
- C) Countdown timers
- D) Big promotions

10. In the net-practice analogy, you…

- A) Copy their score
- B) Steal their bat
- C) Study opponents but play your own game
- D) Skip practice

Sources

1. Found, Weight loss news and Zepbound vs Ozempic, viewed 30 October 2026
2. Ro, Weight loss and Semaglutide nausea guide, viewed 30 October 2026
3. Hims, Weight loss, viewed 30 October 2026
4. WeightWatchers, Blog, viewed 30 October 2026
5. Noom, Blog, viewed 30 October 2026
6. Mochi Health, homepage, viewed 30 October 2026
7. Fridays, homepage, viewed 30 October 2026
8. Calibrate, homepage, viewed 30 October 2026
9. IVIM Health, homepage, viewed 30 October 2026
10. Henry Meds, Weight management, viewed 30 October 2026
11. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026

Answer key, Chapter 14: 1-B · 2-C · 3-B · 4-A · 5-D · 6-B · 7-D · 8-A · 9-B · 10-C

LEVEL 2 · CHAPTER 15

Chapter 15: Building the Outline and Writing Headings

An outline is the skeleton of your page. If the skeleton is right, the writing almost writes itself. If it's wrong, no amount of good sentences will save it.

> 💡 **Trivia**
>
> Mayo Clinic's semaglutide drug page uses the same sections every time: Description, Before Using, Proper Use, Precautions, Side Effects. Side effects are grouped by how common they are. [1] ● Official (Mayo Clinic)
>
> A predictable outline lets a worried reader jump straight to what they need.

15.1 The outline in five steps

Step	Action	Output
1	Write the purpose statement (Chapter 9)	One sentence
2	List the reader's questions (Chapter 12)	8–15 questions
3	Group and order them WHY → WHAT → HOW → WHAT NEXT	4–8 groups
4	Turn each group into an H2 (often a question)	H2 list
5	Under each H2, note the answer in one line + facts needed	Answer notes + claim log links

15.2 Heading rules (hierarchy)

Rule	Why
One H1 per page: the title	Tells readers and Google the main topic
H2s for main sections; **H3s** for parts of an H2	Clear structure for skimmers and screen readers
Never skip levels (H2 → H4)	Breaks accessibility and logic
Headings describe content; don't use them just for big text	Book Four, Chapter 7

Rule	Why
1–2 answer sentences right after every H2, before any H3	The "answer block" (Chapter 20)

15.3 Label headings vs anticipatory headings

❌ Label heading	✅ Anticipatory (question) heading
Side effects	What side effects can semaglutide cause?
Duration	How long does semaglutide nausea last?
Cost	How much does compounded semaglutide cost at Ongo Weight Loss?
Eligibility	Who can get a prescription through Ongo Weight Loss?

Question headings match how people search, help People Also Ask and AI answers, and make the page feel like it's reading the reader's mind. Use them for most H2s, but not every heading has to be a question. 🔵

> 🪔 **Desi analogy: shaadi ka card**
>
> Shaadi ke card pe saaf likha hota hai: Mehendi, Sangeet, Shaadi, Reception: date, time, jagah. Mehmaan seedha apne function pe pahunch jaata hai. **Achhe headings shaadi ke card jaise hain:** reader seedha apne sawaal pe pahunch jaaye.

15.4 Sample outline (side-effect guide)

```
H1  Semaglutide nausea: why it happens, how long it lasts, and what helps
    [Trust box: author · medical reviewer · last medically reviewed date]
    [Key takeaways: 3–5 bullets]
    Intro: answer first (2–3 sentences)
H2  Why does semaglutide cause nausea?
H2  How long does nausea last on semaglutide?
H2  What helps with semaglutide nausea? (numbered tips)
    H3  Eating habits   H3  Foods to try and avoid   H3  Hydration
H2  When should you call your provider? (red flags)
H2  Can nausea be a sign the dose is too high?
H2  The bottom line
H2  Frequently asked questions (4–6)
    Sources · Disclaimer · Related articles
```

15.5 Outline checks

☐	Check
☐	Every H2 serves the purpose statement
☐	Order follows the reader's questions
☐	Safety section exists for any medical topic
☐	Information gain spot is marked (Chapter 26)
☐	Internal links and CTA positions are marked
☐	No two H2s answer the same question

> 🟧 **Ongo Weight Loss example**
>
> Our dosing-guide title lists five items ("Vial Sizes, Weekly Doses, Titration & How to Measure Your Dose"). Those belong as **H2s** inside the page, not all in the H1. A clean H1: "Compounded semaglutide dosing guide", with question H2s such as "How do I measure my dose?" 🔵

15.6 Do's and Don'ts

✅ Do	❌ Don't
Outline before drafting	Write and "see where it goes"
Use question H2s where natural	Use vague labels ("Overview", "More info")
Keep one H1	Use several H1s for style
Get the outline approved for big pages	Rewrite 2,000 words after review

15.7 Pros and cons of detailed outlines

✅ Pros	❌ Cons
Faster drafting, fewer rewrites	Takes 20–40 minutes up front
SEO and reviewer can comment early	Can feel rigid
Clear structure for designers	Must stay flexible if research changes

15.8 Where freshers go wrong in Chapter 15 topics

Mistake	Why it happens	Avoid it by	Fix it by
Headings as labels only	Habit from school essays	15.3 table	Rewriting as questions
Skipping from H2 to H4	Using headings for size	Hierarchy rules	Fixing levels
No safety section	Forgetting YMYL	Outline checks	Adding red flags

15.9 Thumb rules

👍 Skeleton first, skin later.

👍 Headings are the questions; paragraphs are the answers.

👍 One H1, no skipped levels.

15.10 Notes to remember

📝 1–2 answer sentences right after every H2.

📝 Every medical outline needs a "when to call your provider" section.

Chapter 15 Glossary

Term	Simple meaning
Outline	The planned structure of headings and points
H1, H2, H3	Heading levels from main title down
Anticipatory heading	A heading phrased as the reader's question
Answer block	The 1–2 sentences right after a heading that answer it
Hierarchy	The order and nesting of headings

Chapter 15 Quiz

1. How many H1s should a page have?

- A) One
- B) None
- C) One per section
- D) Three

2. Which is an anticipatory heading?

- A) "Overview"
- B) "More info"
- C) "How long does semaglutide nausea last?"
- D) "Duration"

3. What comes right after each H2?

- A) 1–2 answer sentences
- B) An image only
- C) A CTA button
- D) Nothing

4. Can you skip from H2 to H4?

- A) Only in FAQs
- B) No
- C) Only on mobile
- D) Yes

5. What's step 1 of the outline process?

- A) Write the conclusion
- B) Choose fonts
- C) Write the purpose statement
- D) Add images

6. What section must medical outlines include?

- A) Celebrity quotes
- B) Company history
- C) When to call your provider (red flags)
- D) Discounts

7. How did Mayo Clinic organise side effects?

- A) By how common they are
- B) Alphabetically by brand
- C) Randomly
- D) By price

8. What's wrong with listing five items in an H1?

- A) H1s can't have keywords
- B) They belong as H2s inside the page
- C) Nothing
- D) H1s must be one word

9. Should every heading be a question?

- A) Yes, always
- B) Never
- C) No; most H2s can be, but not all
- D) Only H1

10. In the wedding-card analogy, good headings…

- A) Confuse guests
- B) Take readers straight to what they need
- C) Hide the venue
- D) List only the menu

Sources

1. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026 🟢
2. Ongo Weight Loss, Book Four, Chapter 7 (headings), internal 🔵

Answer key, Chapter 15: 1-A · 2-C · 3-A · 4-B · 5-C · 6-C · 7-A · 8-B · 9-C · 10-B

LEVEL 2 · CHAPTER 16

Chapter 16: Fact Sheets and Claim Logs

Professional health writers don't keep facts in their heads. They write every claim and its source in a log before and while drafting. It makes review faster and mistakes rarer.

> 💡 **Trivia**
>
> When we checked Found's "Zepbound vs Ozempic" article (30 October 2026), one reference's link pointed to the wrong journal, and one trial cited in the text wasn't in the reference list. [1] 🟠 Industry
>
> Even professional teams with fact-checkers slip. A claim log catches these before publishing.

16.1 The claim log (one row per claim)

#	Claim (as written)	Type	Source (exact link)	Source says (quote/number)	Date checked	Reviewer note
1	"In STEP 1, adults on brand semaglutide 2.4 mg lost 14.9% on average over 68 weeks."	FACTUAL	doi.org/10.1056/NEJMoa2032183	−14.9% vs −2.4% placebo, 68 wks	2026-10-30	
2	"Nausea is often more noticeable when the dose increases."	CONDITIONAL	(label / review)	…		
3	"Compounded medications are not FDA-approved."	FACTUAL	FDA / internal rule	—		

Rule: if a claim has no source in the log, it doesn't go in the draft. 🔵

> 🪔 **Desi analogy: kirana ki udhaar copy**
>
> Kirana waala har udhaar copy mein likhta hai: kisne, kya, kitna, kab. Mahine ke end mein koi jhagda nahi hota. **Claim log hamari udhaar copy hai:** har fact ka hisaab.

16.2 The fact sheet (topic summary)

For bigger topics, keep a one-page fact sheet you can reuse across articles:

Section	Contents
Topic	Compounded semaglutide
Approved wording	"compounded semaglutide"; "not FDA-approved"; "provider decides"
Key facts	Mechanism (plain); common side effects; titration idea
Numbers	Trial results with product, dose, duration
Red flags	Symptoms needing urgent care (reviewer-approved)
Don't say	"same as", "generic", outcome promises
Sources	Links with dates
Last reviewed	Date + reviewer name

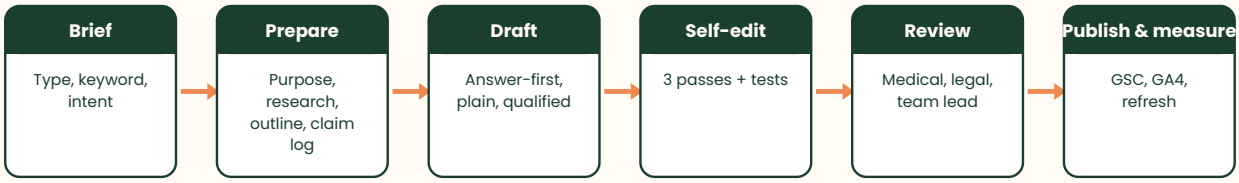
16.3 The submission pack

Every draft you submit includes:

Item	Why
Doc header block (top of the Google Doc): meta title, meta description, slug, primary keyword the page should rank for	SEO and developers copy them exactly
Sources list (every resource you used, linked)	Reviewers can check every fact
Purpose statement	Reviewer knows the goal
Keyword table	SEO can check targeting
Claim log	Medical/legal review is faster
Information gain note	Shows what's new (Chapter 26)

Item	Why
Internal link list	SEO can confirm the funnel path
Image suggestions with alt text	Design can start
Open questions	Anything you weren't sure about

From Brief to Live Page



Every piece, every time.

16.4 VERIFY: the fact-check routine

Letter	Action
Validate	Check each fact against the original source
Examine scope	Is it true for this product, dose and group?
Recheck dates	Is it still true today?
Identify assumptions	What does the sentence imply that the source doesn't say?
Flag uncertainty	Where should "may", "some" or "varies" be added?
Your responsibility	Would you defend this claim publicly?

> 🟧 **Ongo Weight Loss example**
>
> Our homepage says "$44 to $119 less per month" compared with competitors' "lowest listed" prices. A claim like this needs a log row with each competitor's live price, the date checked, and a plan to recheck monthly; otherwise it becomes false the day a competitor changes price. 🔵

16.5 Do's and Don'ts

✅ Do	❌ Don't
Log every claim with a link and date	Keep facts in your head
Quote the exact number from the source	Round or paraphrase numbers loosely
Submit the full pack	Send only the draft
Recheck time-sensitive claims	Assume prices and rules stay the same

16.6 Pros and cons of claim logs

✅ Pros	❌ Cons
Faster medical/legal review	Extra admin
Easy updates later	Needs discipline
Protects you and the company	—

16.7 Where freshers go wrong in Chapter 16 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Studies show..." with no source	Habit	Claim log rule	Adding source or cutting
Number changed while editing	Rewriting freely	Copy exact numbers	Rechecking the log
Price comparison never rechecked	Forgetting	Date column	Monthly check

16.8 Thumb rules

👍 No source in the log, no claim on the page.

👍 Exact numbers, exact products, exact dates.

👍 VERIFY before you submit.

16.9 Notes to remember

📝 Submission pack: purpose, keywords, claim log, info gain, links, images, questions.

📝 Time-sensitive claims need a recheck date.

Chapter 16 Glossary

Term	Simple meaning
Claim log	A table of every claim with its source
Fact sheet	A reusable one-page summary of key facts on a topic
Submission pack	Everything you send with a draft
VERIFY	Our six-step fact-check routine
Scope	Who and what a fact applies to

Chapter 16 Quiz

1. What happens to a claim with no source in the log?

- A) It goes in the title
- B) It doesn't go in the draft
- C) It's fine
- D) It goes in bold

2. What does the "E" in VERIFY stand for?

- A) Examine scope
- B) Email legal
- C) Edit fonts
- D) End early

3. What did we find in Found's article?

- A) A wrong-journal reference link and a missing reference
- B) No references at all
- C) Perfect references
- D) Only videos

4. What goes in the submission pack?

- A) Passwords
- B) Purpose, keywords, claim log, info gain, links, images, questions
- C) Only the draft
- D) A competitor's article

5. How should numbers be copied?

- A) Exactly from the source
- B) Rounded up
- C) From a competitor
- D) From memory

6. What's a fact sheet?

- A) An invoice
- B) A social post
- C) A reusable one-page summary of key facts
- D) A font guide

7. Why does a price comparison need a recheck date?

- A) Competitor prices change
- B) It's decorative
- C) It doesn't
- D) Google requires dates

8. What's the "Y" in VERIFY?

- A) Yes to everything
- B) Your responsibility: would you defend it publicly?
- C) YouTube
- D) Yearly update

9. Which claim type is "Compounded medications are not FDA-approved"?

- A) PROHIBITED
- B) Opinion
- C) CONDITIONAL
- D) FACTUAL

10. In the udhaar-copy analogy, the claim log is...

- A) A shopping list
- B) A menu
- C) A diary of feelings

- D) A record of every fact

Sources

1. Found, Zepbound vs Ozempic, viewed 30 October 2026
2. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (VERIFY), internal

Answer key, Chapter 16: 1-B · 2-A · 3-A · 4-B · 5-A · 6-C · 7-A · 8-B · 9-D · 10-D

LEVEL 2 · CHAPTER 17

Chapter 17: Using AI Safely: Consultant, Not Co-Writer

AI tools can help you research, brainstorm and proofread. They can also invent facts, flatten your voice and leak private data. This chapter shows exactly where AI helps and where it must stop.

> 💡 **Trivia**
>
> Google says AI-generated content fits its long-standing approach of rewarding helpful content (February 2023). But using automation or AI "mainly to manipulate search rankings" violates its spam policies. [1][2]
> ● Official (Google)
>
> Google doesn't ban AI. It bans unhelpful and manipulative content, however it's made.

17.1 What AI is good and bad at

✅ Good at	❌ Bad at
Brainstorming questions readers might ask	Medical accuracy (it can "hallucinate" studies and numbers)
Spotting gaps in your outline	Knowing our compliance rules unless told
Suggesting simpler words	Current prices, rules and FDA updates
Listing grammar errors	Keeping your voice when "improving" text
Summarising a source you provide (then you verify)	Being accountable

17.2 Our AI rules

#	Rule
1	**Never** paste patient information, support tickets or private messages into AI tools (privacy)
2	AI may suggest; **you** write the final sentences
3	Every fact AI gives you is verified in a primary source before it enters the claim log
4	No AI-generated testimonials, patient stories, reviews or doctor quotes, ever (FTC; Book Five, Chapter 30)

#	Rule
5	Don't ask AI to "rewrite", "polish" or "make more engaging"; it replaces your voice
6	Tell your editor which parts AI helped with (Google's "How" question)
7	Never paste competitor articles into AI to "spin" them

> **Desi analogy: tez lekin naya intern**
>
> AI ek tez intern hai: idea jaldi deta hai, lekin kabhi-kabhi poore aatmavishwas se galat baat bol deta hai. **Intern se suggestion lo; client ko jo jaata hai woh aap khud likho aur check karo.**

17.3 Safe prompts (copy these)

Proofreading (consultant mode):

```
Proofread this text for spelling and grammar errors only. List each error and a
suggested fix. Do NOT rewrite, do NOT change my sentence structure, word choices
or flow unless something is grammatically wrong. Keep my voice.
```

Question brainstorming:

```
I'm writing for Ongo Weight Loss, a U.S. telehealth weight-care brand. List 15
questions a 50-60-year-old considering compounded semaglutide might ask about
[topic]. Questions only, no answers.
```

Gap check:

```
Here is my outline. What important reader questions or safety points are missing?
Do not write content; list gaps only.
```

Then: fix errors yourself, research the questions in primary sources, and write. ●

17.4 Why "polish my draft" is a trap

What AI does when it "polishes"	Effect
Replaces specific words with common ones	Text becomes generic and predictable
Evens out sentence length	Rhythm becomes flat and robotic (Chapter 19)
Adds confident filler ("It's important to note...")	Longer, emptier, sometimes less accurate
Changes facts while "clarifying"	Errors slip in unnoticed

About AI detectors: they can be wrong, including on human writing, and Google does not use them as a quality test. Don't write to please a detector; write clearly, specifically and with varied rhythm because that's better for **readers**. 🔵

17.5 Grammar tools

Tool	Use	Avoid
Grammarly / LanguageTool	"Correctness" suggestions, one by one	Accepting "rewrite for clarity/engagement" blocks
Hemingway Editor	Spot long sentences and passive voice	Treating its grade as the only goal
Google Docs spelling	Basic checks	—

🟧 **Ongo Weight Loss example**

Paste our rules into any brainstorming prompt: "Ongo Weight Loss offers compounded semaglutide and tirzepatide; they are not FDA-approved; never name brand drugs as our products; no outcome promises; never name the partner pharmacy." Without these, AI will happily suggest "Get Ozempic results for less", a line we can never use. 🔵

17.6 Do's and Don'ts

✅ Do	❌ Don't
Use AI for questions, gaps and error lists	Use AI to write the final copy
Verify every AI fact in a primary source	Trust AI citations without opening them
Fix errors yourself	Paste AI rewrites over your draft
Disclose AI help to your editor	Paste private data into AI

17.7 Pros and cons of AI for writers

✅ Pros	❌ Cons
Faster research starts	Hallucinated facts
Catches grammar errors	Flattened, generic voice
Finds missing questions	Privacy and legal risks

17.8 Where freshers go wrong in Chapter 17 topics

Mistake	Why it happens	Avoid it by	Fix it by
Accepting a full AI rewrite	It "sounds better"	Consultant prompt	Restoring your draft
Citing an AI-invented study	Trusting output	Rule 3	Removing; finding real evidence
AI testimonial "for illustration"	Thinking it's harmless	Rule 4	Deleting

17.9 Thumb rules

👍 AI suggests; you decide and write.

👍 Open every source AI mentions.

👍 Private data never goes into AI.

17.10 Notes to remember

📝 Google judges helpfulness, not whether AI was used.

📝 AI used mainly to manipulate rankings = spam.

Chapter 17 Glossary

Term	Simple meaning
LLM	Large language model, the technology behind ChatGPT, Claude, Gemini
Hallucination	Confident but false AI output
Prompt	Instructions you give an AI tool
Consultant mode	Using AI to suggest, not to write
AI detector	A tool that guesses if text was AI-written (often unreliable)

Chapter 17 Quiz

1. What is AI's role for our writers?

- A) Legal reviewer
- B) Medical reviewer
- C) Consultant, not co-writer
- D) Final author

2. What does Google say about AI content?

- A) Nothing
- B) AI content always ranks higher
- C) Helpful content is rewarded however it's made; manipulation is spam
- D) All AI content is banned

3. Can AI write patient testimonials?

- A) If labelled small
- B) Yes
- C) Never
- D) For social only

4. What's wrong with "polish my draft"?

- A) It adds sources
- B) AI replaces your voice and can change facts
- C) It's too slow
- D) Nothing

5. What should never be pasted into AI?

- A) Patient information or private messages
- B) Public questions
- C) A grammar question
- D) Your outline

6. What do you do with an AI-suggested fact?

- A) Bold it
- B) Publish it
- C) Verify it in a primary source
- D) Ignore sources

7. Are AI detectors reliable quality tests?

- A) Google requires them
- B) Yes, perfectly

- C) No; they can be wrong and Google doesn't use them
- D) They replace review

8. Which Grammarly suggestions do you accept?

- A) Engagement rewrites
- B) All "rewrite" blocks
- C) None ever
- D) Correctness, one by one

9. What goes into every brainstorming prompt?

- A) Passwords
- B) Competitor text
- C) Our compliance rules
- D) Patient names

10. In the intern analogy, AI is...

- A) Fast but sometimes confidently wrong
- B) Useless
- C) Always right
- D) The boss

Sources

1. Google Search Central Blog, Google Search's guidance about AI-generated content (February 2023) 🟢
2. Google Search Central, Creating helpful, reliable, people-first content 🟢
3. Ongo Weight Loss, AI use rules (writing guidelines), internal 🔵

Answer key, Chapter 17: 1-C · 2-C · 3-C · 4-B · 5-A · 6-C · 7-C · 8-D · 9-C · 10-A

03

LEVEL 3

Writing Techniques

Plain language, rhythm, inverted pyramid, hypophora, the fourth wall, clinical neutrality, persuasion and explanation models, stories, headlines, FAQs and CTAs.

18 · Plain Language: The Foundation Technique

19 · Readability, Rhythm and Sounding Human

20 · The Inverted Pyramid and BLUF: Answer First

21 · Hypophora: Ask the Reader's Question, Then Answer It

22 · Breaking the Fourth Wall and the Conversational "You"

23 · Clinical Neutrality and Hedging

24 · Persuasion Models, Used Ethically

25 · Explanation Models: Making Hard Things Easy

26 · Information Gain: Be 1% More Useful

27 · Storytelling and Patient Stories

28 · Headlines and Titles That Are Honest and Still Get Clicks

29 · Introductions, Key Points and Conclusions

30 · Scannability: Headings, Lists, Tables and Bold

31 · Writing for AI Answers, Snippets and FAQs

32 · CTAs and Microcopy: Buttons, Forms and Errors

LEVEL 3 · CHAPTER 18

Chapter 18: Plain Language: The Foundation Technique

Plain language isn't simple-minded writing. It's writing that a busy, worried person understands the first time they read it. Every other technique in this book sits on top of it.

> 💡 **Trivia**
>
> The U.S. **Plain Writing Act of 2010** requires federal agencies to write public documents clearly for their audience. [1] ● Official (digital.gov)
>
> If the U.S. government has to write plainly, a health brand certainly should.

18.1 The ten plain-language rules

#	Rule	❌ Before	✅ After
1	**Use short, common words**	"utilize", "commence", "prior to"	"use", "start", "before"
2	**Keep sentences short** (average 15–20 words)	One 45-word sentence	Three 15-word sentences
3	**Use active voice**	"Your dose will be reviewed by a provider."	"A provider reviews your dose."
4	**Talk to the reader ("you")**	"Patients should contact their provider."	"Contact your provider."
5	**One idea per paragraph**	Price, side effects and shipping in one block	Three short paragraphs
6	**Put the main point first**	Background, then answer	Answer, then background
7	**Define terms the first time**	"Titration starts low..."	"Titration (raising your dose slowly) starts low..."
8	**Use lists for steps and options**	"First you..., then you..., and then..."	1. 2. 3.
9	**Use numbers people understand**	"0.25 mg/0.5 mL" alone	"0.25 mg (your starting dose)" + the measurement

#	Rule	❌ Before	✅ After
10	**Cut words that don't work**	"In order to", "due to the fact that"	"To", "because"

NIH's guidance agrees: sentences should be "simple, specific, direct, and written in the active voice," and the most important points go first and last. [2] ●

> 🪔 **Desi analogy: GPS waali didi**
>
> GPS kehti hai "200 meter baad left lijiye." Woh nahi kehti "aapke vaahan ko agle chauraahe pe vaam disha ki or mod dena uchit hoga." **Plain language GPS jaisi hai:** chhota, saaf, abhi kaam aane waala.

18.2 Passive voice: how to spot and fix it

Spot it: "is/was/will be/has been + verb (+ by someone)".

Passive	Active
"Medications are shipped in 3–5 days."	"We ship medications in [timeframe as on the site]."
"It was found that nausea decreased."	"In the study, nausea decreased over time."
"Eligibility is determined by a provider."	"A provider decides if you're eligible."

When passive is OK: when the doer doesn't matter or is unknown ("The study was published in 2021"), or in careful legal statements.

18.3 Word swap list (starter)

❌ Instead of	✅ Use
administer	give, take
adverse event	side effect
approximately	about
assist	help
commence	start
discontinue	stop
individuals	people
obtain	get

❌ Instead of	✅ Use
optimal	best (if proven) / right for you
sufficient	enough
subcutaneous injection	an injection under the skin (subcutaneous)

Full list: Appendix C.

18.4 Numbers and risk, plainly

Rule	Example
Use whole numbers and percentages people know	"About 1 in 5 people" alongside "20%"
Give the comparison	"14.9% with semaglutide vs 2.4% with placebo"
Don't mix formats in one sentence	Not "1 in 5 (0.2) or 20%"
Say what the number is about	"...of their body weight, on average, over 68 weeks"

The CDC Clear Communication Index has separate sections for numbers and risk, because they confuse readers most. [3] 🟢

> 🟧 **Ongo Weight Loss example**
>
> ❌ "Eligibility is determined individually, and a prescription is never guaranteed." (homepage) is accurate but passive. ✅ "A provider looks at your health history to decide if treatment is right for you. A prescription isn't guaranteed." Same meaning, easier to read. 🔵

18.5 Do's and Don'ts

✅ Do	❌ Don't
Use everyday words	Use jargon to sound expert
Write in active voice by default	Hide who does what
Define terms once, simply	Assume readers know medical words
Put numbers in context	Drop raw statistics

18.6 Pros and cons of plain language

✅ Pros	❌ Cons
More people understand and act	Hard to write at first
Fewer support questions	Some legal text resists simplification
Better for mobile, voice and AI answers	Experts may want more detail (link to it)

18.7 Where freshers go wrong in Chapter 18 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Utilize" and "facilitate" everywhere	Sounds professional	Swap list	Replacing words
Long, passive sentences	Copying research papers	Rules 2–3	Splitting and activating
Undefined terms	Expert blind spot	Rule 7	Adding short definitions

18.8 Thumb rules

👍 Short words, short sentences, active voice.

👍 If you'd say it differently out loud, write it that way.

👍 Numbers always get context.

18.9 Notes to remember

📝 NIH: simple, specific, direct, active.

📝 Plain Writing Act of 2010: clarity is a public standard.

Chapter 18 Glossary

Term	Simple meaning
Plain language	Writing the audience understands the first time

Term	Simple meaning
Active voice	The doer comes first ("A provider reviews...")
Passive voice	The action comes first ("...is reviewed by...")
Jargon	Specialist words most readers don't know
Plain Writing Act	U.S. law requiring clear government writing (2010)

Chapter 18 Quiz

1. Which is active voice?

- A) "It was reviewed."
- B) "A provider reviews your dose."
- C) "Reviewing was done."
- D) "Your dose will be reviewed."

2. What does NIH recommend?

- A) Passive voice
- B) Jargon
- C) Long academic sentences
- D) Simple, specific, direct, active sentences

3. What's the plain word for "discontinue"?

- A) Abort
- B) Stop
- C) Cease operations
- D) Terminate

4. What's a good average sentence length?

- A) 15–20 words
- B) 100 words
- C) 60 words
- D) 3 words

5. When is passive voice OK?

- A) In every heading
- B) Always
- C) Never
- D) When the doer doesn't matter or is unknown

6. Which law requires clear federal writing?

- A) Tax Code
- B) Plain Writing Act of 2010
- C) HIPAA
- D) Copyright Act

7. How should you present "14.9%"?

- A) Hidden
- B) Rounded to 15% guaranteed
- C) With context and comparison (vs placebo, over 68 weeks)
- D) Alone in bold

8. How many ideas per paragraph?

- A) Five
- B) As many as possible
- C) One
- D) None

9. What do you do the first time you use a medical term?

- A) Avoid explaining
- B) Use it three times
- C) Bold it only
- D) Define it simply

10. In the GPS analogy, plain language is...

- A) Poetic
- B) Silent
- C) Formal and long
- D) Short, clear and useful right now

Sources

1. digital.gov, Plain language guides 🟢
2. NIH, Clear & Simple 🟢
3. CDC, Clear Communication Index 🟢
4. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Answer key, Chapter 18: 1-B · 2-D · 3-B · 4-A · 5-D · 6-B · 7-C · 8-C · 9-D · 10-D

LEVEL 3 · CHAPTER 19

Chapter 19: Readability, Rhythm and Sounding Human

Two drafts can have the same facts and the same reading level, yet one feels warm and alive and the other feels robotic. The difference is rhythm.

> 💡 **Trivia**
>
> NIH's Clear & Simple guide warns that readability formulas "only measure the difficulty of the vocabulary used and average sentence length." [1] 🟢 Official (NIH)
>
> A score tells you if words and sentences are long. It can't tell you if the writing is clear, kind or interesting.

19.1 Readability scores you'll see

Score	What it measures	Target for us
Flesch Reading Ease	0–100; higher = easier	60–70+ for money pages
Flesch-Kincaid Grade	U.S. school grade	6–8
SMOG	Grade, based on long words	8 or below
Hemingway grade	Grade + highlights long/passive sentences	6–8

Use the score as a smoke alarm, not a judge. If it's high, look for long sentences and long words. 🔵

19.2 Rhythm (what some call "burstiness")

Human writing varies. Short sentences hit hard. Longer ones carry detail, explain a reason, or add a gentle aside that makes the reader feel you're talking with them. Then a medium one. Like this.

Flat (same length every time)	Varied rhythm
"Semaglutide can cause nausea. Nausea is common in the first weeks. It often improves over time. Eating smaller meals may help. You should talk to your provider."	"Nausea is common in the first weeks. For many people, it eases as the body adjusts, often within a few weeks, though it can return briefly when the dose goes up. Smaller meals help. So does slowing down. If it doesn't improve, talk to your provider."

How to add rhythm:

Move	Example
Follow a long sentence with a very short one	"...as your dose increases. That's normal."
Use a dash for a thought	"Protein matters — especially when you're eating less."
Use a parenthetical aside	"Ginger (tea, chews or fresh) may help some people."
Vary paragraph length	One-line paragraph after a four-line one
Start sentences differently	Not every sentence with "You" or "Semaglutide"

Why: varied rhythm is easier and nicer to read aloud. That's the reason; not to "beat AI detectors".

> 🪔 **Desi analogy: tabla ki taal**
>
> Tabla pe agar har thaap ek jaisi ho, "dha dha dha dha", toh neend aa jaati hai. Achha tabla vaadak dheema, tez, ruk ke, phir zor se bajata hai. **Sentences bhi taal mein ho:** chhota, lamba, beech ka.

19.3 The read-aloud test

☐	Check
☐	Did you run out of breath? (sentence too long)
☐	Did you stumble? (unclear word order)
☐	Did it sound like a person or a brochure?
☐	Did every sentence start the same way?

☐	Check
☐	Did you hear filler ("It is important to note")?

19.4 Common robotic patterns to remove

Pattern	Example	Fix
Triple adjectives	"a seamless, personalized, comprehensive program"	Pick one specific fact instead
Empty openers	"In today's fast-paced world..."	Start with the answer
Over-signposting	"Let's dive in!", "Without further ado"	Cut
Same-shape lists	Every bullet "Boosts...", "Improves...", "Enhances..."	Vary verbs; add specifics
Vague intensifiers	"very", "really", "truly", "incredibly"	Cut or replace with a fact

> 🟧 **Ongo Weight Loss example**
>
> "Guided. Tailored. Care." (homepage) is three one-word fragments: rhythm without meaning. Rhythm should serve meaning: "Your provider guides you. Your plan fits your health history. And you can message us anytime." 🔵

19.5 Do's and Don'ts

✅ Do	❌ Don't
Read every draft aloud	Trust the score alone
Mix sentence lengths	Write every sentence 15–18 words
Replace adjectives with facts	Stack buzzwords
Vary sentence openings	Start five sentences with "You"

19.6 Pros and cons of rhythm work

✅ Pros	❌ Cons
Warmer, more human writing	Takes an extra editing pass
Readers stay longer	Can be overdone (too many fragments)
Easier to listen to (voice, accessibility)	Must not break clarity

19.7 Where freshers go wrong in Chapter 19 topics

Mistake	Why it happens	Avoid it by	Fix it by
Chasing a grade score with choppy sentences	Over-focusing on numbers	Read-aloud test	Combining some sentences
Too many fragments	Copying ad style	Meaning first	Turning fragments into sentences
Buzzword stacks	Brochure habit	19.4 table	One concrete fact

19.8 Thumb rules

👍 Short, long, medium: let it breathe.

👍 Scores are smoke alarms, ears are judges.

👍 Facts beat adjectives.

19.9 Notes to remember

📝 Target grade 6–8 on money pages.

📝 Rhythm is for readers, not detectors.

Chapter 19 Glossary

Term	Simple meaning
Readability	How easy text is to read
Flesch Reading Ease	A 0–100 score; higher is easier
Rhythm / burstiness	Variation in sentence length and shape
Fragment	An incomplete sentence used for effect
Intensifier	A word like "very" that adds emphasis but little meaning

Chapter 19 Quiz

1. What does NIH say formulas measure?

- A) Search ranking
- B) Truth and accuracy
- C) Kindness
- D) Only vocabulary difficulty and sentence length

2. What's the target grade for money pages?

- A) 2
- B) 14
- C) 18
- D) 6–8

3. What is rhythm in writing?

- A) Using emojis
- B) Varying sentence length and shape
- C) Bold text
- D) Using rhymes

4. Why add rhythm?

- A) To add words
- B) To rank instantly

- C) To trick detectors
- D) It's easier and nicer for readers

5. Which is a robotic pattern?

- A) "Talk to your provider."
- B) "Smaller meals help."
- C) "A seamless, personalized, comprehensive program"
- D) "Nausea is common in the first weeks."

6. What does the read-aloud test catch?

- A) Image size
- B) Long, stumbling or brochure-like sentences
- C) Broken links
- D) Price changes

7. What should replace vague intensifiers?

- A) Capital letters
- B) More intensifiers
- C) Exclamation marks
- D) A specific fact, or nothing

8. Which Flesch Reading Ease range is easier?

- A) Higher (60–70+)
- B) Negative
- C) Exactly 0
- D) Lower (10–20)

9. What's wrong with "Guided. Tailored. Care."?

- A) Nothing
- B) It has too many facts
- C) It's too long
- D) Rhythm without meaning

10. In the tabla analogy, good writing…

- A) Repeats one beat
- B) Varies its beat

- C) Is silent
- D) Is always fast

Sources

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (rhythm), internal 🔵

Answer key, Chapter 19: 1-D · 2-D · 3-B · 4-D · 5-C · 6-B · 7-D · 8-A · 9-D · 10-B

Chapter 20: The Inverted Pyramid and BLUF: Answer First

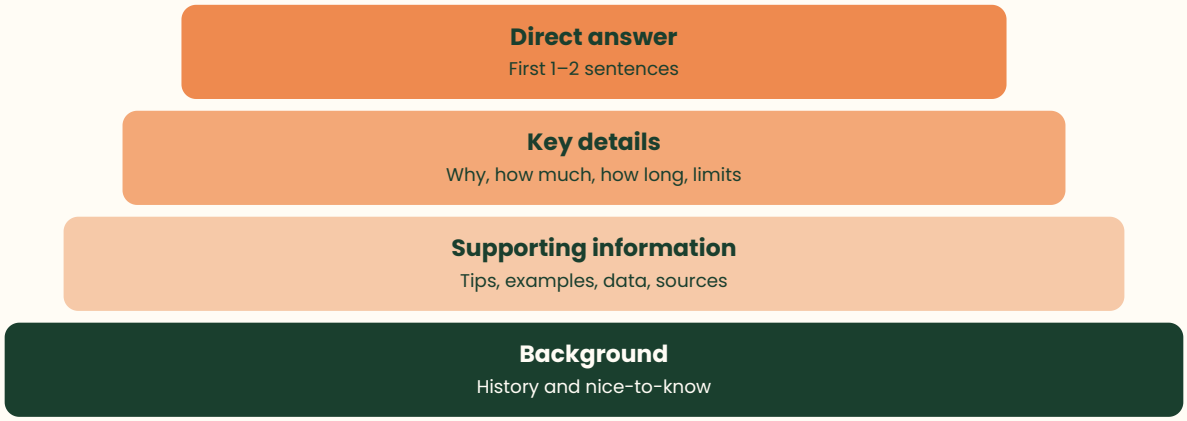
Readers decide in seconds whether a page will help. Give them the answer immediately, then the details. This is the single most useful technique in this book.

> 💡 **Trivia**
>
> Ro's semaglutide nausea guide opens: "Semaglutide, the active ingredient in Wegovy, Ozempic, and Rybelsus, commonly causes nausea. This is likely because semaglutide slows gastric emptying..." [1] 🟠 Industry
>
> Answer in sentence one. Reason in sentence two. That's the inverted pyramid. (We'd write it without the brand names, but the structure is perfect.)

20.1 The inverted pyramid

Journalists invented it so readers who stop early still get the news. Online, it also helps People Also Ask, featured snippets and AI answers pick up your answer. 🔵

The Inverted Pyramid



Most important first. Readers who stop early still get the answer.

20.2 BLUF: Bottom Line Up Front

BLUF (from military writing) means: state the conclusion first, then explain.

❌ Bottom line last	✅ BLUF
"GLP-1 medications have a long history. Researchers first studied... Over time, providers noticed... So, yes, you can usually drink coffee."	"Yes, most people can drink coffee while taking compounded semaglutide. Coffee may make nausea or reflux worse for some people, so start with smaller amounts and watch how you feel."

20.3 The answer block (use under every H2)

Part	Length	Example (H2: "How long does semaglutide nausea last?")
Direct answer	1 sentence	"For many people, nausea is strongest in the first few weeks and after each dose increase, then eases."
Key qualifier or detail	1–2 sentences	"It varies from person to person. If it lasts or becomes severe, contact your provider."
Then details	As needed	Tips, timelines, sources

Aim for **40–60 words** in the answer block. It should make sense if someone reads nothing else.

> **🍛 Desi analogy: cricket score update**
>
> Dost poochta hai "score kya hai?" Aap kehte ho "India 245/3, 40 overs." Aap yeh nahi batate ki toss kisne jeeta, pitch kaisi thi, mausam kaisa tha. **Pehle score, phir kahaani.**

20.4 Where to use the inverted pyramid

Place	How
Page intro	Answer the H1's question in the first 2 sentences
Under each H2	Answer block
FAQs	Answer in the first sentence
Meta description	The answer + benefit (Chapter 36)
Emails and captions	Main point first line
Service pages	What it is, who it's for, price/next step at the top

20.5 Before/after rewrites

❌ Before	✅ After
"Weight loss is a journey many Americans are on. With so many options, it can be hard to know where to start. That's where compounded semaglutide comes in."	"Compounded semaglutide is a weekly injection that may reduce appetite and help with weight loss when a provider decides it's right for you. It is not FDA-approved. Here's how it works, what it costs, and what to expect."
"Before we talk about price, let's look at what goes into care..."	"[Price exactly as on the website] includes your provider visits, medication and shipping. Here's what that covers."

> **🟧 Ongo Weight Loss example**
>
> Our homepage H1, "Doctor-Supervised Virtual GLP-1 Weight Loss Program", tells what it is, which is good. The first lines below it should answer the next questions in order: what medication, is it FDA-approved, how much, and how to start. That's the inverted pyramid for a service page. 🔵

20.6 Do's and Don'ts

✅ Do	❌ Don't
Answer in the first 1–2 sentences	Warm up with history or "In today's world"
Write a 40–60-word answer block per H2	Hide the answer in paragraph four
Put the qualifier right after the answer	Bury limits at the end
Then add depth	Stop at the answer if more is needed

20.7 Pros and cons of answer-first writing

✅ Pros	❌ Cons
Readers get help instantly	Some readers leave after the answer (that's OK: they were helped)
Better for snippets and AI answers	Takes practice to resist warm-ups
Clear for reviewers	Needs good qualifiers to stay accurate

20.8 Where freshers go wrong in Chapter 20 topics

Mistake	Why it happens	Avoid it by	Fix it by
History-first intros	School essay habit	Pyramid diagram	Moving the answer up
Answer with no qualifier	Wanting a clean answer	Answer block	Adding "varies / talk to your provider"
Answer blocks that are 150 words	Over-explaining	40–60-word target	Trimming

20.9 Thumb rules

👍 Score first, story later.

👍 Every H2 gets a 40–60-word answer block.

👍 Answer, qualifier, details: in that order.

20.10 Notes to remember

📝 Inverted pyramid: most important → key details → support → background.

📝 BLUF = Bottom Line Up Front.

Chapter 20 Glossary

Term	Simple meaning
Inverted pyramid	Most important information first, least important last
BLUF	Bottom Line Up Front
Answer block	A 40–60-word direct answer right after a heading
Featured snippet	A highlighted answer at the top of Google results
Warm-up	Unnecessary lead-in before the point

Chapter 20 Quiz

1. What comes first in the inverted pyramid?

- A) Sources
- B) Background
- C) History
- D) The most important information: the direct answer

2. What does BLUF mean?

- A) Bold Links Under Fold
- B) Big Long Useful Facts
- C) Blog Layout Under Footer
- D) Bottom Line Up Front

3. How long should an answer block be?

- A) About 40–60 words
- B) 500 words
- C) 5 words
- D) 1,000 words

4. Where does the qualifier go?

- A) Right after the answer
- B) In the last paragraph only
- C) Nowhere
- D) In the footer

5. Which intro is answer-first?

- A) "Compounded semaglutide is a weekly injection that may reduce appetite..."
- B) "Weight loss is a journey many are on..."
- C) "Let's dive in!"
- D) "In today's world..."

6. What did Ro's nausea guide do in sentence one?

- A) Gave the answer (it commonly causes nausea)
- B) Showed a price
- C) Told a story
- D) Asked a question only

7. Why does answer-first help AI answers?

- A) It hides content
- B) It doesn't
- C) The answer is easy to find and extract
- D) It adds keywords

8. Who invented the inverted pyramid?

- A) Cricketers
- B) Accountants
- C) Chefs
- D) Journalists

9. On a service page, what should come right after the H1?

- A) Company history
- B) What medication, FDA status, price and how to start
- C) A long quote
- D) Social links

10. In the cricket analogy, you give…

- A) The toss result only
- B) The score first, the story later
- C) The weather first
- D) Nothing

Sources

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026)
2. NIH, Clear & Simple (most important points first)
3. Ongo Weight Loss, homepage review, 30 October 2026 (internal)

Answer key, Chapter 20: 1-D · 2-D · 3-A · 4-A · 5-A · 6-A · 7-C · 8-D · 9-B · 10-B

LEVEL 3 · CHAPTER 21

Chapter 21: Hypophora: Ask the Reader's Question, Then Answer It

Hypophora is when the writer asks a question the reader is silently thinking, then answers it straight away. Done well, readers feel you read their mind.

> 💡 **Trivia**
>
> "Hypophora" comes from Greek rhetoric. It's different from a **rhetorical question**, which is asked for effect and **not** answered ("Who doesn't want to feel better?"). 🔵 Expert
>
> In health writing, unanswered questions frustrate readers. Hypophora always answers.

21.1 Hypophora vs rhetorical question

Device	Pattern	Example	Use?
Hypophora	Question → immediate answer	"Will I have to inject myself? Yes. Compounded semaglutide is a weekly injection under the skin, and your care team shows you how."	✅ Often
Rhetorical question	Question, no answer	"Isn't it time you took control?"	⚠️ Rarely; can sound salesy
Leading question	Pushes one answer	"Wouldn't you rather pay less than everyone else?"	❌ Avoid in health copy

21.2 Where hypophora works best

Place	Example
After a fact that creates a worry	"Nausea is the most common side effect. Does that mean you'll feel sick all the time? No. For many people it's strongest early and eases over time."
Before a price or step	"So what does it cost? [Price as on the site], and that includes..."
To handle objections	"Why compounded instead of the brand? Cost and availability are common reasons. It's not FDA-approved, and a provider decides if it's right for you."
Headings	"Can I drink alcohol on semaglutide?" (H2) → answer block
FAQs	Every FAQ is hypophora by design

> 🪔 **Desi analogy: mohalle ki chachi**
>
> Chachi aapke chehre se samajh jaati hain: "Bhookh lagi hai na? Ruko, paratha bana deti hoon." Aapke bolne se pehle sawaal bhi, jawaab bhi. **Hypophora wahi chachi hai:** reader ka sawaal pehchaano aur turant jawaab do.

21.3 How to find the silent questions

Method	What to look for
People Also Ask	Real follow-up questions
Support and comment logs	"Is this normal?", "When will I see results?"
Read your own paragraph and ask "So what?" / "Is that bad?" / "What do I do?"	The next question your reader has
Provider interviews	Questions patients ask every week

21.4 Rules for good hypophora

Rule	❌ Weak	✅ Strong
Ask a question readers actually have	"What is life without health?"	"How soon might I notice a change in appetite?"
Answer in the **next** sentence	Question, then three paragraphs of background	Question, then the direct answer
Keep the answer honest	"Will it work? Absolutely!"	"Will it work for you? Results vary; your provider will track your progress and adjust your plan."
Don't overuse it	Every paragraph starts with a question	2–5 per article, plus question H2s and FAQs

21.5 Practice: turn statements into hypophora

Statement	Hypophora version
"Shipping takes [X] days."	"When will your medication arrive? Usually [timeframe as on the site] after your provider approves your prescription."
"Side effects can occur."	"Will you have side effects? Some people do, most often stomach-related ones like nausea, especially early on."
"Prescriptions aren't guaranteed."	"Does everyone get a prescription? No. A provider reviews your health history and decides if treatment is right for you."

> 🟧 **Ongo Weight Loss example**
>
> Our homepage has a strong question-style line: "Your Weight Loss Shouldn't Need Insurance." Turn it into hypophora in the section below: "Do you need insurance? No. Ongo Weight Loss is self-pay, so you won't deal with prior authorizations or coverage denials. [Price as on the site] includes..." 🔵

21.6 Do's and Don'ts

✅ Do	❌ Don't
Answer immediately	Leave questions hanging
Use real reader questions	Invent dramatic questions
Keep answers honest and qualified	Answer with hype ("Absolutely!")
Use it where worries appear	Start every paragraph with a question

21.7 Pros and cons of hypophora

✅ Pros	❌ Cons
Feels personal and caring	Overuse feels gimmicky
Matches how people search (questions)	Weak questions waste space
Great for snippets and AI answers	Needs real research to find good questions

21.8 Where freshers go wrong in Chapter 21 topics

Mistake	Why it happens	Avoid it by	Fix it by
Rhetorical sales questions	Ad habit	21.1 table	Turning into real Q&A
Delayed answers	Building suspense	"Next sentence" rule	Moving the answer up
"Absolutely!" answers	Wanting confidence	Honesty rule	Adding qualifiers

21.9 Thumb rules

👍 Ask what they're thinking; answer in the next breath.

👍 Real questions, honest answers.

👍 Two to five per article is plenty.

21.10 Notes to remember

📝 Hypophora answers; rhetorical questions don't.

📝 Leading questions don't belong in health copy.

Chapter 21 Glossary

Term	Simple meaning
Hypophora	Asking a question and answering it immediately
Rhetorical question	A question asked for effect, not answered
Leading question	A question that pushes one answer
Objection	A reason a reader hesitates
Silent question	What the reader is thinking but hasn't asked

Chapter 21 Quiz

1. What is hypophora?

- A) A question never answered
- B) A spelling rule
- C) A type of image
- D) Asking a question and answering it immediately

2. Which is a leading question to avoid?

- A) "When will it arrive?"
- B) "Do you need insurance?"
- C) "How long does nausea last?"
- D) "Wouldn't you rather pay less than everyone else?"

3. When should the answer come?

- A) In the next sentence
- B) Never
- C) Three paragraphs later
- D) At the end of the page

4. How many hypophora uses per article (plus H2s and FAQs)?

- A) About 2–5
- B) 0
- C) 50
- D) Every paragraph

5. Which answer is honest?

- A) "Absolutely, guaranteed!"
- B) "No doubt at all!"
- C) "Results vary; your provider will track your progress."
- D) "Of course, for everyone!"

6. Where can you find silent questions?

- A) Only in your head
- B) Nowhere
- C) People Also Ask, support logs, provider interviews
- D) Competitor ads

7. What's a rhetorical question?

- A) A heading
- B) An FAQ
- C) A question asked for effect, not answered
- D) A question always answered

8. Which is good hypophora about prescriptions?

- A) "Want a prescription? Click now!"
- B) "Who needs doctors?"
- C) "Prescriptions? Easy!"
- D) "Does everyone get a prescription? No. A provider reviews your health history..."

9. Every FAQ is...

- A) A rhetorical question
- B) Hypophora by design
- C) Optional
- D) A leading question

10. In the chachi analogy, hypophora means...

- A) Ignoring the guest
- B) Talking about yourself
- C) Recognising the question and answering right away
- D) Asking without answering

Sources

1. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (hypophora), internal

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Answer key, Chapter 21: 1-D · 2-D · 3-A · 4-A · 5-C · 6-C · 7-C · 8-D · 9-B · 10-C

Chapter 22: Breaking the Fourth Wall and the Conversational "You"

Sometimes the page stops "presenting" and speaks directly, almost personally, to the reader. Used well, it builds trust. Used badly, it feels fake or too casual for health.

> 💡 **Trivia**
>
> The "fourth wall" is a theatre term: the invisible wall between actors and the audience. When an actor turns and talks to the audience, they "break the fourth wall". 🔵 Expert
>
> In writing, it's the moment the page openly acknowledges the reader and the situation they're in.

22.1 Three levels of reader closeness

Level	What it sounds like	Where we use it
1 · Neutral third person	"Patients may experience nausea."	Research summaries, policies
2 · Conversational "you"	"You may notice nausea in the first weeks."	Most pages (default)
3 · Fourth-wall moment	"If you're reading this at 2 a.m. because you feel sick after your first dose, here's what helps, and when to call someone."	Once or twice per piece, at emotional moments

22.2 What a fourth-wall break looks like

Technique	Example
Naming the reader's situation	"If you've tried diet after diet and you're tired of starting over, this part is for you."
Acknowledging the page itself	"We'll be honest in this section, even where the answer isn't what you hoped."
Admitting a limit	"We can't tell you exactly how much you'll lose. Nobody honestly can. Here's what we can tell you."
Pausing to reassure	"That's a lot of information. Here's the short version."

> 🍛 **Desi analogy: shaadi ka MC**
>
> Shaadi ka MC beech mein ruk ke kehta hai, "Pata hai aap sab bhookhe ho, bas paanch minute aur!" Sab hans dete hain aur relax ho jaate hain. **Fourth wall todna yahi hai:** sach mein dekh ke baat karna, lekin sirf sahi waqt pe.

22.3 When to break the fourth wall

✅ Good moments	❌ Bad moments
Opening an emotional topic (shame, failed diets)	Dosing instructions (be precise, not chatty)
Before hard truths (cost, limits, side effects)	Safety warnings (be clear and direct)
Long pages: a "here's the short version" pause	Legal and policy text
Ending: "Whatever you decide, here's one thing worth knowing..."	To push a sale ("We know you want this, so click now")

22.4 Conversational, not casual

✅ Conversational	❌ Too casual for health
"Here's the honest answer."	"Real talk, bestie 💅"

✅ Conversational	❌ Too casual for health
"That's normal, and it often gets easier."	"Lol, everyone pukes at first, no biggie."
"Let's look at the costs together."	"Let's get this bread!"
Contractions (you're, it's, don't)	Slang, memes, emojis on web pages

22.5 Second person done right

Rule	Example
"You" for the reader's actions and feelings	"You can message your provider anytime."
Don't promise "you will" outcomes	❌ "You will lose weight." ✅ "You may notice..."
Don't assume the reader's situation	❌ "You've failed every diet." ✅ "If you've tried other approaches..."
Use "we" honestly	"We don't offer brand-name medications."

> 🟧 **Ongo Weight Loss example**
>
> Homepage: "You don't have to figure this out alone." It's warm but generic. A sharper fourth-wall version: "If you've been researching GLP-1s for weeks and still aren't sure what's real, you're not alone, and you don't have to sort it out by yourself. A provider can walk through your options with you." 🔵

22.6 Do's and Don'ts

✅ Do	❌ Don't
Use "you" by default	Write in stiff third person everywhere
Break the fourth wall at emotional moments	Use it in dosing or safety instructions
Stay warm and respectful	Use slang, memes or emojis on web pages

✅ Do	❌ Don't
Say "if you..." instead of assuming	Tell readers how they feel

22.7 Pros and cons of the conversational style

✅ Pros	❌ Cons
Builds connection and trust	Can feel fake if overdone
Easier to read	Risk of sounding unprofessional
Readers feel seen	Must not soften safety messages

22.8 Where freshers go wrong in Chapter 22 topics

Mistake	Why it happens	Avoid it by	Fix it by
Fourth-wall jokes in a side-effect section	Trying to lighten the mood	22.3 table	Removing; staying direct
"You will lose weight"	Over-confident "you"	22.5 rules	Adding "may"
Slang and emojis on pages	Social habit	22.4 table	Plain warm wording

22.9 Thumb rules

> 👍 Talk with the reader, not at them.
>
> 👍 Break the wall at emotional moments; keep it up for instructions.
>
> 👍 Warm, not casual.

22.10 Notes to remember

📝 Default: conversational "you". Fourth-wall moments: 1–2 per piece.

📝 Never use "you will" for outcomes.

Chapter 22 Glossary

Term	Simple meaning
Fourth wall	The invisible line between a work and its audience
Breaking the fourth wall	Speaking directly and openly to the reader about their situation
Second person	Writing with "you"
Conversational tone	Writing like a natural, respectful conversation
Register	How formal or informal language is

Chapter 22 Quiz

1. What does "breaking the fourth wall" mean in writing?

- A) Using four images
- B) Speaking directly and openly to the reader about their situation
- C) Deleting the conclusion
- D) Adding a fourth heading

2. Where should you NOT break the fourth wall?

- A) Long-page summaries
- B) Emotional introductions
- C) Before hard truths
- D) Dosing instructions and safety warnings

3. What's our default point of view?

- A) Passive voice
- B) Formal third person
- C) First person "I"
- D) Conversational "you"

4. Which is too casual for health pages?

- A) "Here's the honest answer."
- B) "That's normal, and it often gets easier."
- C) "Real talk, bestie 💅"
- D) "Let's look at the costs together."

5. Which "you" sentence is acceptable?

- A) "You will lose 20 lbs."
- B) "You've failed every diet."
- C) "You may notice changes in appetite."
- D) "You must buy today."

6. How many fourth-wall moments per piece?

- A) 20
- B) None ever
- C) 1–2
- D) Every paragraph

7. Which is a good fourth-wall technique?

- A) Pushing a sale
- B) Using memes
- C) Admitting a limit ("We can't tell you exactly how much you'll lose.")
- D) Hiding prices

8. Should you assume the reader's situation?

- A) Only in titles
- B) Yes, always
- C) No; use "if you..."
- D) Only in emails

9. Are contractions allowed?

- A) Never
- B) Yes, for warmth (not in legal pages)
- C) Only in policies
- D) Only in headings

10. In the shaadi-MC analogy, the MC…

- A) Talks nonstop
- B) Speaks directly at the right moment to relax everyone
- C) Sells tickets
- D) Never speaks

Sources

1. Ongo Weight Loss, homepage review, 30 October 2026, and voice guide (Chapter 5), internal

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Answer key, Chapter 22: 1-B · 2-D · 3-D · 4-C · 5-C · 6-C · 7-C · 8-C · 9-B · 10-B

LEVEL 3 · CHAPTER 23

Chapter 23: Clinical Neutrality and Hedging

The most trusted health sites sound calm, specific and careful. Never breathless, never absolute. That voice is called clinical neutrality, and it's required for every medical sentence we write.

> 💡 **Trivia**
>
> Mayo Clinic's semaglutide page opens its side-effects section with: "Along with its needed effects, a medicine may cause some unwanted effects." [1] ● Official (Mayo Clinic)
>
> Calm. Balanced. A hedge ("may"). No drama. That's clinical neutrality in one sentence.

23.1 Three rules of clinical neutrality

Rule	❌ Marketing voice	✅ Clinical neutrality
1 · Remove hype adjectives	"Semaglutide is a revolutionary, life-changing solution."	"Semaglutide is a prescription medication that may reduce appetite."
2 · Hedge appropriately	"You will lose weight."	"In clinical trials of brand semaglutide, many participants lost weight; results vary."
3 · Balance benefit and risk	"Lose weight with zero hassle!"	"It can help with weight loss. Common side effects include nausea and constipation."

23.2 The hedging toolkit

Hedge	Use when	Example
may / can / might	Possible, not certain	"Ginger may ease nausea for some people."
often / usually / commonly	Frequent but not always	"Nausea is usually worst early on."

Hedge	Use when	Example
some / many / most	Part of a group	"Some people notice changes within weeks."
is associated with / is linked to	Correlation, not cause	"Higher protein intake is associated with..."
suggests / indicates	Evidence points that way	"Research suggests..."
in the study / in trials	Results from a specific study	"In STEP 1, participants..."
talk to your provider	Individual decisions	"Talk to your provider before changing your dose."

23.3 Don't over-hedge

Too many hedges make you sound unsure and confuse readers.

❌ Over-hedged	✅ Right amount
"It may possibly perhaps help some people in certain cases, potentially."	"It may help some people."
"Compounded medications might not be FDA-approved."	"Compounded medications are not FDA-approved." (a fact; no hedge needed)

Rule: hedge **uncertain** things; state **facts** plainly. 🔵

> 🪔 **Desi analogy: mausam vibhaag**
>
> Mausam vibhaag kehta hai: "Kal baarish ki 70% sambhavna hai." Woh nahi kehta "pakka baarish hogi" aur na hi "shayad, kabhi, kuch jagah, ho bhi sakti hai." **Sahi andaaza, sahi shabdon mein.**

23.4 Balancing benefit and risk

Pattern	Example
Benefit + limit in the same paragraph	"GLP-1 medications can reduce appetite. They aren't right for everyone, and a provider decides if they're appropriate."

Pattern	Example
Benefit + side effect	"Many people notice they feel full sooner. Some also notice nausea, especially early on."
Result + context	"Participants lost 14.9% on average over 68 weeks in STEP 1 (brand semaglutide). Compounded versions weren't studied."

Leaving out risks is as misleading as stating something false. 🔵

23.5 Neutral words vs loaded words

Loaded	Neutral
shocking, alarming, terrifying	notable, important
miracle, breakthrough, game-changer	new option, treatment
dangerous (without context)	"can cause serious side effects in rare cases"
proven (without source)	"shown in [study]"
everyone, always, never	many, often, rarely

> 🟧 **Ongo Weight Loss example**
>
> Homepage testimonial header: "Real Transformations, Backed by Science." "Transformations" is loaded and "backed by science" implies our product was studied. A neutral version: "Experiences shared by patients (with permission). Individual results vary." 🔵

23.6 Do's and Don'ts

✅ Do	❌ Don't
Hedge uncertain claims	Hedge plain facts
Pair benefits with limits	List benefits alone

✅ Do	❌ Don't
Use neutral, specific words	Use hype or fear words
Name the study behind a result	Say "proven" without a source

23.7 Pros and cons of clinical neutrality

✅ Pros	❌ Cons
Highest trust with readers, raters and reviewers	Less "exciting" copy
Lower legal risk	Over-hedging risk
Consistent medical voice	Needs practice to stay warm

23.8 Where freshers go wrong in Chapter 23 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Proven to work"	Marketing habit	Loaded-word list	"Shown in [study]"
Hedging a fact ("might not be FDA-approved")	Over-caution	Facts-plain rule	Stating it plainly
Benefits-only section	Wanting persuasion	Balance patterns	Adding limits and side effects

23.9 Thumb rules

> 👍 Calm, specific, balanced.
>
> 👍 Hedge the uncertain; state the certain.
>
> 👍 Every benefit travels with its limit.

23.10 Notes to remember

📝 Three rules: remove hype, hedge appropriately, balance benefit and risk.

📝 Omission of risks is a form of misleading.

Chapter 23 Glossary

Term	Simple meaning
Clinical neutrality	Calm, balanced, evidence-based medical voice
Hedge	A word that limits certainty ("may", "often")
Loaded word	A word that pushes emotion ("miracle", "shocking")
Correlation	Two things happening together, not necessarily cause
Omission	Leaving out important information

Chapter 23 Quiz

1. What are the three rules of clinical neutrality?

- A) Use jokes, emojis and slang
- B) Write long sentences
- C) Add hype, avoid hedges, list benefits
- D) Remove hype, hedge appropriately, balance benefit and risk

2. Should facts be hedged?

- A) Only on Mondays
- B) No; state facts plainly
- C) Only in titles
- D) Yes, always

3. Which is a neutral phrase?

- A) "A new treatment option"
- B) "A miracle breakthrough"
- C) "Game-changer"
- D) "Shocking results"

4. What does "is associated with" signal?

- A) A price
- B) A brand name
- C) Correlation, not proven cause
- D) Guaranteed cause

5. How did Mayo Clinic open its side-effects section?

- A) "Along with its needed effects, a medicine may cause some unwanted effects."
- B) "Terrifying risks ahead"
- C) "Buy now"
- D) "No side effects!"

6. Which is over-hedged?

- A) "Compounded medications are not FDA-approved."
- B) "Talk to your provider."
- C) "It may possibly perhaps help some people in certain cases, potentially."
- D) "It may help some people."

7. Why is leaving out risks a problem?

- A) It's required
- B) It misleads as much as a false statement
- C) It saves space
- D) It improves SEO

8. What should replace "proven" without a source?

- A) "Shown in [named study]"
- B) "100% proven"
- C) Nothing
- D) "Totally proven"

9. What's wrong with "Real Transformations, Backed by Science"?

- A) Loaded word and implies our product was studied
- B) It's too short
- C) It has a source
- D) Nothing

10. In the weather analogy, good writing...

- A) Gives a clear, honest estimate
- B) Says nothing useful
- C) Uses fear
- D) Says "pakka" every time

Sources

1. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026 🟢
2. Wilding et al., STEP 1, *NEJM* (2021) 🟢
3. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Answer key, Chapter 23: 1-D · 2-B · 3-A · 4-C · 5-A · 6-C · 7-B · 8-A · 9-A · 10-A

LEVEL 3 · CHAPTER 24

Chapter 24: Persuasion Models, Used Ethically

Copywriters use proven structures to move readers from interest to action. In health, we can use them only if every step is honest. This chapter shows each model with a compliant Ongo Weight Loss example.

> 💡 **Trivia**
>
> Ro labels its testimonials: "Ro members were paid for their testimonials." Fridays defines its stats: its "success rate" is fulfilment performance and "does not represent clinical outcomes." [1][2] 🟠 Industry
>
> Persuasion and honesty can live on the same page. The disclosure is part of the persuasion: it earns trust.

24.1 The ethics test (before using any model)

☐	Question
☐	Is every claim true and sourced?
☐	Are limits, risks and "not FDA-approved" visible?
☐	Does it inform before it persuades?
☐	Would it still be fair if the reader were your parent?
☐	No fear, shame, fake urgency or fake scarcity?

If any box fails, the model becomes manipulation. 🔵

24.2 The models

AIDA: Attention · Interest · Desire · Action

Step	Ongo Weight Loss example
Attention	"Weight care without insurance hassles."
Interest	"Talk to a licensed provider online. If treatment is right for you, compounded semaglutide ships to your door."
Desire	"One price that includes your visits, medication and support. [Price as on site]."
Action	"Check my eligibility (free, about 5 minutes)."
Plus	"Compounded medications are not FDA-approved. A provider decides if treatment is appropriate."

Best for: landing pages, emails.

PAS: Problem · Agitate · Solve

Step	Example	Ethical limit
Problem	"Brand-name GLP-1s can be hard to afford without insurance."	True, sourced (KFF: 56% say hard to afford)
Agitate	"Many people stop treatment because of cost."	**Gently**, with facts; never fear or shame
Solve	"Ongo Weight Loss offers compounded options at [price], with provider care included."	Add disclaimers

Best for: cost pages, comparison pages. **Risk:** "agitate" can turn into fear; keep it factual.

BAB: Before · After · Bridge

Step	Example	Ethical limit
Before	"You've researched GLP-1s for weeks and still aren't sure what's real."	Describe a situation, not a body
After	"You have a clear answer from a licensed provider about what fits you."	No weight or appearance outcomes

Step	Example	Ethical limit
Bridge	"A free eligibility check and a provider consultation."	—

Best for: how-it-works sections. **Never** use BAB with body images or weight numbers.

FAB: Features · Advantages · Benefits

Feature	Advantage	Benefit
Online provider visits	No waiting rooms	Care that fits your schedule
All-in pricing	No surprise add-ons	Easier to budget
Messaging your care team	Questions answered between visits	Less worry about side effects

Best for: service pages, pricing tables.

The 4 Cs: Clear · Concise · Compelling · Credible

C	Check
Clear	Would Linda (58) understand it?
Concise	Any word you can cut?
Compelling	Does it speak to a real need?
Credible	Is it sourced and qualified?

Best for: checking any copy, especially headlines and CTAs.

The 4 Us: Useful · Urgent · Unique · Ultra-specific (headlines)

Use **Useful, Unique, Ultra-specific**. Use "Urgent" only when something is **truly** time-bound (e.g., a real policy deadline). Never invent urgency.

> 🪔 **Desi analogy: sabzi waale bhaiya**
>
> Achha sabzi waala kehta hai: "Aaj tamatar taaze aaye hain, kal se mehenge honge" (sach ho toh). Bura sabzi waala jhooth bolta hai "aakhri tamatar hain!" jab poora tokra bhara ho. **Persuasion theek hai; jhoothi urgency nahi.**

24.3 Manipulation tactics we never use

❌ Tactic	Example
Fake urgency	"Only 2 hours left!" (when it isn't true)
Fake scarcity	"Only 3 spots remaining!"
Shame	"Stop letting yourself go."
Fear without facts	"Obesity is killing you right now."
Bandwagon without data	"Everyone's switching to us."
Hidden costs	Low headline price, extras in fine print
Confirm-shaming	"No thanks, I don't care about my health"

24.4 Which model where

Page	Model
Landing page	AIDA
Cost / comparison	PAS (gentle)
How it works	BAB (situations only)
Service / pricing	FAB
Headlines, CTAs	4 Cs / 4 Us (honest)

🟧 **Ongo Weight Loss example**

"Patient First, Finance Later" on our homepage is catchy, but ambiguous: it could sound like financing. Apply the 4 Cs: Clear? Not quite. Credible? Unclear. A better line: "Care comes first, with one all-in price and no surprise add-ons." 🔵

24.5 Do's and Don'ts

✅ Do	❌ Don't
Run the ethics test first	Use fake urgency or scarcity
Pair every promise-like line with facts	Agitate with fear or shame
Disclose paid testimonials and data limits	Hide costs in fine print
Use FAB for clarity	Use BAB with bodies or numbers

24.6 Pros and cons of persuasion models

✅ Pros	❌ Cons
Clear, tested structures	Easy to slip into manipulation
Faster drafting of money pages	Can feel formulaic
Better conversions when honest	Needs legal review

24.7 Where freshers go wrong in Chapter 24 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Only 3 spots left!"	Copying ecommerce	Manipulation list	Removing
BAB with before/after weights	Natural fit	BAB limits	Situations, not bodies
PAS that shames	"Agitate" taken too far	Ethics test	Facts, gently

24.8 Thumb rules

👍 Persuade with truth, never with pressure.

👍 Situations, not bodies.

👍 Urgency only when it's real.

24.9 Notes to remember

📝 Models: AIDA, PAS, BAB, FAB, 4 Cs, 4 Us.

📝 Run the five-box ethics test before using any model.

Chapter 24 Glossary

Term	Simple meaning
AIDA	Attention, Interest, Desire, Action
PAS	Problem, Agitate, Solve
BAB	Before, After, Bridge
FAB	Features, Advantages, Benefits
Dark pattern	A design or copy trick that pressures or misleads users

Chapter 24 Quiz

1. What does AIDA stand for?

- A) Ask, Inform, Delay, Agree
- B) None
- C) Attention, Interest, Desire, Action
- D) Add, Insert, Delete, Archive

2. What's the ethical limit for "Agitate" in PAS?

- A) Use shame
- B) Gently, with facts; never fear or shame
- C) No limit
- D) As scary as possible

3. Can BAB use before/after weights?

- A) If small
- B) On social only
- C) Yes
- D) No; situations only

4. What is FAB?

- A) Fast And Bold
- B) Facts About Brands
- C) Features, Advantages, Benefits
- D) Fear And Blame

5. Which is a manipulation tactic?

- A) Clear pricing
- B) Disclosed testimonials
- C) Fake scarcity ("Only 3 spots remaining!")
- D) Sourced facts

6. When is urgency allowed?

- A) Always
- B) Never, even if true
- C) Only when something is truly time-bound
- D) On weekends

7. What did Ro do with testimonials?

- A) Hid payments
- B) Used AI testimonials
- C) Disclosed that members were paid
- D) Nothing

8. Which model suits a service or pricing page?

- A) None
- B) Fake scarcity
- C) Confirm-shaming
- D) FAB

9. What's wrong with "Patient First, Finance Later"?

- A) It has a source
- B) Nothing
- C) It's too long
- D) It's ambiguous and could sound like financing

10. In the sabzi-waala analogy, the honest seller...

- A) Lies about the last tomatoes
- B) Hides prices
- C) Uses urgency only when it's true
- D) Never speaks

Sources

1. Ro, Weight loss, viewed 30 October 2026 🟠
2. Fridays, homepage, viewed 30 October 2026 🟠
3. KFF, GLP-1 poll (November 14, 2025) 🟢
4. FTC, Disclosures 101 for Social Media Influencers 🟢

Answer key, Chapter 24: 1-C · 2-B · 3-D · 4-C · 5-C · 6-C · 7-C · 8-D · 9-D · 10-C

LEVEL 3 · CHAPTER 25

Chapter 25: Explanation Models: Making Hard Things Easy

Persuasion models move people to act. Explanation models help people understand. Most of our content is explanation, so these are the tools you'll use daily.

> 💡 **Trivia**
>
> NIH's Clear & Simple guide says the most important points should come **first and last**, because those are what readers remember best. [1] ● Official (NIH)
>
> Many explanation models are simply smart ways to put the key point in those two spots.

25.1 The explanation toolkit

What · Why · How · What next

Step	Example (titration)
What	"Titration means starting at a low dose and raising it slowly."
Why	"It gives your body time to adjust, which can reduce side effects like nausea."
How	"Your provider sets the schedule and increases the dose in steps."
What next	"Don't change your dose yourself. Message your provider if side effects are hard to manage."

Best for: definitions, glossary entries, short sections.

PEEL: Point · Evidence · Explain · Link

Step	Example
Point	"Protein matters when you're eating less."

Step	Example
Evidence	"[Sourced guidance on protein during weight loss]"
Explain	"When appetite drops, it's easy to skip protein, which helps you stay full and supports muscle."
Link	"Here are 10 high-protein breakfasts that are easy on the stomach."

Best for: body paragraphs in guides.

The Feynman technique (explain it to a 12-year-old)

Step	Action
1	Write the concept as if teaching a child
2	Find where you used jargon or got stuck
3	Go back to the source and learn that part
4	Simplify again, with an analogy

Example: "GLP-1 is a message your gut sends your brain after you eat, saying 'we're getting full'. The medicine helps that message come through more clearly." (Reviewer checks accuracy.)

SCQA: Situation · Complication · Question · Answer

Step	Example
Situation	"You've started compounded semaglutide and your appetite has dropped."
Complication	"But you've also stopped losing weight for three weeks."
Question	"Is the medication still working?"
Answer	"Plateaus are common and don't always mean it's not working. Here's what to look at, and when to talk to your provider."

Best for: intros to problem-solving articles.

Analogy and metaphor

Good analogy	Why it works
"Titration is like getting into a cold pool one step at a time."	Familiar, matches the idea
"GLP-1 is like a 'you're full' text message from your gut to your brain."	Simple, memorable

Rules: the analogy must be accurate enough that it doesn't mislead; then state the real fact.

Show, don't tell

❌ Tell	✅ Show
"Our program is very supportive."	"You can message your care team anytime, and a provider checks in before each dose change."
"Side effects are manageable."	"Most people who feel nausea find that smaller meals, slower eating and avoiding greasy foods help."

The rule of three

People remember lists of three. "Clear pricing, licensed providers, ongoing support." Use it for key messages, not for padding.

The 5 Ws and H

Who, What, When, Where, Why, How: a quick check that a news item, policy update or how-to answers everything.

STAR (for case studies)

Step	Example (consented case)
Situation	Patient hit a plateau at week 10
Task	Understand why and adjust safely
Action	Provider reviewed eating patterns and dosing schedule
Result	Experience described in the patient's words; no promised numbers; "individual experiences vary"

> 🪔 **Desi analogy: dadi ki kahaani**
>
> Dadi mushkil baat bhi kahaani se samjha deti thi: "Beta, chheenk aaye toh rumaal, warna sabko 'sardi ka bhoot' pakad lega." **Achha explanation kahaani + sach dono deta hai.**

25.2 Which model where

Situation	Model
Defining a term	What · Why · How · What next
Body paragraph with evidence	PEEL
Complex science	Feynman + analogy
Problem article intro	SCQA
Case study	STAR
Benefits copy	Show, don't tell
News or policy update	5 Ws and H

> 🟧 **Ongo Weight Loss example**
>
> "Low Dose GLP-1" appears on our homepage without explanation. Using What · Why · How · What next: "A low starting dose (what) helps your body adjust (why). Your provider raises it gradually if appropriate (how). You'll never change it on your own (what next)." 🔵

25.3 Do's and Don'ts

✅ Do	❌ Don't
Choose the model for the job	Use one model for everything
Follow an analogy with the real fact	Let the analogy replace the fact
Show with specifics	Tell with adjectives

✅ Do	❌ Don't
Put key points first and last	Bury them in the middle

25.4 Pros and cons of explanation models

✅ Pros	❌ Cons
Faster, clearer drafting	Can feel mechanical if visible
Readers understand and remember	Analogies can mislead if loose
Easier to review	Need practice to blend smoothly

25.5 Where freshers go wrong in Chapter 25 topics

Mistake	Why it happens	Avoid it by	Fix it by
Analogy that's medically wrong	Wanting simplicity	Reviewer check	Fixing or removing
"Very supportive", "very easy"	Telling	Show, don't tell	Adding specifics
Rule of three as filler	Liking the sound	Purpose test	Cutting the weak third item

25.6 Thumb rules

👍 Explain to a smart 12-year-old, then check with a doctor.

👍 Show the specific; don't tell the adjective.

👍 Key point first, key point last.

25.7 Notes to remember

> 📝 Models: What-Why-How-Next, PEEL, Feynman, SCQA, analogy, show-don't-tell, rule of three, 5 Ws, STAR.
>
> 📝 Analogy first, real fact right after.

Chapter 25 Glossary

Term	Simple meaning
PEEL	Point, Evidence, Explain, Link
Feynman technique	Explaining something simply to find gaps in understanding
SCQA	Situation, Complication, Question, Answer
STAR	Situation, Task, Action, Result
Show, don't tell	Using specific details instead of adjectives

Chapter 25 Quiz

1. What does PEEL stand for?

- A) Plan, Execute, End, Leave
- B) None
- C) Point, Evidence, Explain, Link
- D) Price, Email, Edit, Launch

2. What's the Feynman technique?

- A) A design tool
- B) Explaining simply to find gaps, then simplifying again
- C) Using jargon
- D) Writing longer sentences

3. Which model suits a problem-article intro?

- A) Fake urgency
- B) SCQA
- C) None
- D) FAB

4. Which is "show, don't tell"?

- A) "You can message your care team anytime."
- B) "Best care ever."
- C) "Our program is very supportive."
- D) "We're amazing."

5. Where do readers remember points best (NIH)?

- A) Middle only
- B) Image captions
- C) Footnotes
- D) First and last

6. What must follow an analogy?

- A) Another analogy
- B) A joke
- C) A CTA
- D) The real fact

7. Which model is used for case studies?

- A) 4 Us
- B) PAS
- C) AIDA
- D) STAR

8. What does "What · Why · How · What next" suit best?

- A) Error messages
- B) Social hashtags
- C) Price tables only
- D) Defining a term

9. When is the rule of three wrong?

- A) In headings
- B) Always
- C) When the third item is filler
- D) Never

10. In the dadi analogy, a good explanation gives…

- A) Only a lecture
- B) Nothing
- C) Only a story
- D) A story and the truth

Sources

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Answer key, Chapter 25: 1-C · 2-B · 3-B · 4-A · 5-D · 6-D · 7-D · 8-D · 9-C · 10-D

Chapter 26: Information Gain: Be 1% More Useful

If your page says exactly what the other nine results say, why should anyone read yours, or rank it? Information gain is the habit of adding something genuinely new.

> 💡 **Trivia**
>
> Google asks content creators: "Does the content provide original information, reporting, research, or analysis?" and "Does the content provide substantial value when compared to other pages in search results?" [1] 🟢 Official (Google)
>
> Google also holds a patent on scoring "information gain", though it hasn't said how or whether it's used in ranking. 🟠 Industry
>
> Either way, the habit is right: add something the others don't.

26.1 What information gain means

Information gain = something useful on your page that a reader wouldn't get from the other top results. You don't need to be 100% different. You need to be **1% more useful**. 🔵

26.2 Seven sources of information gain for Ongo Weight Loss

Source	What it looks like	Example
1 · Provider insight	A reviewed note from our medical team	"Our providers often suggest writing down when nausea happens, so patterns are easier to spot."
2 · Process clarity	Exactly how our process works	"After you submit your intake, a provider reviews it, usually within [timeframe as stated by ops]."
3 · Visual translation	Turning text into a table, checklist or timeline	"Week-by-week: what you may notice" table

Source	What it looks like	Example
4 · Practical tools	Templates, trackers, question lists	"10 questions to ask your provider" printable
5 · Honest comparison	What's actually different, including limits	Compounded vs brand table with "not FDA-approved" row
6 · Aggregated patient questions	Patterns from support logs (no personal data)	"The question we hear most in week two is..."
7 · Expert counter-point	Correcting a common myth, carefully	"Many posts say you must stop eating carbs. Providers usually focus on protein and balance instead."

> 🍛 **Desi analogy: shaadi ka gift**
>
> Shaadi mein paanch log same dinner set dete hain. Jo ek dost haath se likhi recipe book deta hai, sab use yaad rakhte hain. **Wahi 1% alag cheez information gain hai.**

26.3 The information gain check

☐	Question
☐	Did I read the top 5 results (Chapter 14)?
☐	What do all of them miss or explain badly?
☐	Which of the seven sources can I add?
☐	Is my addition accurate and reviewed?
☐	Did I note it in the submission pack?

26.4 What is NOT information gain

❌ Not gain	Why
Rewording competitors	Same information, different words
More words on the same points	Length isn't value

❌ Not gain	Why
Invented "insider" stats	Dishonest and risky
Patient stories without consent	Privacy violation
Opinions without expertise	Not trustworthy in YMYL

🟧 **Ongo Weight Loss example**

Competitor guides list generic nausea tips. Our information gain could be a reviewed "first-dose day checklist" (what to eat that day, what to have at home, when to call) plus a simple symptom diary table. Ro's guide mentions a symptom diary; we can go further with a ready-to-use template. 🔵

26.5 Do's and Don'ts

✅ Do	❌ Don't
Add one real, useful extra per piece	Pad with more of the same
Turn walls of text into tables and checklists	Invent stats or stories
Get provider insights reviewed	Quote providers without approval
Note your gain in the submission	Assume reviewers will spot it

26.6 Pros and cons of chasing information gain

✅ Pros	❌ Cons
Content stands out to readers	Takes more research
More shares, links and AI citations	Needs provider time
Builds Experience and Expertise	Must stay accurate and reviewed

26.7 Where freshers go wrong in Chapter 26 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Our data shows..." with no data	Wanting authority	Honesty rule	Removing or getting real data
Same tips as everyone	Copying the top result	Top-5 review	Adding one of the seven sources
Unreviewed provider quotes	Speed	Review rule	Sending for approval

26.8 Thumb rules

👍 Read the top five; write what they're missing.

👍 A table can be the gain.

👍 Real, reviewed, or not at all.

26.9 Notes to remember

📝 Seven sources: provider insight, process clarity, visual translation, tools, honest comparison, aggregated questions, counter-point.

📝 Google asks about "substantial value compared to other pages".

Chapter 26 Glossary

Term	Simple meaning
Information gain	New, useful information not found in other top results
Visual translation	Turning text into a table, list or chart
Counter-point	A careful correction of a common belief

Term	Simple meaning
Aggregated	Combined into patterns without personal details
Commodity content	Content that says the same as everyone else

Chapter 26 Quiz

1. What is information gain?

- A) Useful information other top results don't have
- B) More images
- C) More keywords
- D) More words

2. How much more useful do you need to be?

- A) About 1%
- B) 100% different
- C) 50% longer
- D) Exactly the same

3. Which is a source of information gain?

- A) A reviewed provider insight
- B) Unconsented stories
- C) Rewording a competitor
- D) Invented stats

4. What does Google ask about value?

- A) Is it the longest?
- B) Does it use the most keywords?
- C) Does it provide substantial value compared to other pages?
- D) Does it have the most ads?

5. Has Google confirmed how an information gain patent is used in ranking?

- A) Yes, for all sites
- B) Yes, it's the top factor

- C) It doesn't exist
- D) No

6. Which is NOT information gain?

- A) A clear comparison table
- B) A reviewed provider tip
- C) A new checklist
- D) More words on the same points

7. What's step 1 of the information gain check?

- A) Read the top 5 results
- B) Pick fonts
- C) Write the conclusion
- D) Add emojis

8. Can support-log patterns be used?

- A) Yes, aggregated without personal data
- B) Yes, with names
- C) No, never
- D) Only with screenshots

9. What could be our nausea-guide information gain?

- A) More brand names
- B) A countdown timer
- C) A reviewed first-dose checklist and symptom diary template
- D) A longer intro

10. In the shaadi-gift analogy, the remembered gift is…

- A) The fifth dinner set
- B) The heaviest
- C) The most expensive
- D) The one that's different and personal

Sources

1. Google Search Central, Creating helpful, reliable, people-first content
2. Ro, Semaglutide nausea guide, viewed 30 October 2026
3. Ongo Weight Loss, writing guidelines (information gain), internal

Answer key, Chapter 26: 1-A · 2-A · 3-A · 4-C · 5-D · 6-D · 7-A · 8-A · 9-C · 10-D

LEVEL 3 · CHAPTER 27

Chapter 27: Storytelling and Patient Stories

Stories help people see themselves in your writing. In health, they also carry the most risk: privacy, consent and misleading results. This chapter shows how to tell stories safely.

> 💡 **Trivia**
>
> The FTC's rule on fake reviews and testimonials (effective October 2024) bans testimonials from people who don't exist (including AI-generated ones), from people without real experience, and testimonials that misrepresent someone's experience. Penalties can reach **$51,744 per violation** (2024 figure). [1] ● Industry (DWT)
>
> A made-up "Sarah lost 40 lbs" story isn't creative writing. It's a legal violation.

27.1 Three kinds of stories you can use

Story type	Example	Rules
Scenario (clearly hypothetical)	"Imagine you've just taken your first dose and feel a little queasy that evening..."	Must be obviously hypothetical; no names that suggest a real patient
Composite persona (internal planning)	Linda, 58 (Chapter 6)	**Never** presented as a real patient on public pages
Real, consented patient story	"In her words: 'Having someone to ask made the difference.'"	Signed consent; real; unedited; typicality disclosure; legal and medical review

27.2 The story arc (simple version)

Part	Question	Example (consented experience story)
Before	What was the situation?	"She'd researched GLP-1s for months and felt overwhelmed by conflicting advice."
Turning point	What changed?	"A provider walked her through her options and explained what to expect."
During	What was it like?	"The first weeks brought some nausea; smaller meals and messages to her care team helped."
Now	How does she describe it today?	"'I finally feel like I understand my plan.'"
Disclosure	—	"Shared with permission. Individual experiences vary."

Focus on experience, not numbers. Avoid pounds lost, timelines or body descriptions. 🔵

> 🍛 **Desi analogy: shaadi ki album**
>
> Shaadi ki album mein asli pal achhe lagte hain: dadi ki hasi, dost ka dance. Agar photographer chehre badal de ya nakli log daal de, toh album jhooth ban jaata hai. **Patient story bhi asli ho, bina "edit" ke.**

27.3 Consent and accuracy checklist

☐	Item
☐	Signed release (where, how long, what can be edited) + HIPAA authorisation if health details
☐	Patient's own words; edits only for length, with their approval
☐	No outcome numbers or "the medication did this" claims
☐	Typicality disclaimer ("Individual experiences vary")
☐	Paid or gifted? Disclosed
☐	Medical and legal review

☐	Item
☐	Consent stored with the content file

27.4 Mini-stories inside articles

You don't always need a full patient story. Small, honest storytelling moments work:

Technique	Example
Second-person scenario	"Picture the morning of your first dose. You might feel nervous, and that's normal."
Provider anecdote (reviewed, no patient details)	"Our providers say the most common question in week one is whether nausea means something is wrong."
Process story	"Here's what happens after you click 'Check my eligibility'..."

> 🟧 **Ongo Weight Loss example**
>
> Our homepage repeats the same three testimonials and labels them "Real Transformations". Better: a small set of consented **experience** stories (support, convenience, feeling informed), each with "Shared with permission. Individual experiences vary," and no transformation language. 🔵

27.5 Do's and Don'ts

✅ Do	❌ Don't
Use clear hypothetical scenarios	Invent "real" patients
Get signed consent for real stories	Quote reviews or DMs without permission
Focus on experience and feelings	Lead with pounds lost
Disclose typicality and payments	Hide incentives

27.6 Pros and cons of storytelling

✅ Pros	❌ Cons
Readers connect and remember	Heavy consent and review
Shows Experience (E-E-A-T)	High legal risk if done wrong
Makes process feel human	Real stories take time to gather

27.7 Where freshers go wrong in Chapter 27 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Meet Sarah, who lost 40 lbs" (invented)	Wanting emotion	27.1 rules	Deleting; using a scenario
Quoting a Trustpilot review on a page without permission	It's public	Consent checklist	Requesting permission
"The medication changed her life"	Natural phrasing	Experience focus	Rewriting around care

27.8 Thumb rules

👍 Real and consented, or clearly imagined.

👍 Feelings and care, not numbers and bodies.

👍 Every real story carries a disclosure.

27.9 Notes to remember

📝 FTC rule: no fake, AI or misrepresented testimonials.

📝 Composite personas are for planning only.

Chapter 27 Glossary

Term	Simple meaning
Testimonial	A statement about someone's experience with us
Composite persona	A made-up planning character based on research
Typicality disclosure	A note that results or experiences vary
Release form	Signed permission to use someone's story
Story arc	The shape of a story: before, turning point, during, now

Chapter 27 Quiz

1. Can we invent a patient story and present it as real?

- A) If positive
- B) Never
- C) If short
- D) Yes

2. What's the FTC penalty figure cited (2024)?

- A) $500
- B) None
- C) $10
- D) Up to $51,744 per violation

3. What should patient stories focus on?

- A) Speed of results
- B) Body photos
- C) Pounds lost
- D) Experience and care

4. Can composite personas appear as real patients publicly?

- A) On social only

- B) Yes
- C) With a stock photo
- D) No; planning only

5. What's needed before using a real story?

- A) A follow
- B) A like on social
- C) Nothing
- D) Signed consent (and HIPAA authorisation if health details)

6. Which is a safe scenario opening?

- A) "Our patient John was cured..."
- B) "Picture the morning of your first dose..."
- C) "Everyone loses 20%"
- D) "Meet Sarah, who lost 40 lbs..."

7. What disclosure goes with real stories?

- A) "Guaranteed"
- B) None
- C) "Shared with permission. Individual experiences vary."
- D) "Typical results"

8. What should replace "Real Transformations"?

- A) AI photos
- B) Consented experience stories with disclosures
- C) Nothing
- D) Bigger transformation claims

9. Can reviews from Trustpilot be quoted on pages without permission?

- A) Yes, freely
- B) Ask permission first
- C) Only negative ones
- D) Only edited ones

10. In the wedding-album analogy, an edited patient story is...

- A) A required album

- B) A better album
- C) A false album
- D) A free album

Sources

1. DWT, FTC finalizes rule banning fake consumer reviews (August 2024)
2. FTC, Disclosures 101 for Social Media Influencers
3. Ongo Weight Loss, homepage review, 30 October 2026 (internal)

Answer key, Chapter 27: 1-B · 2-D · 3-D · 4-D · 5-D · 6-B · 7-C · 8-B · 9-B · 10-C

LEVEL 3 · CHAPTER 28

Chapter 28: Headlines and Titles That Are Honest and Still Get Clicks

The headline is read by many times more people than the article. It must be clear, specific and true, and still make the right person want to click.

> 💡 **Trivia**
>
> Noom's blog titles are mostly sentence case and practical: "5 ways to spot a quick fix—and what to do instead", "Stopping a GLP-1: What to expect & how to manage weight after." [1] ● Industry
>
> Specific promise of help, no hype, plain words.

28.1 The four jobs of a headline

Job	Question it answers
Match	Does it match what the searcher typed?
Promise	What will I get from reading?
Qualify	Is this for me (patient, beginner, comparer)?
Honest	Does the page deliver exactly this?

28.2 Seven headline formulas (with Ongo Weight Loss examples)

Formula	Example
Question	"How long does nausea last on semaglutide?"
Topic: benefit	"Semaglutide nausea: why it happens and what helps"
How to	"How to measure your compounded semaglutide dose"
Number list	"10 high-protein breakfasts that are easy on your stomach"
X vs Y	"Semaglutide vs tirzepatide: how they compare"
What to expect	"Your first month on compounded semaglutide: what to expect"
Myth vs fact	"7 GLP-1 myths, and what the research says"

28.3 Headline rules for health

✅ Rule	❌ Example to avoid
Specific and accurate	"The ultimate secret to weight loss"
No outcome promises	"Lose 20 lbs in 30 days with semaglutide"
No brand drug names in commercial titles	"Cheaper than Wegovy"
No fear bait	"This side effect could kill you"
No vague clickbait	"You won't believe what happened next"
Year only if the content truly updates yearly	"Best GLP-1 tips 2026" (unless updated)
Keep the primary keyword near the start	"Semaglutide nausea: ..."
~50–60 characters for title tags (Book Four)	—

> 🪔 **Desi analogy: dukaan ka board**
>
> Dukaan ka board "Sharma General Store: Doodh, Bread, Anda" saaf batata hai andar kya hai. Agar board pe likha ho "Duniya ki sabse jaadui dukaan!" aur andar sirf doodh mile, log naraz hote hain. **Headline board hai: jo likho, woh andar ho.**

28.4 H1 vs meta title vs social title

Element	Purpose	Example
H1 (on page)	Clear topic for the reader	"Semaglutide nausea: why it happens, how long it lasts, and what helps"
Meta title (search results)	Click-worthy, fits ~60 characters, brand at end	"Semaglutide Nausea: Causes & What Helps | Ongo Weight Loss"
Social / email title	Short and human	"Feeling queasy on semaglutide? Try these first."

28.5 Before/after headlines

❌ Before	✅ After	Why
"Semaglutide Dosing Guide: Vial Sizes, Weekly Doses, Titration & How to Measure Your Dose"	"How to measure your compounded semaglutide dose"	One clear promise; details become H2s
"The Solution That Finally Works!"	"Doctor-led weight care, online, with one all-in price"	Specific and true
"Melt Fat Fast With GLP-1s"	"How GLP-1 medications affect appetite"	Accurate, no hype
"Real Transformations, Backed by Science"	"What patients say about their care (shared with permission)"	Honest framing

> 🟧 **Ongo Weight Loss example**
>
> "What are GLP-1 Medications and How Do They Support Weight Loss?" (our blog) is clear and question-led: keep that style. Just apply sentence case for blog titles per our house style: "What are GLP-1 medications and how do they support weight loss?" 🔵

28.6 Do's and Don'ts

✅ Do	❌ Don't
Promise exactly what the page delivers	Overpromise to get the click
Write 5–10 options, then pick	Use the first idea
Put the keyword early	Stuff several keywords
Check with the 4 Cs (Chapter 24)	Use hype or fear

28.7 Pros and cons of honest headlines

✅ Pros	❌ Cons
Readers trust and return	Fewer "curiosity" clicks
Lower bounce, better Needs Met	Harder to make exciting
Safe for compliance	Requires specificity

28.8 Where freshers go wrong in Chapter 28 topics

Mistake	Why it happens	Avoid it by	Fix it by
Keyword list as a title	SEO habit	One-promise rule	Moving details to H2s
"Ultimate guide" for a short post	Copying templates	Honest rule	Plain title

Mistake	Why it happens	Avoid it by	Fix it by
Year that never updates	Freshness trick	Year rule	Removing the year

28.9 Thumb rules

👍 One clear promise, kept.

👍 Keyword early, hype never.

👍 Write ten, pick one.

28.10 Notes to remember

📝 Seven formulas: question, topic-benefit, how-to, list, vs, what-to-expect, myth-vs-fact.

📝 Meta titles ~50–60 characters (Book Four).

Chapter 28 Glossary

Term	Simple meaning
Headline	The main title readers see
Meta title	The title shown in search results (title tag)
Clickbait	A misleading headline that overpromises
Sentence case	Only the first word and names capitalised
Title tag	The HTML element that sets the meta title

Chapter 28 Quiz

1. What are the four jobs of a headline?

- A) Rhyme, bold, emoji, caps
- B) Shock, scare, sell, shout
- C) Match, promise, qualify, be honest
- D) None

2. Which is an honest health headline?

- A) "The ultimate secret"
- B) "Melt Fat Fast With GLP-1s"
- C) "How GLP-1 medications affect appetite"
- D) "You won't believe this"

3. How long should a meta title be?

- A) 200 characters
- B) About 50–60 characters
- C) 5 characters
- D) Unlimited

4. When can a year go in a title?

- A) Only in December
- B) Never
- C) Only if the content truly updates yearly
- D) Always

5. Which is the "X vs Y" formula?

- A) "10 breakfasts"
- B) "Your first month"
- C) "Semaglutide vs tirzepatide: how they compare"
- D) "How to measure your dose"

6. What's wrong with a long keyword-list title?

- A) It's too short
- B) It's too honest
- C) It lacks one clear promise; details belong in H2s
- D) Nothing

7. Where should the primary keyword go?

- A) Near the start
- B) Repeated three times
- C) At the very end only
- D) Hidden

8. Which headline style does our house style use for blog titles?

- A) Sentence case
- B) ALL CAPS
- C) Random case
- D) Title case only

9. How many headline options should you write?

- A) 5–10, then pick
- B) 100
- C) One
- D) None

10. In the dukaan-board analogy, a headline must...

- A) Hide the products
- B) Be as big as possible
- C) Promise magic
- D) Match what's inside

Sources

1. Noom, Blog, viewed 30 October 2026
2. Ongo Weight Loss, blog listing and homepage, viewed 30 October 2026
3. Henry Meds, Weight management, viewed 30 October 2026

Answer key, Chapter 28: 1-C · 2-C · 3-B · 4-C · 5-C · 6-C · 7-A · 8-A · 9-A · 10-D

LEVEL 3 · CHAPTER 29

Chapter 29: Introductions, Key Points and Conclusions

The first 100 words decide if people stay. The last 100 decide what they do next. This chapter gives you formulas for both, plus the key-points box in between.

> 💡 **Trivia**
>
> Ro places a four-bullet **key takeaways** box above its nausea article, and closes with a "Bottom line" section, FAQs, references and "Why trust our experts?". [1] 🟠 Industry
>
> Strong beginning, strong ending, proof at the bottom.

29.1 Intro formulas (pick one)

Formula	Pattern	Example
Answer-first (default)	Answer → key qualifier → what the page covers	"Nausea is the most common side effect of semaglutide, and it's usually strongest in the first weeks. It often eases with time and simple changes. Here's why it happens, how long it lasts, and what helps."
Problem-empathy	Name the worry → reassure → promise	"Feeling queasy after your first dose can be worrying. It's common, and there's a lot you can do. This guide covers..."
Question hook (hypophora)	Reader's question → answer	"Can you drink alcohol on semaglutide? Many people can in moderation, but it may make side effects worse..."
Surprising fact	Sourced fact → why it matters → promise	"About 2 in 5 U.S. adults have obesity (CDC). If you're one of them, you're far from alone. Here's how medical weight care works."

Every intro: primary keyword in the first 100 words, no filler, under ~100 words. 🔵

29.2 The key-points box (right after the intro)

Rule	Example
3–5 bullets	—
Each bullet is a complete, useful fact	"Nausea is often strongest when starting or increasing a dose."
Includes the safety point	"Call your provider for severe or ongoing stomach pain."
No marketing bullets	❌ "Ongo Weight Loss is the best choice."

> 🍛 **Desi analogy: restaurant ka starter**
>
> Achha starter bhookh jagata hai aur batata hai ki main course kaisa hoga. Agar starter aadha ghanta late aaye ya bekaar ho, log menu dekhna band kar dete hain. **Intro starter hai, key points thali ka preview.**

29.3 Conclusion formulas

Formula	Pattern	Example
Bottom line	3–4 sentence summary + next step	"Nausea on semaglutide is common and often temporary. Smaller meals, slower eating and avoiding greasy foods can help. If it's severe or doesn't improve, contact your provider."
Decision helper	Summarise options → who each suits → next step	For comparison pages
What next	Link to the next funnel step (Chapter 11)	"Next: what to eat in your first month"

Don't: start with "In conclusion", repeat the whole article, or add new claims at the end.

29.4 Business mention: natural, not pushy

From our guidelines: highlight how Ongo Weight Loss helps **naturally**, without a formal "Why choose us" heading on informational pages.

Funnel	Example
TOFU	No brand push; link to a deeper guide
MOFU	"If you're considering treatment, an Ongo Weight Loss provider can review your health history and talk through options."
BOFU	Clear CTA block (Chapter 32)

29.5 The end-of-article stack

Order	Element
1	Bottom line / conclusion
2	Soft or clear CTA (by funnel)
3	FAQs (Chapter 31)
4	Disclaimer
5	Sources
6	Author / reviewer box (if not at top) and related articles

> 🟧 **Ongo Weight Loss example**
>
> Our homepage intro line "Losing stubborn weight is possible with the right medical support" is a soft promise. An answer-first alternative: "Ongo Weight Loss connects you with licensed providers online. If treatment is right for you, compounded semaglutide or tirzepatide (not FDA-approved) ships to you, for one all-in price." 🔵

29.6 Do's and Don'ts

✅ Do	❌ Don't
Answer or empathise in sentence one	Start with filler or history
Add a 3–5-bullet key-points box	Put marketing in key points

✅ Do	❌ Don't
End with a clear next step	End with "In conclusion..." and repetition
Keep the business mention natural	Add hard CTAs to TOFU articles

29.7 Pros and cons of formula intros

✅ Pros	❌ Cons
Faster drafting	Can feel samey across pages
Readers instantly oriented	Need variation to stay fresh
Snippet-friendly	Must stay accurate

29.8 Where freshers go wrong in Chapter 29 topics

Mistake	Why it happens	Avoid it by	Fix it by
200-word intro	Warming up	100-word limit	Cutting to the answer
Key points that tease ("Find out the #1 tip!")	Clickbait habit	Complete-fact rule	Stating the tip
New claim in the conclusion	Last-minute idea	Stack order	Moving it into the body (and claim log)

29.9 Thumb rules

> 👍 First sentence answers or empathises.
>
> 👍 Key points are real facts, not teasers.
>
> 👍 End with one clear next step.

29.10 Notes to remember

> 📝 Intro: under ~100 words, keyword in first 100 words.
>
> 📝 End stack: bottom line → CTA → FAQs → disclaimer → sources.

Chapter 29 Glossary

Term	Simple meaning
Intro	The opening paragraph(s)
Key points box	3–5 bullets summarising the article
Bottom line	A short concluding summary
Hook	The line that makes a reader keep reading
End stack	The ordered elements at the end of an article

Chapter 29 Quiz

1. What's the default intro formula?

- A) Answer-first
- B) Price-first
- C) Joke-first
- D) History-first

2. How many bullets in a key-points box?

- A) 20
- B) 1
- C) 3–5
- D) None

3. What should key points be?

- A) Questions only
- B) Complete, useful facts
- C) Marketing slogans
- D) Teasers

4. What's wrong with "In conclusion..."?

- A) It's required
- B) Nothing
- C) It's filler; use a real bottom line
- D) It's too short

5. Where does the primary keyword go in the intro?

- A) Nowhere
- B) In every sentence
- C) Within the first 100 words
- D) Only at the end

6. How should Ongo Weight Loss be mentioned in a MOFU article?

- A) Never
- B) Naturally, e.g., "a provider can review your health history"
- C) With a "Why we're the best" heading
- D) With countdown timers

7. What comes first in the end stack?

- A) Related articles
- B) Bottom line / conclusion
- C) Sources
- D) Disclaimer

8. Can new claims appear in the conclusion?

- A) No; claims belong in the body and claim log
- B) Yes
- C) Only big ones
- D) Only without sources

9. Which intro uses hypophora?

- A) "In today's world..."
- B) "Weight loss is a journey..."
- C) "Can you drink alcohol on semaglutide? Many people can in moderation..."
- D) "Let's dive in!"

10. In the restaurant analogy, the intro is...

- A) The dessert
- B) The parking
- C) The bill
- D) The starter that previews the meal

Sources

1. Ro, Semaglutide nausea guide (updated August 24, 2026)
2. CDC, Adult Obesity Facts
3. Ongo Weight Loss, writing guidelines and homepage review, October 2026 (internal)

Answer key, Chapter 29: 1-A · 2-C · 3-B · 4-C · 5-C · 6-B · 7-B · 8-A · 9-C · 10-D

LEVEL 3 · CHAPTER 30

Chapter 30: Scannability: Headings, Lists, Tables and Bold

Most people don't read web pages word by word; they scan. Good formatting lets them find their answer in seconds, then read the part that matters to them.

> 💡 **Trivia**
>
> NIH's Clear & Simple guide recommends using headings and breaking text into short, ordered chunks. [1] ● Official (NIH)
>
> Formatting isn't decoration. It's part of being understood.

30.1 How people scan

People often scan in an **F-shape**: across the top, down the left side, and across again when something catches their eye. So:

Put here	Why
The answer at the top	First line read
Key words at the **start** of headings, bullets and paragraphs	Left side is scanned
Bold on 1–3 key phrases per section	Catches the eye on the way down

30.2 Formatting tools and when to use them

Tool	Use for	Don't use for
Headings (H2/H3)	Every new question or section	Decoration or big text
Short paragraphs (1–4 sentences)	All body text	10-line blocks

Tool	Use for	Don't use for
Bulleted list	3+ items without order	Two items, or full arguments
Numbered list	Steps or ranked items	Unordered ideas
Table	Comparing options, schedules, specs	One-dimensional lists
Bold	Key takeaways and warnings	Whole paragraphs or every keyword
Callout box	Safety notes, tips, "did you know" facts	Ads
Horizontal line	Separating major sections (per design)	Between every paragraph
Images/infographics	Explaining steps or concepts	Filler stock photos

> 🍛 **Desi analogy: tiffin ke dabbe**
>
> Tiffin mein dal, sabzi, roti, chawal alag-alag dabbon mein hote hain: jo chahiye, turant mil jaata hai. Sab ek dabbe mein daal do toh khichdi ban jaati hai. **Formatting tiffin ke dabbe hain.**

30.3 Writing good bullets

Rule	❌ Weak	✅ Strong
Start with the key word	"It's a good idea to eat smaller meals"	"**Smaller meals** can ease nausea"
Parallel structure	"Eat slowly", "Hydration is important", "You should avoid grease"	"Eat slowly", "Sip water", "Avoid greasy foods"
One idea per bullet	Two tips in one bullet	Split them
Keep them short	3 lines each	1 line, 2 max

30.4 Writing good tables

Rule	Why
Clear column headers	Readers know what they're comparing
2–5 columns on mobile-friendly pages	Wide tables break on phones
Same type of info in each column	Easy scanning
Include limits ("Not FDA-approved" row)	Honest comparison
Short cells	Tables are for scanning

Example: compounded vs brand (education page, reviewed)

	Compounded semaglutide (Ongo Weight Loss)	Brand-name semaglutide
FDA-approved?	No	Yes (for specific uses)
Who makes it	Licensed U.S. compounding pharmacy	Manufacturer
Prescribed by	Licensed provider, when appropriate	Licensed provider, when appropriate
Studied in large trials?	Not these products	Yes (e.g., STEP trials)

(Ongo Weight Loss does not offer brand-name medications.)

30.5 Image suggestions

Writers suggest images; designers create them.

Suggest	Avoid
Step diagrams, timelines, comparison graphics	Before/after bodies
Food and recipe photos	Syringe close-ups on general pages
Provider photos (real, consented)	Stock "doctors" presented as ours
Infographics with sources	Unsourced statistics graphics

Write the alt text with each suggestion (Chapter 36).

Image brief for the graphic designer or UI/UX designer (one per image, in the Google Doc where the image goes):

Field	Example
Purpose	Show the first-week routine at a glance
Type	Infographic / table graphic / photo / diagram
Content	4 steps: smaller meals · sip water · avoid greasy foods · call if severe
Placement	After H2 "What helps with semaglutide nausea?"
Alt text	"Infographic: four ways to ease nausea on semaglutide"
Caption (optional)	"Simple steps that help many people in the first weeks."
Source (if data)	Link from the claim log
Must avoid	Before/after bodies, brand drug packaging, real patients without consent

Designers set file format, size and dimensions per Book Four. 🔵

> 🟧 **Ongo Weight Loss example**
>
> Our dosing guides list "Vial Sizes, Weekly Doses, Titration" in the title. Inside the page, a **table** (week · dose · amount to draw) checked by the medical reviewer would be far easier to scan than paragraphs, as long as it says "follow your provider's instructions". 🔵

30.6 Do's and Don'ts

✅ Do	❌ Don't
Keep paragraphs to 1–4 sentences	Write walls of text
Use tables for comparisons	Explain comparisons in long prose
Bold sparingly	Bold every keyword
Suggest useful visuals with alt text	Suggest filler stock photos

30.7 Pros and cons of heavy formatting

✅ Pros	❌ Cons
Faster understanding	Overuse breaks the reading flow
Better on mobile	Too many boxes look like ads
Helps snippets and AI answers	Tables need mobile design checks

30.8 Where freshers go wrong in Chapter 30 topics

Mistake	Why it happens	Avoid it by	Fix it by
Bolding every keyword	Old SEO advice	1–3 per section rule	Removing extra bold
Bullets with full paragraphs	Not trimming	Bullet rules	Splitting or shortening
8-column tables	Too much data	Column limit	Splitting the table

30.9 Thumb rules

👍 Scan-proof it: could someone find the answer in 10 seconds?

👍 Lists for items, numbers for steps, tables for comparisons.

👍 Bold is a highlighter, not a paint roller.

30.10 Notes to remember

📝 Paragraphs: 1–4 sentences.

📝 Every comparison table includes limits and our "does not offer" note where brands appear.

Chapter 30 Glossary

Term	Simple meaning
Scannability	How easily readers can find information by skimming
F-pattern	A common eye-scanning shape on web pages
Parallel structure	List items written in the same grammatical form
Callout box	A highlighted box for tips or warnings
Infographic	A visual that explains information

Chapter 30 Quiz

1. How do many people read web pages?

- A) They scan, often in an F-shape
- B) Only images
- C) Every word, slowly
- D) Bottom to top

2. When do you use a numbered list?

- A) For unordered ideas
- B) For steps or ranked items
- C) For one item
- D) Never

3. How many bold phrases per section?

- A) Every keyword
- B) About 1–3
- C) Whole paragraphs
- D) None ever

4. What's parallel structure?

- A) Two columns of text
- B) Side-by-side images

- C) Repeating keywords
- D) List items in the same grammatical form

5. Which is the strong bullet?

- A) "Eat smaller meals and also drink water and avoid grease"
- B) "Smaller meals can ease nausea"
- C) "Meals"
- D) "It's a good idea to maybe think about eating smaller meals"

6. What should a comparison table include?

- A) Prices from memory
- B) Only benefits
- C) Competitor logos
- D) Limits, such as "Not FDA-approved"

7. How long should paragraphs be?

- A) 1–4 sentences
- B) A whole page
- C) One word
- D) 15 sentences

8. Which image should writers NOT suggest?

- A) Sourced infographics
- B) Food photos
- C) Before/after bodies
- D) Step diagrams

9. Why use tables?

- A) To hide text
- B) To add keywords
- C) To compare options, schedules or specs clearly
- D) To decorate

10. In the tiffin analogy, formatting is...

- A) Separate boxes so you find what you need
- B) Eating fast

- C) Mixing everything together
- D) Throwing food away

Sources

1. NIH, Clear & Simple
2. Ongo Weight Loss, blog listing, viewed 30 October 2026

Answer key, Chapter 30: 1-A · 2-B · 3-B · 4-D · 5-B · 6-D · 7-A · 8-C · 9-C · 10-A

LEVEL 3 · CHAPTER 31

Chapter 31: Writing for AI Answers, Snippets and FAQs

More and more people get their first answer from Google's AI Overviews, featured snippets, ChatGPT or Gemini. Clear, well-structured, trustworthy writing is what these systems quote.

> 💡 **Trivia**
>
> Google says its guidance for helpful, reliable content applies across Search, and it gives more weight to trust for YMYL topics like health. [1] ● Official (Google)
>
> There's no secret "AI format". The pages AI tools quote are usually the ones that answer clearly, accurately and early.

31.1 What AI answers and snippets like to quote

Pattern	Example
Definition sentence	"Titration is the process of starting a medication at a low dose and increasing it gradually."
Direct answer after a question heading	H2: "How long does semaglutide nausea last?" → 40–60-word answer
Numbered steps	"How to store compounded semaglutide: 1. ... 2. ... 3. ..." (reviewed)
Short lists	"Common side effects include: nausea, diarrhea, constipation, vomiting."
Clear tables	Comparison of two options
Named sources	"In the STEP 1 trial (NEJM, 2021)..."

31.2 The "quotable sentence" test

A sentence is quotable if it:

☐	Check
☐	Makes sense on its own (no "this", "it" or "as mentioned above" without context)
☐	Names the subject ("Semaglutide...", not "It...")
☐	Is accurate and qualified
☐	Is under ~30 words

❌ Not quotable	✅ Quotable
"It usually gets better after a while, as we said."	"Nausea from semaglutide often eases after the first few weeks of a new dose."

> 🪔 **Desi analogy: IT office ka handover note**
>
> Achha handover note aisa hota hai ki koi bhi naya banda padh ke samajh jaaye, bina poochhe "yeh 'woh wala issue' kaunsa hai?" **AI ke liye likha sentence bhi aisa ho:** akela padho, poora samajh aaye.

31.3 Writing great FAQs

Rule	Detail
Use real questions	From People Also Ask, support logs, provider interviews
Answer in the first sentence	Then 1–3 sentences of detail
Length	**40–80 words** per answer is usually enough; longer only if truly needed (link to a full section instead)
Don't repeat the article	FAQs cover **extra** questions or quick versions
4–8 FAQs per article	More becomes a separate FAQ page
Qualify medical answers	"Talk to your provider..."

Rule	Detail
No sales FAQs on TOFU pages	❌ "Why is Ongo Weight Loss the best?"

Note: our older guidelines suggested 200–250 words per FAQ answer. For snippet and AI eligibility and for readers, shorter, direct answers work better; use bullets only when the answer is a list. 🔵

Example FAQ:

> **Can I drink coffee while taking compounded semaglutide?** Most people can. Coffee may make nausea or reflux worse for some people, especially early in treatment, so start with smaller amounts and see how you feel. If you have questions about your situation, ask your provider.

31.4 Structured data note

FAQ and article schema are handled by SEO and developers (Book Four, Chapter 23). Your job: write FAQs that are **visible on the page** and **exactly match** what goes into schema. Never write hidden FAQs just for markup. 🔵

31.5 Trust signals AI and readers look for

Signal	Where
Author and medical reviewer with credentials	Top of article
Dates (published, last medically reviewed)	Top of article
Sources linked	Bottom + in-line
Clear company info	About, editorial policy, contact
Consistent facts across pages	Price, process, medication wording

> 🟧 **Ongo Weight Loss example**
>
> Our blog listing page had `noindex, nofollow` on 30 October 2026, and posts show no reviewer or dates. AI systems and search engines may struggle to find or trust those pages. The writer's part: add answer blocks, quotable definitions, reviewer and date details; the SEO team handles indexing. 🔵

31.6 Do's and Don'ts

✅ Do	❌ Don't
Write sentences that stand alone	Rely on "as mentioned above"
Answer FAQs in the first sentence	Write 250-word FAQ essays by default
Keep FAQs visible and matching schema	Hide FAQs for markup only
Name sources and dates	Write unsourced "facts"

31.7 Pros and cons of writing for AI answers

✅ Pros	❌ Cons
Visibility in AI Overviews and assistants	Some people get the answer without clicking
Same techniques help human readers	Formats keep changing
Rewards accuracy and structure	Hard to measure directly

31.8 Where freshers go wrong in Chapter 31 topics

Mistake	Why it happens	Avoid it by	Fix it by
FAQ answers that repeat the article	Filling space	"Extra questions" rule	New questions only
"It" and "this" everywhere	Natural speech	Quotable test	Naming the subject

Mistake	Why it happens	Avoid it by	Fix it by
Sales FAQs on informational pages	Conversion pressure	Funnel rules	Removing

31.9 Thumb rules

👍 Every key sentence should make sense on its own.

👍 FAQ: answer first, 40–80 words.

👍 Visible on page = what's in the schema.

31.10 Notes to remember

📝 4–8 FAQs per article; more becomes an FAQ page.

📝 There's no secret AI format: clear, accurate and early wins.

Chapter 31 Glossary

Term	Simple meaning
AI Overview	Google's AI-generated summary at the top of some results
Featured snippet	A highlighted answer box in Google results
Quotable sentence	A sentence that makes sense on its own
FAQ schema	Code describing FAQs for search engines
Stand-alone sentence	A sentence that needs no surrounding context

Chapter 31 Quiz

1. What makes a sentence quotable?

- A) It uses "it" and "this"
- B) It makes sense on its own and names the subject
- C) It has no subject
- D) It's 80 words long

2. How long should most FAQ answers be?

- A) 250 words always
- B) 5 words
- C) 1,000 words
- D) About 40–80 words

3. Should FAQs repeat the article?

- A) Only the intro
- B) No; cover extra or quick-answer questions
- C) Only the sources
- D) Yes, word for word

4. Can FAQs be hidden just for schema?

- A) Yes
- B) No; visible FAQs must match schema
- C) Only on TOFU pages
- D) Only on mobile

5. How many FAQs per article?

- A) 4–8
- B) 30
- C) 1
- D) 0

6. Which is a quotable definition?

- A) "It's that thing we mentioned."
- B) "Titration is the process of starting a medication at a low dose and increasing it gradually."
- C) "As said above, it works."
- D) "This is important."

7. Is there a secret AI-only format?

- A) Yes, hidden text
- B) No; clear, accurate, early answers win
- C) Yes, all caps
- D) Yes, special codes

8. Which FAQ is wrong for a TOFU page?

- A) "Why is Ongo Weight Loss the best?"
- B) "What is titration?"
- C) "How long does nausea last?"
- D) "Can I drink coffee on semaglutide?"

9. Who handles FAQ schema?

- A) Nobody
- B) Graphic designer
- C) SEO and developers (writers supply matching visible FAQs)
- D) Social team

10. In the handover-note analogy, a good sentence…

- A) Needs the previous page
- B) Is blank
- C) Uses inside jokes
- D) Makes sense to anyone reading it alone

Sources

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, blog listing, viewed 30 October 2026, and Book Four (schema), internal 🔵

Answer key, Chapter 31: 1-B · 2-D · 3-B · 4-B · 5-A · 6-B · 7-B · 8-A · 9-C · 10-D

LEVEL 3 · CHAPTER 32

Chapter 32: CTAs and Microcopy: Buttons, Forms and Errors

The smallest words on the page do the most work: buttons, form labels, help text and error messages. Clear microcopy reduces anxiety and helps people take the next step.

> 💡 **Trivia**
>
> Competitor CTAs observed on 30 October 2026: Ro: "See if you qualify"; Hims: "Start the journey"; Mochi: "Check my coverage" and "I'll pay myself"; Fridays: "Take the eligibility quiz →". [1–4] 🟠 Industry
>
> The best CTAs say exactly what happens next, in the reader's own words.

32.1 What makes a good CTA

Quality	❌ Weak	✅ Strong
Specific	"Submit", "Click here"	"Check my eligibility"
Honest about what happens	"Get your prescription"	"Start my free assessment"
Low pressure	"Buy now before it's gone!"	"See if it's right for me"
First person (often)	"Check your eligibility"	"Check my eligibility" (test both)
Expectation set	"Get started"	"Get started: about 5 minutes" (if true)

Never promise a prescription, approval or result in a button. 🔵

32.2 CTAs by funnel stage (with Book Four event names)

Stage	CTA example	Event (Book Four)
TOFU	"Read: How GLP-1 medications work"	`internal_link_click`
MOFU	"See how our program works"	`cta_[pagegroup]_how_it_works`
BOFU	"Check my eligibility"	`cta_[pagegroup]_get_started`
Patient support	"Message my care team"	`exit_to_portal`

Use the **exact approved CTA wording** in the brief so events and reports stay consistent. 🔵

> 🍛 **Desi analogy: railway station ke board**
>
> Station pe "Platform 3: Rajdhani" saaf likha ho toh log seedha pahunchte hain. Agar board pe sirf "Yahan jaao" likha ho, toh sab confused. **Button bhi board hai:** batao kahan le jaayega.

32.3 Microcopy types

Type	Purpose	Example
Form labels	Name the field	"Date of birth"
Help text	Explain why or how	"We ask this to check if treatment is safe for you."
Placeholder	Example format (not a label)	"MM/DD/YYYY"
Error messages	Say what's wrong and how to fix it	"Please enter your date of birth as MM/DD/YYYY."
Empty states	When there's nothing yet	"No messages yet. Your care team will reply here."
Confirmation	Reassure after an action	"Thanks! A provider will review your answers. We'll email you within [timeframe]."
Tooltips	Short extra info	"BMI: a screening measure of weight for height."
Consent text	Clear, legal-approved	Written with legal

32.4 Error message formula

What happened + how to fix it + (optional) reassurance. No blame, no jargon.

❌ Bad	✅ Good
"Invalid input."	"Please enter a valid email address, like name@example.com."
"Error 422."	"Something went wrong on our side. Please try again, or contact us at [contact]."
"You are not eligible."	"Based on your answers, this program may not be the right fit. [Next step: talk to your doctor / contact us]." (legal and medical approve)

32.5 Sensitive microcopy (health)

Situation	Guidance
Asking for weight/height	Explain why: "We use this to calculate BMI, one of the factors providers consider."
Medical history questions	Calm, neutral; "This helps your provider make a safe decision."
Not eligible results	Kind, clear, with a next step; reviewed by medical and legal
Payment	Exact amount, what's included, when charged

> 🟧 **Ongo Weight Loss example**
>
> Our homepage uses several CTA variants: "See If I'm Eligible", "Check My Eligibility", "Take free assessment", "Explore Weight Loss Plans". If they all go to the same place, pick **one** primary wording so readers (and GA4 reports) aren't confused; keep a different label only when the destination is different. 🔵

32.6 Do's and Don'ts

✅ Do	❌ Don't
Say exactly what happens next	Use "Submit" or "Click here"

✅ Do	❌ Don't
Explain why you ask sensitive questions	Ask for health data without context
Write kind, fixable error messages	Blame the user
Use one wording per destination	Use five labels for one action

32.7 Pros and cons of investing in microcopy

✅ Pros	❌ Cons
Fewer drop-offs and support tickets	Needs close work with design and dev
Builds trust at anxious moments	Legal review for consent/payment text
Cleaner analytics	Changes must be coordinated

32.8 Where freshers go wrong in Chapter 32 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Get your prescription" button	Wanting excitement	No-promise rule	"Start my free assessment"
"Invalid" errors	Default dev text	Error formula	Rewriting with a fix
Many CTA labels for one action	Different writers	One-wording rule	Standardising

32.9 Thumb rules

👍 Buttons say what happens next.

👍 Errors explain and fix; they never blame.

👍 One action, one label.

32.10 Notes to remember

📝 Never promise approval, prescriptions or results in a CTA.

📝 Match CTA wording to the Book Four event names.

Chapter 32 Glossary

Term	Simple meaning
CTA	Call to action: what you ask the reader to do
Microcopy	Small interface text (buttons, labels, errors)
Help text	A short explanation under a form field
Empty state	What a screen shows when there's no content yet
Error message	Text explaining what went wrong and how to fix it

Chapter 32 Quiz

1. What should a CTA say?

- A) "Click here"
- B) Exactly what happens next
- C) "Submit"
- D) Nothing

2. Which CTA is acceptable?

- A) "Start my free assessment"
- B) "Guaranteed approval"
- C) "Get your prescription now"
- D) "Buy before it's gone"

3. What's the error message formula?

- A) What happened + how to fix it + optional reassurance
- B) Blame the user
- C) Leave it blank
- D) Error code only

4. Why explain sensitive questions?

- A) It adds keywords
- B) It's not needed
- C) It reduces anxiety and builds trust
- D) It's decorative

5. Which event tracks BOFU CTA clicks?

- A) `page_not_found`
- B) `scroll`
- C) `cta_[pagegroup]_get_started`
- D) `video_start`

6. What's a placeholder for?

- A) The label itself
- B) Legal text
- C) A CTA
- D) Showing an example format, not replacing a label

7. Who approves "not eligible" messages?

- A) The social team
- B) Nobody
- C) Medical and legal reviewers
- D) A random user

8. What's wrong with five CTA labels for one action?

- A) Nothing
- B) It improves SEO
- C) It's required
- D) Confuses readers and reports

9. Which error message is good?

- A) "Wrong!"
- B) "Invalid input."
- C) "Error 422."
- D) "Please enter a valid email address, like name@example.com."

10. In the railway-board analogy, a button should…

- A) Be hidden
- B) Say clearly where it takes you
- C) Say only "go here"
- D) Change every day

Sources

1. Ro, Weight loss, viewed 30 October 2026 🟠
2. Hims, Weight loss, viewed 30 October 2026 🟠
3. Mochi Health, homepage, viewed 30 October 2026 🟠
4. Fridays, homepage, viewed 30 October 2026 🟠
5. Ongo Weight Loss, homepage review and Book Four event dictionary, internal 🔵

Answer key, Chapter 32: 1-B · 2-A · 3-A · 4-C · 5-C · 6-D · 7-C · 8-D · 9-D · 10-B

04

LEVEL 4

Page Types, Review and Growth

Money, blog and trust pages, short formats, self-editing, fact-checking, review, SEO handoff, accessibility, tools, measurement and ZETA.

33 · Writing Money Pages: Service, Medication, Cost, How It Works, Landing

34 · Writing Blog Content: Guides, Comparisons, Lists and More

35 · Writing Trust Pages: Profiles, Policies, Testimonials and Reports

36 · Short Formats: Meta, Alt Text, Emails, Captions and Video Scripts

37 · Self-Editing: Three Passes and the Checklist Tests

38 · Fact-Checking, Citations and Honest Dates

39 · Medical and Legal Review: Submitting and Responding

40 · SEO Handoff and Internal Linking for Writers

41 · Accessible and Inclusive Writing

42 · The Writer's Toolkit: Free, Freemium and Paid

43 · Measuring, Refreshing and Your 30/60/90-Day Plan

44 · The ZETA Model for Writers and the Final Gate

LEVEL 4 · CHAPTER 33

Chapter 33: Writing Money Pages: Service, Medication, Cost, How It Works, Landing

Money pages are where people decide. They need the clearest writing, the most careful claims and the strongest trust signals on the site.

> 💡 **Trivia**
>
> Mochi's homepage headline is seven words: "Weight loss medication, prescribed online." Its compounded products are labelled "compounded" and sit next to the disclaimer that compounded medications "are not approved or evaluated by the FDA". [1] ● Industry
>
> On money pages, clarity and honesty sit side by side.

33.1 Service page skeleton

Section	What to write	Example
H1 + subhead	What it is, for whom	"Online weight care with licensed providers" / "Self-pay. No insurance needed."
Answer block	What you get + key disclaimer	"If a provider decides it's right for you, compounded semaglutide or tirzepatide ships to your door. Compounded medications are not FDA-approved."
Primary CTA	Next step	"Check my eligibility"
How it works	3–5 steps (numbered)	Assessment → provider review → prescription if appropriate → delivery → ongoing support
What's included	FAB list (Chapter 24)	Visits, medication, shipping, messaging

Section	What to write	Example
Price	Exact, matching the site	"[Price as on site]" + what's included
Who it's for / not for	Honest eligibility	"Not for people who are pregnant..." (medical review)
Safety	Side effects + red flags	Plain language
Providers	Names, credentials, photos	Real, consented
Trust	LegitScript, HIPAA practices, real reviews	Sourced only
FAQs	6–10 real questions	Chapter 31
Final CTA + disclaimer	—	—

33.2 Medication page skeleton (e.g., compounded semaglutide)

Section	Must include
H1	"Compounded semaglutide" (no brand names)
What it is	Plain mechanism; "compounded"; "not FDA-approved"
How it's prescribed	Provider decides; titration idea
How you take it	Weekly injection; how-to guide link (reviewed)
Side effects and safety	Common effects; red flags; who should not use it (medical review)
What results can look like	Brand trial results named + "compounded versions weren't studied"; "results vary"
Price and what's included	Matches the site
Pharmacy	"A licensed U.S. compounding pharmacy" (never the name)

Section	Must include
FAQs, disclaimer, sources	—

> 🪔 **Desi analogy: sunar ki dukaan**
>
> Achha sunar har gehne pe purity, wazan aur daam saaf likhta hai. Jo "sabse sasta, sabse shuddh" chillaata hai par bill mein chhupa charge daalta hai, uske paas koi wapas nahi jaata. **Money page sunar ki rate-list jaisi saaf honi chahiye.**

33.3 Cost page skeleton

Section	Rule
Price first	"[Exact price]", per month, as on the site
What's included	Line by line
What's not included	Be explicit (e.g., labs, if applicable)
How billing works	When charged, cancellations, refunds (link policy)
Why compounded can cost less	Factual, reviewed; no "cheaper than [brand]"
Comparisons	Only with dated, sourced, legal-approved data
FAQs	Insurance (self-pay), HSA/FSA (if true), changes

33.4 How-it-works page skeleton

Step	Write
1. Assessment	What you'll be asked and why; how long it takes
2. Provider review	Who reviews, how long, what they decide
3. Prescription (if appropriate)	"Not guaranteed"
4. Delivery	Timeframe exactly as operations confirm

Step	Write
5. Ongoing care	Check-ins, messaging, dose changes by provider

33.5 Landing page skeleton (one goal)

Element	Rule
One audience, one message, one CTA	No navigation clutter
AIDA order	Chapter 24
Proof	Real, sourced trust signals
Disclaimers	Next to claims
Message match	The ad/email promise = the page headline

> 🟧 **Ongo Weight Loss example**
>
> Homepage findings to fix with these skeletons: (1) move "Compounded... not FDA-approved" next to product cards; (2) explain the price range and what each plan covers; (3) clarify "Low Dose GLP-1"; (4) source or remove "4.9/5 Ratings" and "Trusted by 11945+ people"; (5) reconcile delivery timing wording. 🔵

33.6 Do's and Don'ts

✅ Do	❌ Don't
Put price, inclusions and disclaimers near the top	Hide fees or disclaimers in footers
Use one primary CTA wording	Use five variants
Source every trust number	Show unsourced ratings or counts
Get medical + legal review	Publish money pages without review

33.7 Pros and cons of strict money-page skeletons

✅ Pros	❌ Cons
Faster builds, consistent pages	Less design freedom
Fewer compliance gaps	Needs updates when offers change
Easier testing (Chapter 43)	—

33.8 Where freshers go wrong in Chapter 33 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Cheaper than Wegovy"	Wanting a hook	Cost skeleton rule	Factual inclusions instead
Missing "who it's not for"	Fear of losing sign-ups	Service skeleton	Adding (reviewed)
Delivery promise ops can't meet	Guessing	Confirm with ops	Correcting

33.9 Thumb rules

👍 Price, inclusions, disclaimers: up top.

👍 Every number on a money page has a source or goes.

👍 One page, one main action.

33.10 Notes to remember

📝 Money pages always get medical + legal review.

📝 Medication pages: no brand names, "compounded", "not FDA-approved", "provider decides".

Chapter 33 Glossary

Term	Simple meaning
Service page	A page describing a service and how to start
Landing page	A focused page with one goal, often for campaigns
Message match	The page headline matches the ad or email promise
Inclusions	What the price covers
Trust signal	Evidence that builds confidence (certifications, reviews)

Chapter 33 Quiz

1. Where do disclaimers go on money pages?

- A) In images only
- B) Only in the footer
- C) Nowhere
- D) Next to the claims

2. What's the H1 for the semaglutide medication page?

- A) "Generic Wegovy"
- B) "Ozempic for less"
- C) "Compounded semaglutide"
- D) "Miracle shot"

3. What must the cost page show first?

- A) Company history
- B) A countdown
- C) Testimonials
- D) The exact price matching the site

4. How is the pharmacy described?

- A) By its brand name
- B) "A licensed U.S. compounding pharmacy"
- C) "Secret lab"
- D) "FDA-approved pharmacy"

5. How many main actions on a landing page?

- A) Ten
- B) None
- C) Five
- D) One

6. What's "message match"?

- A) Matching fonts
- B) Matching colours
- C) Matching competitors
- D) The page headline matches the ad/email promise

7. Which trust number is acceptable?

- A) An invented count
- B) "Trusted by millions" (unverified)
- C) A sourced rating with review count and platform
- D) "4.9/5" with no source

8. Should "who it's not for" be included?

- A) Only on social
- B) No, it reduces sign-ups
- C) Only in footnotes
- D) Yes, reviewed by medical

9. Who confirms delivery timeframes?

- A) The writer's guess
- B) AI
- C) Operations
- D) Competitors

10. In the sunar analogy, a money page should be…

- A) As clear as a jeweller's rate list
- B) Full of hidden charges
- C) Loud and vague
- D) Empty

Sources

1. Mochi Health, homepage, viewed 30 October 2026 🟠
2. Ongo Weight Loss, homepage review, 30 October 2026, and Books Three–Four (page templates), internal 🔵

Answer key, Chapter 33: 1-D · 2-C · 3-D · 4-B · 5-D · 6-D · 7-C · 8-D · 9-C · 10-A

LEVEL 4 · CHAPTER 34

Chapter 34: Writing Blog Content: Guides, Comparisons, Lists and More

Each blog type has a shape readers expect. Use the right skeleton and you'll write faster, rank better and help more.

> 💡 **Trivia**
>
> WeightWatchers' article titles are practical, everyday questions: "How often should you weigh yourself?", "What is food noise–and how can you quiet it?". Noom adds myth-busting: "4 weight-loss myths to leave behind in 2026". [1][2] ● Industry
>
> The best weight-care blogs answer daily-life questions, not just medication questions.

34.1 Blog standard elements (every post)

Element	Rule
H1	Question or topic-benefit title (Chapter 28)
Trust box	Author, medical reviewer, published + last medically reviewed dates
Intro	Answer-first, under ~100 words
Key points	3–5 bullets
Body	Question H2s with answer blocks
Information gain	At least one (Chapter 26)
Safety section	For any medical topic
Bottom line + CTA by funnel	Chapter 29

Element	Rule
FAQs (4–8)	Chapter 31
Disclaimer + sources	Chapter 4, 38
Image suggestions + alt text	Chapter 36

Length: whatever fully answers the question (Goldilocks). Our older guideline of 700–1,000 words is a **starting estimate**, not a target. 🔵

34.2 Skeletons by type

Guide (step-by-step education)

Intro → key points → "What is…" → "Why it matters" → step-by-step H2s → common mistakes → when to call your provider → bottom line → FAQs.

Tutorial / how-to

What you'll need → numbered steps (one action each) → tips → safety notes → "follow your provider's instructions" → FAQs. Medical review required for any dosing or injection how-to.

Comparison article (X vs Y)

Answer box ("The main differences are…") → comparison **table** → what each is → key differences H2s (how it works, results in trials, side effects, cost, availability) → who might consider each (provider decides) → bottom line. Include "Ongo Weight Loss does not offer [brand]" where brands appear (reviewed education only).

Listicle

Intro → numbered items (bold lead + 2–3 sentences) → why the list works → bottom line. Every item useful; no filler to reach a round number.

Myth vs fact

Intro → each myth as an H2 ("Myth: You can't eat carbs on GLP-1s") → "Fact:" answer with source → bottom line. Never repeat a myth in a way that could be quoted as true without the fact.

Checklist

Purpose → ☐ items grouped by stage → printable version idea → when to call your provider.

Glossary / vocabulary

Alphabetical terms → one-sentence definition → "why it matters" line → link to deeper page.

Side-effect guide

What it is → why it happens → how common (sourced) → what helps → how long it lasts → red flags → when to call → FAQs.

Timeline / what to expect

Week-by-week table (reviewed, "everyone is different") → what you may notice → tips per stage → when to call.

Recipe / meal plan

Who it's for → ingredients table → steps → protein/fibre notes (sourced or dietitian-reviewed) → swaps → storage. No "guilt-free" or calorie-shaming words.

News article

5 Ws and H in the first paragraph → what it means for patients → what's not changing → source links to the original agency → date stamp. Legal review for any regulatory news.

> 🍮 **Desi analogy: mithai ki dukaan ke saanche**
>
> Halwai ke paas har mithai ka alag saancha hota hai: modak ka alag, barfi ka alag. Saancha sahi ho toh mithai sundar aur ek jaisi banti hai. **Blog skeletons hamare saanche hain.**

34.3 Blog ideas for Ongo Weight Loss (by type)

Type	Idea
Side-effect guide	Constipation on GLP-1 medications: what helps
Timeline	Your first 12 weeks on compounded semaglutide
Myth vs fact	7 GLP-1 myths, checked against research
Listicle	10 high-protein breakfasts that are easy on your stomach
Comparison	Semaglutide vs tirzepatide: how they compare (reviewed)
Checklist	12 questions to ask your provider before starting

Type	Idea
Glossary	GLP-1 terms explained in plain English
Guide	Strength training while losing weight: a beginner's guide

Each needs a unique keyword and a place in the Book Three map before writing. 🔵

> 🟧 **Ongo Weight Loss example**
>
> All five current blog posts are medication explainers or dosing guides. Competitors win attention with everyday questions (food, plateaus, myths, weighing yourself). Adding these types builds the TOFU and MOFU layers our funnel needs. 🔵

34.4 Do's and Don'ts

✅ Do	❌ Don't
Use the skeleton for the type	Freestyle every post
Include the trust box and dates	Publish anonymous posts
Write about daily life, not just drugs	Only write medication explainers
Keep lists honest (no filler items)	Pad to "Top 10"

34.5 Pros and cons of a varied blog

✅ Pros	❌ Cons
Reaches people earlier in their journey	More topics to research and review
More internal-link paths	Needs a content calendar
More social content (Book Five)	Recipes and fitness need expert review

34.6 Where freshers go wrong in Chapter 34 topics

Mistake	Why it happens	Avoid it by	Fix it by
Comparison with no table	Writing habit	Comparison skeleton	Adding a table
Myth repeated without the fact nearby	Structure slip	Myth-fact pairing	Moving the fact up
Dosing tutorial without review	Seems factual	Review rule	Sending to medical

34.7 Thumb rules

👍 Right type, right skeleton.

👍 Everyday questions build trust before medication questions.

👍 Every blog has a trust box and dates.

34.8 Notes to remember

📝 Word counts are estimates, not targets.

📝 Dosing/injection tutorials always need medical review.

Chapter 34 Glossary

Term	Simple meaning
Skeleton	A standard structure for a content type
Listicle	An article built around a numbered list
Myth vs fact	An article correcting common false beliefs
Timeline article	A "what to expect" week-by-week guide
Trust box	Author, reviewer and date block

Chapter 34 Quiz

1. What must every blog post include?

- A) Five CTAs
- B) Trust box with author, reviewer and dates
- C) Competitor links
- D) A countdown timer

2. What's the core element of a comparison article?

- A) A video only
- B) A long story
- C) Brand ads
- D) A comparison table

3. Is 700–1,000 words a strict target?

- A) Yes, always
- B) Minimum 3,000
- C) No; it's an estimate; write what fully answers
- D) Exactly 1,000

4. What does a news article start with?

- A) The 5 Ws and H
- B) A joke
- C) A CTA
- D) Company history

5. Which words are banned in recipe posts?

- A) "Ingredients"
- B) "Protein"
- C) "Guilt-free" and calorie-shaming words
- D) "Steps"

6. What do dosing tutorials need?

- A) Nothing
- B) Medical review

- C) Only SEO review
- D) Only design

7. In myth vs fact, where does the fact go?

- A) At the end of the page
- B) In a footnote
- C) Nowhere
- D) Right with the myth

8. What everyday topics do competitors use?

- A) Only celebrity news
- B) Food, plateaus, myths, weighing yourself
- C) Only legal updates
- D) Only drug prices

9. What does each new blog post need before writing?

- A) A competitor's draft
- B) A countdown
- C) An AI rewrite
- D) A unique keyword and a place in the content map

10. In the mithai analogy, skeletons are...

- A) Moulds that make consistent results
- B) Price tags
- C) Extra sugar
- D) Wrapping paper

Sources

1. WeightWatchers, Blog, viewed 30 October 2026
2. Noom, Blog, viewed 30 October 2026
3. Ongo Weight Loss, blog listing, viewed 30 October 2026

Answer key, Chapter 34: 1-B · 2-D · 3-C · 4-A · 5-C · 6-B · 7-D · 8-B · 9-D · 10-A

LEVEL 4 · CHAPTER 35

Chapter 35: Writing Trust Pages: Profiles, Policies, Testimonials and Reports

Trust pages answer "Who are you, and why should I believe you?" They're often ignored by writers, yet they're the pages raters and careful readers check first.

💡 **Trivia**

Google's rater guidelines say YMYL pages or websites "that handle sensitive data with absolutely no information about the website or content creator" should be rated Lowest. [1]
● Official (Google)

About pages, author bios and policies aren't paperwork. They're part of ranking and conversion.

35.1 Doctor / provider profile page

Section	Example
Name and credentials	"Johnathan Chance Miller, MD, Medical Director"
Licences and board certifications	As verified by HR/medical (states, boards)
Role at Ongo Weight Loss	"Oversees clinical protocols and medical review of content."
Experience	Years, areas of practice (factual)
Why weight care (in their words, approved)	Short quote
Content they've reviewed	Linked list
Photo	Real, consented

Rules: facts only; no "world-renowned", "best"; no specialties they don't hold; profile approved by the provider. 🔵

35.2 Author profile page

Section	Rule
Name, role	Real person
Background	Writing and health experience, honestly
What they write about	Topics
Editorial note	"All medical content is reviewed by a licensed provider."
Articles	Linked list

Never create fake authors or AI personas (Google calls invented profiles deceptive). [2] 🟢

> 🪔 **Desi analogy: mohalle ka naya doctor**
>
> Naye doctor ki dukaan ke bahar degree, registration number aur timing likhi ho, toh log bharosa karke andar aate hain. Board khaali ho toh shaq hota hai. **Profile page wahi board hai.**

35.3 Editorial policy and medical review policy

Section	Contents
Who writes	Roles and qualifications
How topics are chosen	Patient questions, provider input, search research
Sources we use	Evidence ladder (Chapter 13)
Medical review	Who reviews, what they check, how often
AI use	Honest statement of how AI is (or isn't) used
Corrections	How readers report errors; how we fix and date them
Independence	Brand mentions, no paid placements

Section	Contents
Dates	What "published", "updated" and "medically reviewed" mean

35.4 Testimonials and reviews page

Rule	Detail
Real reviews only	From verified platforms or consented patients
Show the source	"Trustpilot, [date]"; link if allowed
No cherry-picking that misleads	Don't hide that negative reviews exist
Disclose incentives	If any review was incentivised
Typicality	"Individual experiences vary."
No review suppression	FTC rule (Chapter 27)

35.5 Case studies, interviews, expert opinion, reports and whitepapers

Type	Skeleton	Key rule
Case study	STAR (Chapter 25)	Consent; no outcome promises
Interview	Intro → Q&A → takeaways	Approved transcript; credentials
Expert opinion	Expert intro → view → evidence → limits	Clearly labelled as opinion
Report	Summary → method → findings → limits → sources	State sample, dates, method
Whitepaper	Executive summary → background → evidence → implications → references	Deep sourcing; legal review

35.6 Policy pages (privacy, refunds, shipping, telehealth consent)

Rule	Why
Legal writes or approves the substance	Legal accuracy
Writer improves clarity (headings, plain words, summaries)	Readability
Add a plain-language summary box at the top (legal-approved)	Readers understand key points
Keep dates and version numbers	Accountability
Never contradict money pages	Consistency

> 🟧 **Ongo Weight Loss example**
>
> Our homepage provider bios include phrases like "Patients love him" and specialties largely outside obesity medicine. Profile pages should state verified facts (licences, roles, experience) and let the provider's approved words speak, without subjective praise. Blog posts should link to these profiles from the trust box. 🔵

35.7 Do's and Don'ts

✅ Do	❌ Don't
Verify every credential	Invent or inflate credentials
Publish editorial and medical review policies	Leave "How" invisible
Show real review sources	Show unsourced star ratings
Add plain summaries to policies (approved)	Rewrite legal meaning yourself

35.8 Pros and cons of investing in trust pages

✅ Pros	❌ Cons
Stronger E-E-A-T and conversions	Need approvals from providers and legal
Fewer "is this legit?" questions	Must be kept current
Supports every other page	Not "exciting" to write

35.9 Where freshers go wrong in Chapter 35 topics

Mistake	Why it happens	Avoid it by	Fix it by
"World-class expert" in a bio	Wanting impressiveness	Facts-only rule	Listing real credentials
Fake author persona	Thinking it's harmless	Google's deception warning	Removing
Policy rewritten "for clarity" changing meaning	Simplifying	Legal approval	Restoring and re-reviewing

35.10 Thumb rules

👍 Verified facts beat flattering adjectives.

👍 Show who, how and when on every trust page.

👍 Legal owns meaning; writers own clarity.

35.11 Notes to remember

📝 Medical Director: Johnathan Chance Miller, MD.

📝 Editorial policy includes AI use and corrections.

Chapter 35 Glossary

Term	Simple meaning
Trust page	A page that shows who we are and how we work
Editorial policy	Public rules for how content is created
Medical review policy	How and by whom medical content is checked
Credentials	Verified qualifications and licences
Whitepaper	A deep, evidence-based document

Chapter 35 Quiz

1. What do raters say about YMYL sites with no creator information?

- A) Promote
- B) Rate Lowest
- C) Rate Highest
- D) Ignore

2. What belongs in a provider profile?

- A) Verified credentials and role
- B) "World-renowned genius"
- C) Stock photos
- D) Invented specialties

3. Are AI author personas allowed?

- A) If they have nice names
- B) No; invented profiles are deceptive
- C) Yes
- D) Only for blogs

4. What should an editorial policy include?

- A) Only a logo

- B) Nothing
- C) Who writes, sources, review, AI use, corrections
- D) Prices only

5. Who owns the meaning of policy pages?

- A) AI
- B) Legal
- C) The writer alone
- D) The social team

6. What should a testimonials page show?

- A) Real reviews with sources and typicality notes
- B) AI reviews
- C) Edited reviews
- D) Only 5-star reviews with no source

7. Which skeleton fits a case study?

- A) Fake urgency
- B) AIDA
- C) None
- D) STAR

8. What should reports state?

- A) Nothing
- B) Only positives
- C) Sample, dates, method and limits
- D) Only headlines

9. Who is our Medical Director?

- A) Johnathan Chance Miller, MD
- B) A writer
- C) An AI
- D) Unknown

10. In the naya-doctor analogy, the profile page is…

- A) An advertisement

- B) A menu
- C) The board showing degrees and registration
- D) A secret

Sources

1. Google, Search Quality Rater Guidelines (September 11, 2025) 🟢
2. Google Search Central, Creating helpful, reliable, people-first content 🟢
3. Ongo Weight Loss, homepage review, 30 October 2026, and Book One (Medical Director), internal 🔵

Answer key, Chapter 35: 1-B · 2-A · 3-B · 4-C · 5-B · 6-A · 7-D · 8-C · 9-A · 10-C

LEVEL 4 · CHAPTER 36

Chapter 36: Short Formats: Meta, Alt Text, Emails, Captions and Video Scripts

Some of the most-read words you'll write are the shortest: the title in Google, the description under it, the alt text a screen reader speaks, an email subject line. Every word has to count.

> 💡 **Trivia**
>
> Google's helpful-content guidance warns against writing to a word count. That applies to short copy too: the right length is the length that's clear and complete. [1] ● Official (Google)
>
> For meta copy, Book Four's limits are guides for how much shows on screen, not targets to fill.

36.1 Meta titles and descriptions

Element	Guide (Book Four)	Formula	Example
Meta title	~50–60 characters; brand at end	Primary keyword + benefit | Brand	"Semaglutide Nausea: Causes & What Helps | Ongo Weight Loss"
Meta description	~140–160 characters	Answer or benefit + qualifier + soft CTA	"Nausea is common in the first weeks of semaglutide. Learn why it happens, how long it may last, what helps, and when to call your provider for help."
Slug	Short, lowercase, hyphens	Primary keyword	`/blog/semaglutide-nausea/`

Rules: match the page; no brand drug names on commercial pages; no promises; H1 and meta title cover the same topic (they don't have to be identical). 🔵

36.2 Image alt text

Rule	❌ Weak	✅ Strong
Describe what's in the image	"semaglutide weight loss best online"	"Pen and notebook next to a weekly medication schedule"
Be concise (about one sentence)	3 sentences	One clear line
Include the keyword only if it truly describes the image	Keyword stuffing	Natural description
Decorative images get empty alt (`alt=""`)	Describing a background swirl	Empty alt (developer sets it)
Charts: state the key point	"Chart"	"Bar chart: average weight change in STEP 1, 14.9% vs 2.4% with placebo"

Alt text is for people using screen readers first (Book Four, Chapter 13). 🔵

> 🪔 **Desi analogy: radio commentary**
>
> Radio pe cricket sunne waale ko commentator batata hai "Kohli ne cover drive maara, gend seemaa ke paar." Woh nahi kehta "achha shot, cricket, India, best." **Alt text radio commentary hai:** jo dikh raha hai, woh saaf batao.

36.3 Email copy

Element	Rule	Example
Subject line	Clear benefit, ~40 characters, no hype or ALL CAPS	"Your first-week checklist"
Preview text	Extends the subject	"What to eat, what to have at home, and when to call."

Element	Rule	Example
Opening line	Main point first	"Here's a short checklist for your first week."
Body	Short paragraphs, one main CTA	—
Compliance	Same claim rules; unsubscribe; no PHI in subject lines	—

Never put health details in subject lines ("Your semaglutide dose increase") where others may see them; keep subject lines neutral for patient emails. 🔵

36.4 Social captions (hand-off to Book Five)

Writers often supply captions. Follow Book Five: keyword-first line → value → one CTA → disclaimer if treatment → 3–5 hashtags, no brand drug hashtags.

36.5 Video scripts

Section	Time (60–90 s video)	Example
Hook	0–3 s	"Nausea in week one? Here's what helps."
Answer	3–20 s	Three tips
Detail	20–60 s	Why and when to call
CTA	Last 5–10 s	"More in our guide, link below."
On-screen disclaimer	Throughout or end	"Educational only. Talk to your provider."

Write for the ear: shorter sentences, no tables, repeat the key point once. 🔵

> 🟧 **Ongo Weight Loss example**
>
> A meta description for our semaglutide page: "Compounded semaglutide prescribed online by licensed providers when appropriate. Not FDA-approved. See pricing and how it works." It's clear, compliant, and sets expectations. 🔵

36.6 Do's and Don'ts

✅ Do	❌ Don't
Treat short copy as carefully as long copy	Paste the first draft into the meta field
Describe images for real people	Stuff keywords into alt text
Keep patient email subjects neutral	Put health details in subject lines
Write scripts for listening	Read tables aloud

36.7 Pros and cons of short-format focus

✅ Pros	❌ Cons
Better click-through and accessibility	Hard to be complete in few words
Consistent brand across channels	Easy to slip into hype
Quick wins on existing pages	Needs coordination with SEO, design, social

36.8 Where freshers go wrong in Chapter 36 topics

Mistake	Why it happens	Avoid it by	Fix it by
Alt text full of keywords	Old SEO advice	Describe-first rule	Rewriting
Meta description promising results	Wanting clicks	Claim rules	Qualifying
Health info in email subjects	Personalisation	Privacy rule	Neutral subjects

36.9 Thumb rules

👍 Short copy, full care.

👍 Alt text: say what a sighted person sees.

👍 Write scripts for the ear.

36.10 Notes to remember

📝 Meta title ~50–60 characters; description ~140–160.

📝 No health details in email subject lines.

Chapter 36 Glossary

Term	Simple meaning
Meta description	The summary under a title in search results
Slug	The end part of a URL
Alt text	Text describing an image for screen readers
Preview text	The line shown after an email subject
Hook	The first seconds or words that grab attention

Chapter 36 Quiz

1. How long is a meta description (guide)?

- A) 10 characters
- B) 500 characters
- C) About 140–160 characters
- D) Unlimited

2. What's alt text mainly for?

- A) Prices
- B) People using screen readers
- C) Keyword stuffing
- D) Decoration

3. What alt text do decorative images get?

- A) A paragraph
- B) A keyword list
- C) The brand name
- D) Empty alt

4. Which email subject is right for patients?

- A) "Your semaglutide dose increase"
- B) "Your first-week checklist"
- C) "URGENT!!! READ NOW"
- D) "Lose weight fast"

5. What's the first part of a video script?

- A) Credits
- B) Disclaimer only
- C) Company history
- D) Hook (0–3 seconds)

6. Which alt text is strong?

- A) "semaglutide weight loss best online"
- B) "image1"
- C) "Pen and notebook next to a weekly medication schedule"
- D) "click"

7. Should the H1 and meta title cover the same topic?

- A) Only on blogs
- B) Never
- C) Yes (they needn't be identical)
- D) No, totally different

8. How should chart alt text be written?

- A) Leave it empty
- B) "Chart"
- C) List all keywords
- D) State the key point the chart shows

9. What's the slug rule?

- A) Long with underscores
- B) All caps
- C) Short, lowercase, hyphens, primary keyword
- D) Random numbers

10. In the radio analogy, alt text should…

- A) Stay silent always
- B) Sell the product
- C) Describe clearly what's seen
- D) List keywords

Sources

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, Book Four (meta, alt text) and Book Five (captions), internal 🔵

Answer key, Chapter 36: 1-C · 2-B · 3-D · 4-B · 5-D · 6-C · 7-C · 8-D · 9-C · 10-C

Chapter 37: Self-Editing: Three Passes and the Checklist Tests

First drafts are for getting ideas down. Editing is where writing becomes clear, safe and publishable. Professionals edit in passes, each looking for different problems.

> 💡 **Trivia**
>
> NIH calls pretesting "essential for measuring comprehension." [1] 🟢 Official (NIH)
>
> Self-editing is your first test. Reading it as the reader would, before anyone else does.

37.1 The three-pass edit

Pass	Focus	Questions
1 · Structure (big picture)	Purpose, intent, order	Does it fulfil the purpose statement? Answer first? Right order? Anything missing (safety, info gain)? Anything to cut?
2 · Sentences (clarity)	Plain language, rhythm, claims	Short? Active? Qualified? Hypophora where useful? Any PROHIBITED claims?
3 · Words (polish)	Word choice, style, errors	Risky words? Filler? People-first? House style? Spelling and grammar?

Tip: leave time between drafting and editing (even 30 minutes), and read pass 2 aloud. 🔵

37.2 The checklist tests (run in this order)

Test	Letters	Checks
PURPOSE	Beneficial · Clear · Honest · Focused · Achieved	Does the page deserve to exist?

Test	Letters	Checks
HUMAN	Helpful · Understandable · Meaningful depth · Accurate · Not manipulative	Will a real person be helped?
SAFETY	Specific · Accurate & current · Framed with limits · Evidence-aligned · Transparent · You-first	Are limits and risks clear?
CLEAR	Correct · Limited · Explicit · Accountable · Reader-safe	Would a vulnerable reader be safe?
FALSE (claims to remove)	Fully guaranteed · Absolute language · Legal/regulatory overreach · Selective facts · Emotional exploitation	Any of these? Remove
STOP (red flags)	Safety risk · Truth distortion · Overconfidence · Purpose clarity	Any persuasive sentence failing? Rewrite
NATURAL	Natural language · Audience clarity · Topic focus · Useful phrasing · Reader flow · Authentic voice · Logical structure	Does SEO interrupt reading?

All tests are in Appendix H as a printable sheet. 🔵

> 🪔 **Desi analogy: IT office ka code review**
>
> Pehle code chalta hai ya nahi (structure), phir logic saaf hai ya nahi (sentences), phir naming aur formatting (words). Sab ek saath dekhoge toh kuch na kuch chhoot jaayega. **Editing bhi teen alag review hain.**

37.3 Search-and-fix list (use Find)

Search for	Action
Brand drug names (Ozempic, Wegovy, Mounjaro, Zepbound, Rybelsus, Saxenda)	Remove unless reviewed education with the "does not offer" line
"guarantee", "proven", "cure", "fast", "melt", "miracle", "best", "#1"	Rewrite (Chapter 7)
"FDA-approved"	Check it's never applied to our compounded products

Search for	Action
"always", "never", "everyone", "no side effects"	Qualify or remove
"obese"	People-first rewrite
The pharmacy partner's name	Remove
"It is important to note", "In today's world", "In conclusion"	Cut
"will" + outcome ("you will lose")	"may"
Prices	Match the live site

37.4 Before/after edit example

Draft	Edited
"In today's world, many obese individuals struggle with weight. Semaglutide is a proven, FDA-approved miracle that will help you lose weight fast with no side effects!"	"Many people with obesity find weight hard to manage with diet and exercise alone. Compounded semaglutide may reduce appetite and help with weight loss when a provider decides it's right for you. It's not FDA-approved, and side effects like nausea are common at first."

What changed: filler cut; people-first; "proven/FDA-approved/miracle/fast/no side effects" removed; "compounded" added; qualifiers and risk added; provider decides. 🔵

> 🟧 **Ongo Weight Loss example**
>
> Running the search list on our homepage would flag "Backed by Science" (implied proof), "Patients love him" (subjective), an unexplained asterisk on "Doctor-led prescription*", and unsourced "4.9/5". Each takes minutes to fix once found. 🔵

37.5 Do's and Don'ts

✅ Do	❌ Don't
Edit in three separate passes	Edit everything at once

✅ Do	❌ Don't
Read pass 2 aloud	Edit silently only
Run the search list every time	Rely on memory
Take a break before editing	Edit immediately after drafting

37.6 Pros and cons of structured self-editing

✅ Pros	❌ Cons
Fewer review rounds	Takes 30–60 minutes per piece
Catches compliance issues early	Can feel repetitive
Builds your skill fast	—

37.7 Where freshers go wrong in Chapter 37 topics

Mistake	Why it happens	Avoid it by	Fix it by
Only fixing typos	Thinking editing = proofreading	Three passes	Starting with structure
Missing a brand name in an image caption	Not searching captions	Search list on all text	Searching everything
Letting AI "edit"	Speed	Chapter 17 rules	Editing yourself

37.8 Thumb rules

> 👍 Structure, sentences, words: three passes.
>
> 👍 Search before you submit.
>
> 👍 Read it as the most worried reader would.

37.9 Notes to remember

📝 Tests: PURPOSE, HUMAN, SAFETY, CLEAR, FALSE, STOP, NATURAL.

📝 Search list covers brand names, hype words, absolutes, pharmacy name and prices.

Chapter 37 Glossary

Term	Simple meaning
Self-editing	Reviewing and improving your own draft
Pass	One read-through focused on one kind of problem
Structural edit	Checking order, purpose and completeness
Line edit	Improving sentences for clarity and flow
Proofreading	Fixing spelling, grammar and typos

Chapter 37 Quiz

1. What's pass 1 about?

- A) Images
- B) Structure: purpose, intent, order
- C) Commas
- D) Fonts

2. Which test finds claims to remove?

- A) HUMAN
- B) NATURAL
- C) FALSE
- D) PURPOSE

3. What should you do with "you will lose weight"?

- A) Bold it
- B) Change to "may" with context
- C) Keep it
- D) Add "guaranteed"

4. Why read pass 2 aloud?

- A) To time it
- B) No reason
- C) To catch unclear or robotic sentences
- D) To memorise it

5. Is "FDA-approved" ever used for our compounded products?

- A) In meta only
- B) Never
- C) Sometimes
- D) Always

6. What does the NATURAL test check?

- A) Image sizes
- B) Legal text
- C) Prices
- D) Whether SEO interrupts reading and the voice sounds human

7. What should you do before editing?

- A) Delete the outline
- B) Ask AI to rewrite
- C) Publish
- D) Take a short break

8. Which word must be rewritten people-first?

- A) "Provider"
- B) "Nausea"
- C) "Price"
- D) "Obese"

9. What's proofreading?

- A) Rewriting the structure
- B) Choosing keywords
- C) Fixing spelling, grammar and typos
- D) Designing images

10. In the code-review analogy, editing is…

- A) Three separate reviews
- B) One quick glance
- C) Done by the client
- D) Skipped

Sources

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, writing guidelines (checklist tests) and homepage review, October 2026 (internal) 🔵

Answer key, Chapter 37: 1-B · 2-C · 3-B · 4-C · 5-B · 6-D · 7-D · 8-D · 9-C · 10-A

LEVEL 4 · CHAPTER 38

Chapter 38: Fact-Checking, Citations and Honest Dates

A health page is only as good as its facts. This chapter covers how to check them, how to cite them, and how to date pages honestly.

> 💡 **Trivia**
>
> Google lists "changing the date of pages to make them seem fresh when the content has not substantially changed" as a sign of search-engine-first content. [1] ● Official (Google)
>
> Honest dates are a trust signal. Fake dates are a warning sign.

38.1 The fact-check routine (VERIFY recap)

Step	Action
Validate	Open the source; confirm the exact words or number
Examine scope	Same product, dose, population?
Recheck dates	Still current? (Rules, prices, availability change)
Identify assumptions	Does your sentence imply more than the source says?
Flag uncertainty	Add qualifiers where the evidence is limited
Your responsibility	Would you defend it publicly?

38.2 What must always be checked

Fact type	Check against
Prices and inclusions	Live website / team lead

Fact type	Check against
Delivery times, process steps	Operations
Medication facts, side effects	FDA labels, peer-reviewed sources, medical reviewer
Trial results	Original paper (product, dose, duration)
Statistics	Original survey/agency page (with year)
Regulations	Agency statement + legal review
Provider credentials	HR / provider
Competitor facts (if ever used)	Their live page, dated; legal approval

38.3 Citation style

Rule	Example
In-text link on descriptive words	"In the STEP 1 trial, adults on brand semaglutide..."
Sources list at the end	"Wilding et al., Once-Weekly Semaglutide in Adults with Overweight or Obesity, NEJM, 2021"
Link to the exact page	DOI, PubMed, the FDA page itself
Don't over-link	1–2 sources per key claim is enough
External links open in a new tab (developer setting)	—
No competitor links as sources	Chapter 13

> 🍛 **Desi analogy: shaadi ka kundli milaan**
>
> Pandit ji kundli milaate waqt har ghar ka hisaab dekhte hain, andaaze se nahi. **Fact-check bhi har claim ko source se milaana hai:** ek-ek karke.

38.4 Honest dates

Date	Meaning	When to change
Published	First went live	Never (unless republished as new)
Last updated	Content changed meaningfully	After real edits (facts, sections), not typo fixes
Last medically reviewed	A reviewer re-checked medical content	Only after an actual medical review

Never update a date without a real change. Never show a future date. Keep both "updated" and "medically reviewed" honest. 🔵

38.5 Corrections

Step	Action
1	Fix the error
2	If it's material (dose, safety, price), add a short correction note with date
3	Tell the team lead and reviewer
4	Check other pages for the same error
5	Update the claim log

> 🟧 **Ongo Weight Loss example**
>
> Our blog posts showed no dates on 30 October 2026. When dates are added, they must reflect real history: the true publish date, and "last medically reviewed" only once a reviewer has actually re-checked each post. 🔵

38.6 Do's and Don'ts

✅ Do	❌ Don't
Check every fact at the source	Trust your memory or a summary
Use descriptive link text	Use "click here" or bare URLs
Change dates only for real updates	Refresh dates for "freshness"
Note corrections openly	Fix silently when it matters

38.7 Pros and cons of strict fact-checking

✅ Pros	❌ Cons
Fewer errors, faster approvals	Time-consuming
Strong trust signals	Paywalled sources need workarounds
Easier updates	Needs a maintained claim log

38.8 Where freshers go wrong in Chapter 38 topics

Mistake	Why it happens	Avoid it by	Fix it by
Updating "last updated" after a typo fix	Freshness myth	Honest-date rule	Reverting
Citing a news article instead of the study	Easier to find	Original-source rule	Linking the study
Silent fix of a wrong dose	Embarrassment	Correction steps	Adding a dated note and informing

38.9 Thumb rules

👍 Every claim, its source, its date.

👍 Dates tell the truth or they don't change.

👍 Fix fast, tell openly.

38.10 Notes to remember

📝 Three dates: published, last updated, last medically reviewed.

📝 Fake freshness is a Google warning sign.

Chapter 38 Glossary

Term	Simple meaning
Citation	A reference to the source of a fact
DOI	A permanent link ID for research papers
Anchor text	The clickable words of a link
Correction note	A visible note explaining a fixed error
Last medically reviewed	The date a medical reviewer last checked the page

Chapter 38 Quiz

1. What does Google say about changing dates without real changes?

- A) It's a search-engine-first warning sign
- B) Nothing
- C) It boosts rankings
- D) It's recommended

2. When should "last medically reviewed" change?

- A) After typo fixes
- B) Only after an actual medical review
- C) Never
- D) Every week

3. What's good anchor text?

- A) "click here"
- B) "link"
- C) "the STEP 1 trial"
- D) A bare URL

4. Who confirms delivery times?

- A) AI
- B) Competitors
- C) Operations
- D) The writer

5. What do you check trial results against?

- A) Memory
- B) A competitor blog
- C) A tweet
- D) The original paper

6. What's a DOI?

- A) A permanent link ID for research papers
- B) A price code
- C) A drug name
- D) A design tool

7. What do you do with a material error?

- A) Delete the page
- B) Ignore it
- C) Fix silently
- D) Fix it, add a dated note, inform the team

8. Can future dates be shown?

- A) Never
- B) Yes
- C) For freshness
- D) Only on blogs

9. How many sources per key claim?

- A) 20
- B) 1–2 is usually enough
- C) None
- D) 100

10. In the kundli analogy, fact-checking means…

- A) Asking neighbours
- B) Guessing
- C) Skipping steps
- D) Matching every claim to its source, one by one

Sources

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, blog listing, viewed 30 October 2026 🔵

Answer key, Chapter 38: 1-A · 2-B · 3-C · 4-C · 5-D · 6-A · 7-D · 8-A · 9-B · 10-D

LEVEL 4 · CHAPTER 39

Chapter 39: Medical and Legal Review: Submitting and Responding

Reviewers aren't obstacles. They're the safety net that lets you publish with confidence. This chapter shows how to make reviews fast and painless.

> 💡 **Trivia**
>
> Ro shows "Medically reviewed by Joanna Jan, MD" with credentials, and Found lists an author, a fact-checker and an editor on its articles. [1][2] ● Industry
>
> Visible review is now a category norm. The review must also be real.

39.1 What needs which review

Content	Team lead	Medical	Legal
Medication, side effects, dosing, safety	✅	✅	✅
Prices, offers, comparisons with competitors	✅	—	✅
Testimonials, case studies, patient stories	✅	✅	✅
Brand drug names in education	✅	✅	✅
Policies, consent text, eligibility results	✅	✅ (if clinical)	✅
Lifestyle (food, movement) with health claims	✅	✅	—
Lifestyle with no claims	✅	—	—
Microcopy for forms with health questions	✅	✅	✅

39.2 The submission pack (recap)

Purpose statement · keyword table · claim log · information gain note · internal links · image suggestions with alt text · open questions (Chapter 16).

Make it easy: highlight sentences that need clinical judgement, and ask specific questions ("Is 'often eases within a few weeks' accurate for compounded semaglutide?"). 🔵

> 🪔 **Desi analogy: shaadi se pehle pandit ji ka muhurat**
>
> Shaadi ki taiyaari kitni bhi ho, pandit ji ka muhurat aur sign-off ke bina phere nahi hote. Smart family pehle se sab kaagaz aur kundli taiyaar rakhti hai, toh kaam jaldi hota hai. **Reviewer ka sign-off muhurat hai:** pack taiyaar rakho.

39.3 Who can review: qualifications

Roles at Ongo Weight Loss are **flexible**: the team lead can assign any qualified person as author or reviewer for a piece, based on their expertise. What matters is that the reviewer is **qualified for the claims in that piece**.

Review type	Minimum qualification	Good to have
Medical review (treatment, side effects, dosing, safety)	Licensed clinician with an active U.S. licence: **MD or DO**; or **NP / PA** within their scope	Obesity medicine experience (e.g., ABOM certification), prescribing GLP-1 medications
Pharmacy and medication handling	**PharmD** (licensed pharmacist) or the medical reviewer	Compounding knowledge
Nutrition content (meal plans, protein, recipes with health claims)	**Registered Dietitian (RD/RDN)** or the medical reviewer	Experience with GLP-1 patients
Legal / compliance review (claims, prices, testimonials, policies)	Attorney or compliance professional with **FDA/FTC health advertising** experience	Telehealth and HIPAA experience
Editorial / SEO review	Senior writer, editor or on-page SEO who has completed Books Four and Six	—

Bylines must be true: the person named as "Medically reviewed by" must actually have reviewed that version, and their credentials on the profile page must be verified (Chapter 35). Assign author and reviewer names per piece in the brief. 🔵

39.4 Responding to review comments

Comment type	How to respond
Factual correction	Accept, fix, update the claim log
Rewording for safety	Accept; ask why if unclear so you learn
Cut	Accept; if it removes key reader help, suggest a safer version
Question	Answer with your source
Disagreement	Discuss respectfully with evidence; reviewer has final say on medical/legal

Never publish around a reviewer, or change approved wording after approval without re-review. 🔵

39.5 Review timing

Content	Plan ahead
Blog post	3–5 business days for review
Money page	1–2 weeks (medical + legal)
Regulatory news	Same day if urgent, with legal
Updates to approved pages	Re-review the changed parts

39.6 The approval log

Page	Version	Reviewer	Decision	Date	Notes
/blog/semaglutide-nausea	v3	[Medical reviewer]	Approved with edits	2026-11-04	"eases" → "often eases"

Keep it in the shared sheet with the claim log. 🔵

> 🟧 **Ongo Weight Loss example**
>
> When our blog posts get a "Medically reviewed by" line, it must name the provider who actually reviewed that version, with the date from the approval log. A byline without a real review is worse than no byline. 🔵

39.7 Do's and Don'ts

✅ Do	❌ Don't
Submit the full pack	Send a bare draft
Ask specific questions	Ask "is this OK?" for 2,000 words
Log every approval	Rely on chat messages
Re-review after changes	Edit approved text quietly

39.8 Pros and cons of formal review

✅ Pros	❌ Cons
Safer, more accurate content	Slower publishing
Real E-E-A-T signals	Reviewer availability limits volume
Writers learn fast	Needs planning

39.9 Where freshers go wrong in Chapter 39 topics

Mistake	Why it happens	Avoid it by	Fix it by
Arguing about every edit	Feeling criticised	Response table	Asking "why" to learn
Publishing before legal review	Deadline pressure	Review table	Unpublishing and reviewing

Mistake	Why it happens	Avoid it by	Fix it by
"Medically reviewed" label without review	Copying competitors	Approval log	Removing the label until reviewed

39.10 Thumb rules

👍 Reviewers protect readers, and you.

👍 No approval log entry, no publish.

👍 Changed after approval = not approved.

39.11 Notes to remember

📝 Medical claims → medical; prices, testimonials, policies → legal.

📝 Plan 3–5 days for blog review; 1–2 weeks for money pages.

Chapter 39 Glossary

Term	Simple meaning
Medical review	A licensed provider checks health content
Legal review	Legal checks compliance, claims and policies
Approval log	Record of who approved which version, when
Version	A numbered draft (v1, v2...)
Sign-off	Final approval to publish

Chapter 39 Quiz

1. What needs medical, legal and team-lead review?

- A) Nothing
- B) A no-claims recipe
- C) Medication, side-effect and safety content
- D) A typo fix

2. How should you respond to a factual correction?

- A) Argue
- B) Publish anyway
- C) Ignore
- D) Accept, fix and update the claim log

3. Who has final say on medical accuracy?

- A) AI
- B) The medical reviewer
- C) The writer
- D) The designer

4. How long should you plan for blog review?

- A) None
- B) 6 months
- C) 5 minutes
- D) 3–5 business days

5. What's wrong with editing approved text quietly?

- A) It's faster
- B) It's required
- C) Nothing
- D) It's no longer approved; needs re-review

6. What makes a review request easy?

- A) "Is this OK?"
- B) No sources
- C) Full pack plus specific questions

- D) A screenshot

7. When can "Medically reviewed by" appear?

- A) Always
- B) Never
- C) Only after a real review of that version
- D) If a competitor uses it

8. What does the approval log record?

- A) Page, version, reviewer, decision, date, notes
- B) Patient names
- C) Fonts
- D) Passwords

9. Do prices need legal review?

- A) Yes
- B) Only on blogs
- C) Only in emails
- D) No

10. In the muhurat analogy, the reviewer's sign-off is…

- A) Required before going ahead
- B) After the wedding
- C) Decorative
- D) Optional

Sources

1. Ro, Semaglutide nausea guide, viewed 30 October 2026
2. Found, Zepbound vs Ozempic, viewed 30 October 2026
3. Ongo Weight Loss, review workflow (Book Five, Chapter 38), internal

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Answer key, Chapter 39: 1-C · 2-D · 3-B · 4-D · 5-D · 6-C · 7-C · 8-A · 9-A · 10-A

LEVEL 4 · CHAPTER 40

Chapter 40: SEO Handoff and Internal Linking for Writers

Your words become a web page through SEO and developers. A clean handoff saves everyone time. Internal links turn single pages into a path that guides readers forward.

> 💡 **Trivia**
>
> Google's spam policies define scaled content abuse as "when many pages are generated for the primary purpose of manipulating search rankings and not helping users." [1] ● Official (Google)
>
> Internal links and keywords help when they serve readers. Used to manipulate, they hurt.

40.1 What writers hand to SEO and developers

Item	Format
Final approved copy	Google Doc with heading levels marked (H1, H2, H3)
Meta title, description, slug	At the top of the doc
Primary/secondary keywords	Keyword table
Internal links	Anchor text + target URL, in the doc
External sources	Linked in text + sources list
FAQs	Visible text (for FAQ schema, Book Four)
Images	Suggestions + alt text + captions
CTA wording	Exact approved text (matches events, Chapter 32)
Trust box	Author, reviewer, dates

Item	Format
Approval reference	Approval log row

The Google Doc header block (required on every piece)

```
CONTENT TYPE:   Side-effect guide (blog)
PRIMARY KW:     semaglutide nausea  (the one query to rank for)
SECONDARY:      how long does semaglutide nausea last
                what helps nausea on semaglutide
META TITLE:     Semaglutide Nausea: Causes & What Helps
                | Ongo Weight Loss
META DESC:      Nausea is common in the first weeks of semaglutide.
                Learn why it happens, how long it may last, what
                helps, and when to call your provider for help.
SLUG:           /blog/semaglutide-nausea/
AUTHOR/REVIEWER: [assigned in brief]
SOURCES:        1. … (link)  2. … (link)  every resource used
```

Writers deliver in **Google Docs**. Nothing goes to SEO or developers without this header and a full sources list. 🔵

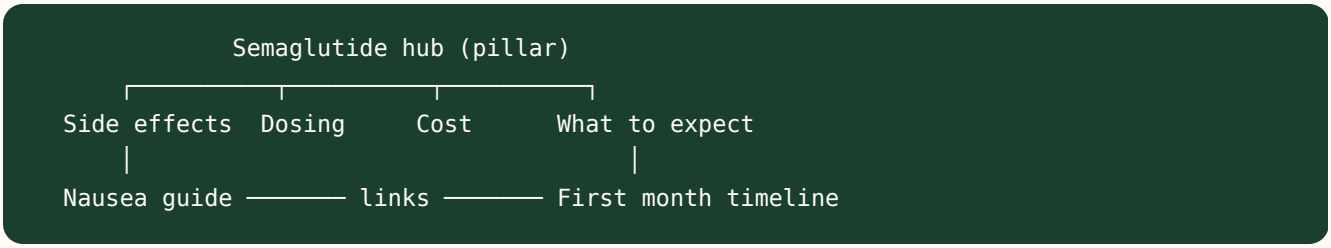
40.2 Internal linking rules for writers

Rule	Example
Link to the next funnel step (Chapter 11)	TOFU "What is GLP-1?" → MOFU "Semaglutide vs tirzepatide"
Link up to the parent hub	Side-effect guide → semaglutide hub
Link across to siblings when useful	Nausea guide ↔ constipation guide
Descriptive anchor text	"our guide to managing nausea" (not "click here", not exact-match keyword every time)
3–8 contextual links in a typical article	Enough to help, not a wall of links
First link high	One relevant link in the first third
No links to competitors	—

Rule	Example
No UTMs on internal links	Book Five, Chapter 40

> 🪔 **Desi analogy: mohalle ke raaste**
>
> Achhe mohalle mein har gali ke mod pe board hota hai: "Mandir →", "Market →", "Hospital →". **Internal links wahi boards hain:** reader ko agle sahi ghar tak pahunchaate hain.

40.3 Hub-and-spoke from a writer's view



- Spokes link to the hub; the hub links to all spokes.
- Each spoke has its own unique keyword (no cannibalisation).
- Book Three's master content map is the source of truth. 🔵

40.4 Avoiding cannibalisation

Check	Action if a problem
Does another page target the same primary keyword?	Re-target, merge, or link to it instead
Does another page answer the same main question?	Different angle or merge
Are two titles nearly the same?	Ask SEO before writing

40.5 On-page SEO checklist (writer's part)

The full, team-wide version with owners and pass rules is **Appendix L**.

☐	Item
☐	Primary keyword in H1, first 100 words, meta title, slug

☐	Item
☐	Question H2s with answer blocks
☐	One H1; no skipped heading levels
☐	3–8 descriptive internal links (funnel, parent, siblings)
☐	Sources linked
☐	Alt text written
☐	FAQs visible
☐	Meta title and description written

> **🟧 Ongo Weight Loss example**
>
> Our two dosing guides (semaglutide and tirzepatide) have near-identical titles and structures. Before writing more, check the content map: each should target its own medication keyword, link up to its own hub, and link across to each other with clear anchors ("tirzepatide dosing guide"). 🔵

40.6 Do's and Don'ts

✅ Do	❌ Don't
Mark heading levels in the doc	Rely on font size
Add internal links with descriptive anchors	Use "click here"
Check for cannibalisation	Write duplicate-intent pages
Hand over everything in one doc	Send pieces by chat

40.7 Pros and cons of writer-led linking

✅ Pros	❌ Cons
Links fit the context naturally	Writers need the content map
Better reader journeys	Links break if URLs change (SEO monitors)
Measurable with Book Four events	Requires coordination

40.8 Where freshers go wrong in Chapter 40 topics

Mistake	Why it happens	Avoid it by	Fix it by
25 links in one article	"More is better"	3–8 rule	Keeping the most helpful
Same exact-match anchor every time	Old SEO habit	Descriptive variety	Varying naturally
Linking to a competitor's guide	It was helpful	No-competitor rule	Linking our page or the original source

40.9 Thumb rules

👍 Every page points up, across and forward.

👍 Anchor text tells the reader where they'll land.

👍 One doc, everything in it.

40.10 Notes to remember

📝 Book Three's content map decides links and keywords.

📝 No UTMs on internal links.

Chapter 40 Glossary

Term	Simple meaning
Internal link	A link to another page on our site
Anchor text	The clickable words of a link
Hub (pillar)	Main page for a topic that links to related pages
Spoke	A supporting page linked to the hub
Handoff	Passing finished copy to SEO and developers

Chapter 40 Quiz

1. How many contextual internal links in a typical article?

- A) 50
- B) 0
- C) 3–8
- D) 1 in the footer

2. What's good anchor text?

- A) "click here"
- B) "this"
- C) A bare URL
- D) "our guide to managing nausea"

3. Where do spokes link?

- A) To the hub (and the hub to them)
- B) Nowhere
- C) Only to the homepage
- D) Only to competitors

4. Should internal links have UTMs?

- A) Yes

- B) No
- C) Only in emails
- D) Only on blogs

5. What's the source of truth for links and keywords?

- A) Book Three's master content map
- B) A competitor's sitemap
- C) The writer's memory
- D) AI

6. How are heading levels handed over?

- A) In a chat message
- B) By font size only
- C) Not at all
- D) Marked in the doc (H1, H2, H3)

7. What do you do if another page targets your keyword?

- A) Ignore it
- B) Copy that page
- C) Write it anyway
- D) Re-target, merge or link instead

8. Can writers link to competitors' guides?

- A) Only in FAQs
- B) Yes
- C) Only good ones
- D) No

9. Where should the first internal link appear?

- A) In the footer
- B) Only at the very end
- C) In the first third of the article
- D) In alt text

10. In the mohalla analogy, internal links are…

- A) Speed bumps

- B) Signboards guiding readers to the next place
- C) Locked gates
- D) Advertisements

Sources

1. Google Search Central, Spam policies
2. Ongo Weight Loss, Book Three (content map) and Book Four (on-page SEO, events), internal

Answer key, Chapter 40: 1-C · 2-D · 3-A · 4-B · 5-A · 6-D · 7-D · 8-D · 9-C · 10-B

LEVEL 4 · CHAPTER 41

Chapter 41: Accessible and Inclusive Writing

Some readers use screen readers, some zoom text to 200%, some read in a second language, some are reading through pain or anxiety. Accessible writing helps all of them, and everyone else too.

> 💡 **Trivia**
>
> The CDC Clear Communication Index scores materials on the main message, calls to action, language, information design, numbers and risk: the same things that make writing accessible. [1] ● Official (CDC)
>
> Clear writing and accessible writing are mostly the same skill.

41.1 Writer's accessibility checklist

☐	Item	Why
☐	Plain language, grade 6–8 for key pages	Cognitive accessibility
☐	Headings in order (H1 → H2 → H3)	Screen-reader navigation
☐	Descriptive link text	Screen readers list links out of context
☐	Alt text for meaningful images; empty for decorative	Blind and low-vision users
☐	Don't rely on colour alone ("click the green button")	Colour-blind users
☐	Don't rely on position alone ("see the box on the right")	Layout changes on mobile
☐	Spell out abbreviations on first use	Clarity
☐	Lists and tables for structure	Easier navigation

☐	Item	Why
☐	Captions/transcripts for video scripts	Deaf and hard-of-hearing users
☐	Clear error and form text (Chapter 32)	Everyone

Book Four, Chapter 34 covers technical accessibility (ARIA, contrast) for developers. 🔵

> 🪔 **Desi analogy: shaadi mein ramp**
>
> Shaadi ke hall mein seedhiyon ke saath ramp ho, toh dadi ki wheelchair bhi aaraam se andar aaye, aur saamaan waale bhi khush. **Accessible writing ramp hai:** kuch logon ke liye zaroori, sabke liye aasaan.

41.2 Link text examples

❌ Bad	✅ Good
"Click here"	"Read our guide to semaglutide side effects"
"Learn more" (repeated 10 times)	"Learn more about our pricing"
"https://ongoweightloss.com/blog/..."	"our dosing guide"

41.3 Inclusive language (recap and additions)

Area	Guidance
Weight	People-first; no shame words (Chapter 7)
Gender	Neutral where possible; specific when research is specific
Age	"Older adults", not "elderly"
Disability	Avoid "suffers from", "confined to a wheelchair"; say "uses a wheelchair"
Culture	Respect food traditions; offer swaps, don't label cuisines bad
Literacy	Avoid idioms that don't translate ("ballpark", "hit the ground running")
Images	Suggest diverse, real people (consented) across ages, bodies and backgrounds

41.4 Writing for screen readers

Tip	Example
Write numbers clearly	"2.4 mg" not "2.4mg" next to other numbers without spaces
Avoid symbols as words	"and" not "&" in body text; "to" not "→"
Emojis not on web pages	Screen readers read their names aloud
Make tables simple	Header row, no merged cells (design/dev)

> 🟧 **Ongo Weight Loss example**
>
> Several homepage CTAs say slightly different things for the same action, and headings like "Guided. Tailored. Care." give a screen-reader user no idea what the section contains. Descriptive headings ("How your provider guides your care") and consistent link text fix both. 🔵

41.5 Do's and Don'ts

✅ Do	❌ Don't
Write descriptive headings and links	Use "click here" or clever but vague headings
Describe meaningful images	Leave alt text empty on charts
Spell out abbreviations once	Assume everyone knows "GI", "BMI", "Rx"
Use inclusive, respectful words	Use idioms and slang

41.6 Pros and cons of accessible writing

✅ Pros	❌ Cons
Reaches more people	Needs habit change
Better SEO and AI understanding	Some legal text resists simplification

✅ Pros	❌ Cons
Lower legal risk (accessibility laws)	Requires design/dev coordination

41.7 Where freshers go wrong in Chapter 41 topics

Mistake	Why it happens	Avoid it by	Fix it by
"Click the orange button"	Visual thinking	Colour rule	"Select 'Check my eligibility'"
Emojis in web copy	Social habit	Screen-reader tip	Removing
Unexplained "Rx", "GI"	Expert habit	Abbreviation rule	Spelling out once

41.8 Thumb rules

👍 Write so it works when heard, zoomed or translated.

👍 Links and headings must make sense on their own.

👍 Never rely on colour or position alone.

41.9 Notes to remember

📝 Writer's accessibility checklist: 10 items.

📝 Technical accessibility lives in Book Four, Chapter 34.

Chapter 41 Glossary

Term	Simple meaning
Accessibility	Making content usable by people with disabilities

Term	Simple meaning
Screen reader	Software that reads screen content aloud
Descriptive link text	Link words that say where the link goes
Cognitive accessibility	Making content easy to understand and remember
Inclusive language	Words that respect all people

Chapter 41 Quiz

1. Which link text is accessible?

- A) "More"
- B) A bare URL
- C) "Click here"
- D) "Read our guide to semaglutide side effects"

2. Why avoid "click the green button"?

- A) It's too short
- B) Colour-blind users may not see it
- C) It's a keyword
- D) Green is unlucky

3. Should emojis be on web pages?

- A) No; screen readers read them aloud
- B) In alt text
- C) Yes, everywhere
- D) In headings only

4. What should decorative images get?

- A) Empty alt text
- B) Long descriptions
- C) The brand name
- D) Keywords

5. Which word is preferred?

- A) "Aged ones"
- B) "Old folks"
- C) "Elderly"
- D) "Older adults"

6. What should you do with "BMI" on first use?

- A) Spell it out (body mass index)
- B) Remove it
- C) Bold it
- D) Leave it

7. Which is a problem heading for screen-reader users?

- A) "How long does nausea last?"
- B) "How your provider guides your care"
- C) "What's included in the price"
- D) "Guided. Tailored. Care."

8. Who handles ARIA and contrast?

- A) Developers and design (Book Four)
- B) Writers only
- C) Nobody
- D) The social team

9. Why avoid idioms?

- A) They're legal terms
- B) They don't translate well and can confuse
- C) They're too short
- D) They're keywords

10. In the ramp analogy, accessible writing…

- A) Helps nobody
- B) Is only decoration
- C) Helps some people greatly and everyone a little
- D) Slows everyone

Sources

1. CDC, Clear Communication Index
2. Ongo Weight Loss, homepage review, 30 October 2026, and Book Four (accessibility), internal

Answer key, Chapter 41: 1-D · 2-B · 3-A · 4-A · 5-D · 6-A · 7-D · 8-A · 9-B · 10-C

LEVEL 4 · CHAPTER 42

Chapter 42: The Writer's Toolkit: Free, Freemium and Paid

Tools don't write for you, but the right ones save hours on research, checking and organisation. Start with free tools; add paid ones only when they solve a real problem.

> 💡 **Trivia**
>
> NIH's Clear & Simple guide presents SMOG as one readability tool, but warns that formulas only measure vocabulary and sentence length. [1] 🟢 Official (NIH)
>
> Every tool in this chapter is an assistant. Your judgment and the reviewers decide.

42.1 How to read this chapter

Label	Meaning
🆓 **Free**	Free to use for our needs
🔓 **Freemium**	Useful free version; paid plan adds features
💳 **Paid**	Paid subscription (team lead approves)

Plans and prices **change often**. Check each tool's pricing page before relying on a feature. 🔵

42.2 Writing and editing

Tool	Tier	Use for	Watch out
Google Docs	🆓	Drafts, comments, version history, suggestion mode for reviewers	Mark heading styles properly
Grammarly	🔓	Spelling and grammar ("Correctness")	Don't accept full rewrites; it can change claims

Tool	Tier	Use for	Watch out
LanguageTool	🔓	Grammar and style; multiple languages	Same as above
Hemingway Editor	🆓 web / 💳 app	Highlights long sentences, passive voice, grade level	Score isn't the goal
ProWritingAid	🔓	Style reports, repetition, sentence variety	Can over-suggest
Readable	💳	Multiple readability scores, reports	Optional
Thesaurus / Merriam-Webster	🆓	Plain-word alternatives, definitions	—

42.3 Research

Tool	Tier	Use for
PubMed	🆓	Peer-reviewed studies
Google Scholar	🆓	Finding papers and citations
Drugs@FDA / DailyMed	🆓	Official drug labels
MedlinePlus	🆓	Plain-language health background
CDC, NIH, KFF, Pew	🆓	Statistics and surveys
Zotero	🆓	Saving and organising sources and citations
Perplexity	🔓	Finding sources fast (always open and verify)
ChatGPT / Claude / Gemini	🔓	Brainstorming questions, gap checks, proofreading lists (Chapter 17)

42.4 Keyword and question research

Tool	Tier	Use for
Google search (autocomplete, PAA)	🆓	Real questions

Tool	Tier	Use for
Google Search Console	🆓	Queries reaching our pages (via SEO team)
Google Trends	🆓	Interest over time, seasonality
AlsoAsked	🔓	Question trees
AnswerThePublic	🔓	Question and preposition maps
Semrush / Ahrefs	💳	Search volume, difficulty, competitor pages (via SEO team)
Surfer / Clearscope (content optimisers)	💳	Topic coverage hints

Caution with content optimisers: they suggest terms competitors use. Use them to spot missing **questions**, not to stuff terms or match word counts. 🔵

> 🍛 **Desi analogy: rasoi ke bartan**
>
> Achhi rasoi mein har bartan ka kaam hai, lekin swaad cook ke haath mein hai. Mehenga pressure cooker bhi bina namak ke dal ko swaadisht nahi bana sakta. **Tools bartan hain; writing aapka haath.**

42.5 Checking and quality

Tool	Tier	Use for	Watch out
Copyscape	💳 (per check)	Accidental plagiarism checks	—
Grammarly plagiarism	💳	Same	—
AI detectors (e.g., Originality.ai, GPTZero)	🔓/💳	Optional internal signal only	Can be wrong on human writing; never a quality test (Chapter 17)
WAVE (WebAIM)	🆓	Accessibility check on live pages	Developer fixes technical issues
Hemingway	🆓	Readability pass	—

42.6 Organisation and collaboration

Tool	Tier	Use for
Google Sheets	🆓	Keyword tables, claim logs, approval logs, content calendar
Trello / Asana / Notion	🔓	Content workflow boards
Google Drive	🆓	Shared folders (restricted folder for consent forms)
Canva	🔓	Quick mock-ups of tables/infographics for designers
Loom	🔓	Short screen recordings to explain edits

42.7 Recommended starter stack (cost: $0)

Google Docs + Sheets · Grammarly or LanguageTool free · Hemingway web · PubMed + FDA + MedlinePlus · Google Trends + PAA + AlsoAsked free · Zotero · Trello free · WAVE. Add Semrush/Ahrefs access through the SEO team. 🔵

> 🟧 **Ongo Weight Loss example**
>
> Our writers already work with SEO, design and medical reviewers. Using one shared Google Sheet (tabs: Briefs, Keyword tables, Claim logs, Approval log, Calendar) gives everyone the same source of truth, at no cost. 🔵

42.8 Do's and Don'ts

✅ Do	❌ Don't
Start with free tools	Buy tools before you need them
Use grammar tools for errors only	Accept AI rewrites
Keep sources in Zotero or the claim log	Lose links in browser tabs
Recheck plan limits quarterly	Assume free features last forever

42.9 Pros and cons of tool-heavy workflows

✅ Pros	❌ Cons
Faster research and checking	Tools can replace thinking if misused
Consistent quality	Subscriptions add up
Easier collaboration	Some tools push "SEO writing" habits

42.10 Where freshers go wrong in Chapter 42 topics

Mistake	Why it happens	Avoid it by	Fix it by
Matching an optimiser's word count	Trusting the score	Questions-not-terms rule	Writing to the purpose
Paraphrasing tools on competitor text	Speed	Never-copy rule	Starting from your outline
Trusting one AI detector	Fear	Chapter 17	Focusing on quality

42.11 Thumb rules

👍 Free first, freemium second, paid when it solves a real problem.

👍 Tools assist; writers decide.

👍 One shared sheet for everything.

42.12 Notes to remember

📝 Starter stack costs $0.

📝 Prices and plans change; check before relying.

Chapter 42 Glossary

Term	Simple meaning
Freemium	Free basic version with paid upgrades
Content optimiser	A tool suggesting topics/terms based on top results
Plagiarism checker	A tool that finds copied text
Reference manager	A tool for saving and citing sources (e.g., Zotero)
WAVE	A free web accessibility checker

Chapter 42 Quiz

1. Which tool finds peer-reviewed studies for free?

- A) Loom
- B) PubMed
- C) Canva
- D) Trello

2. How should you use Grammarly?

- A) Never
- B) Accept full rewrites
- C) For correctness suggestions, one by one
- D) For medical facts

3. What's a content optimiser best used for?

- A) Matching word counts
- B) Spotting missing questions
- C) Writing the article
- D) Stuffing terms

4. Which tool saves and organises sources?

- A) Zotero

- B) WAVE
- C) Hemingway
- D) Photoshop

5. Are AI detectors a quality test?

- A) Google requires them
- B) No; they can be wrong
- C) They replace review
- D) Yes, the main one

6. What's the starter stack's cost?

- A) $500
- B) $0
- C) $1,000/month
- D) Unknown

7. Which tool checks accessibility on live pages?

- A) Perplexity
- B) AlsoAsked
- C) Zotero
- D) WAVE

8. What does "freemium" mean?

- A) Illegal
- B) Always free
- C) Always paid
- D) Free basic version with paid upgrades

9. Where do official drug labels live?

- A) Drugs@FDA / DailyMed
- B) Canva
- C) Competitor blogs
- D) Instagram

10. In the rasoi analogy, tools are…

- A) The guests

- B) The recipe itself
- C) Unnecessary
- D) Utensils; the cook's hand makes the taste

Sources

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, writing guidelines (tools) and team workflow, October 2026 (internal) 🔵

Answer key, Chapter 42: 1-B · 2-C · 3-B · 4-A · 5-B · 6-B · 7-D · 8-D · 9-A · 10-D

Chapter 43: Measuring, Refreshing and Your 30/60/90-Day Plan

Publishing isn't the end. Good writers check how pages perform, update them honestly, and keep improving. This chapter also gives you your first 90 days as a writer.

> 💡 **Trivia**
>
> Google's helpful-content guidance asks whether readers will "leave feeling they've had a satisfying experience". [1] ● Official (Google)
>
> Analytics can't measure satisfaction directly, but engagement, next-step clicks and search performance give strong clues.

43.1 What to measure (with the SEO team)

Question	Metric	Tool
Are people finding it?	Impressions, clicks, average position, queries	Google Search Console
Is the title/description working?	Click-through rate (CTR)	GSC
Are people reading?	Engaged sessions, engagement time, scroll	GA4
Are they moving forward?	`internal_link_click`, `cta_[pagegroup]_[action]`	GA4 (Book Four events)
Are they starting care?	`exit_to_portal` from the page's sessions	GA4
Are they asking the same questions?	Support/comment themes	Support logs

43.2 Reading the signals

Signal	Possible meaning	Writer action
High impressions, low CTR	Title/description not matching intent	Rewrite meta (Chapter 36)
Good clicks, low engagement	Answer not found quickly	Strengthen answer-first intro and blocks
Good engagement, few next-step clicks	Weak or missing links/CTA	Add funnel links; clarify CTA
New queries in GSC	Readers want more	Add an H2 or FAQ
Ranking dropped	Competitors more helpful or content outdated	Refresh with information gain

> 🪔 **Desi analogy: cricket ka match review**
>
> Match ke baad coach video dekhta hai: kaun sa shot chala, kahan wicket gaya. Agle match mein wahi galti nahi hoti. **Content review hamara match review hai.**

43.3 Honest content refreshes

Step	Action
1	Check facts, prices, rules (VERIFY)
2	Add new questions from GSC and support
3	Improve answer blocks and information gain
4	Fix links (internal and sources)
5	Medical/legal re-review for changed claims
6	Update "last updated" (and "last medically reviewed" only if reviewed)
7	Log what changed

Refresh cadence: money pages monthly (prices, offers); medication and safety pages every 6 months or when rules change; blog posts every 6–12 months or when data shows a drop. 🔵

43.4 Your 30/60/90-day plan

Period	Goals	Done when
Days 1–30: Learn	Read Books One, Three (map), Four (on-page) and this book; complete all quizzes (8/10+); shadow one article end to end; build your fact sheets for compounded semaglutide and tirzepatide	Quizzes passed; two fact sheets approved by the medical reviewer
Days 31–60: Write with support	Write 4–6 blog posts (side-effect guide, timeline, myth vs fact, listicle) with full submission packs; rewrite 10 homepage/money-page lines flagged in this book	Pieces pass the Appendix L audit sheet and medical/legal review; feedback from each review is applied in the next piece
Days 61–90: Work independently	Own a cluster (e.g., semaglutide side effects): 6–8 linked pages; run your first refresh using GSC data; write one trust page (editorial policy draft with legal)	Cluster live; first refresh logged; editorial policy submitted

> 🟧 **Ongo Weight Loss example**
>
> A strong first cluster for a new writer: the semaglutide hub's side-effect spokes (nausea, constipation, fatigue, "when to call your provider"), each with its own keyword, linked to the hub and to each other, measured with `internal_link_click`. 🔵

43.5 Do's and Don'ts

✅ Do	❌ Don't
Review page data monthly with SEO	Publish and forget
Refresh with real improvements	Change dates without changes
Track review rounds to improve	Repeat the same mistakes
Follow the 90-day plan	Try to do everything in week one

43.6 Pros and cons of ongoing measurement

✅ Pros	❌ Cons
Content keeps improving	Takes regular time
Clear proof of value	Data can be noisy for new pages
Smarter future topics	Needs SEO/analytics access

43.7 Where freshers go wrong in Chapter 43 topics

Mistake	Why it happens	Avoid it by	Fix it by
Rewriting a page after one week of data	Impatience	Monthly review	Waiting for enough data
Refreshing only the date	Freshness myth	Refresh steps	Real updates
Ignoring GSC queries	Not knowing where to look	SEO pairing	Monthly query review

43.8 Thumb rules

👍 Publish, measure, improve.

👍 Refresh with substance, not just dates.

👍 Learn, then write with support, then own.

43.9 Notes to remember

📝 Money pages monthly; medical pages every 6 months or on rule changes.

📝 Day 90 goal: own a linked cluster independently.

Chapter 43 Glossary

Term	Simple meaning
CTR	Click-through rate: clicks ÷ impressions
Engaged session	A GA4 visit with meaningful activity
Content refresh	Updating a page with real improvements
Cluster	A group of linked pages around one topic
Cadence	How often something is done

Chapter 43 Quiz

1. What does high impressions but low CTR suggest?

- A) Perfect page
- B) Title/description don't match intent
- C) Too many sources
- D) Broken images

2. Which tool shows search queries for a page?

- A) Canva
- B) Google Search Console
- C) Zotero
- D) Loom

3. How often should money pages be refreshed?

- A) Never
- B) Every 5 years
- C) Monthly
- D) Hourly

4. What's a real refresh?

- A) Changing the date only
- B) Adding keywords only

- C) Checking facts, adding questions, improving answers, re-reviewing
- D) Deleting the page

5. Which event shows readers moving to another page?

- A) `internal_link_click`
- B) `form_error`
- C) `page_not_found`
- D) `scroll_stop`

6. What's the goal of days 1–30?

- A) Learn: read books, pass quizzes, build fact sheets
- B) Run ads
- C) Redesign the site
- D) Write 50 posts

7. What's the day-90 goal?

- A) Buy tools
- B) Own a linked cluster independently
- C) Stop writing
- D) Leave the team

8. When does "last medically reviewed" change during a refresh?

- A) Always
- B) Never
- C) Only if a medical review actually happened
- D) Randomly

9. What does good engagement but few next-step clicks suggest?

- A) Perfect page
- B) Weak or missing links/CTA
- C) Bad meta title
- D) Too many links

10. In the match-review analogy, content review helps you…

- A) Skip practice
- B) Blame others

- C) Avoid repeating mistakes
- D) Change the scoreboard

Sources

1. Google Search Central, Creating helpful, reliable, people-first content
2. Ongo Weight Loss, Book Four (GA4 events) and Book Three (content map), internal

Answer key, Chapter 43: 1-B · 2-B · 3-C · 4-C · 5-A · 6-A · 7-B · 8-C · 9-B · 10-C

LEVEL 4 · CHAPTER 44

Chapter 44: The ZETA Model for Writers and the Final Gate

Zero-Error Tolerance Approach: the writing mistakes that must never reach a live page, because they can harm a reader, break the law, or destroy trust in one sentence.

> 💡 **Trivia**
>
> Google's rater guidelines: "If a page on YMYL topics is highly inexpert, it should be considered Untrustworthy and rated Lowest." [1] 🟢 Official (Google)
>
> One careless sentence can move a page from helpful to harmful.

44.1 The ZETA zones (writer's edition)

Zone	Meaning	Example
🟢 **Green**	Low-risk, fix any time	A typo; a weak heading
🟠 **Amber**	Fix before publishing; report it	Missing internal link; no FAQ; vague meta description
🔴 **Red**	Never publish; report the same day	Brand drug name as our product; "FDA-approved" for compounded; invented testimonial

44.2 The Writer's Red List: never, ever

#	Never do this	Why
1	Call compounded medications **FDA-approved**, "generic", or "the same as" a brand	False; FDA-flagged

#	Never do this	Why
2	Use **brand drug names** in commercial titles, CTAs, meta or offers	Launch-phase rule; trademark risk
3	Promise **outcomes** (pounds, speed, guarantees, "works for everyone")	Misleading; FTC/FDA risk
4	Apply **brand trial results** to our compounded products	Not studied; misleading
5	Name the **partner pharmacy**	Standing rule
6	Invent or alter **testimonials, reviews, patient stories, authors or doctor quotes** (including AI)	FTC rule; deception
7	Give **personal medical advice or dosing changes**	Practice of medicine; safety
8	Publish **medical, price, testimonial or policy content without review**	Approval rule
9	Show **prices** that don't match the website	Misleading pricing
10	Put **personal or health data** in content, URLs, AI tools or email subjects	Privacy
11	**Change dates** without real changes, or claim a medical review that didn't happen	Deception; Google warning sign
12	Use **shaming or fear** language about weight or bodies	Harm; trust; people-first standards

> **Desi analogy: cricket mein no-ball**
>
> Bowler kitna bhi achha ho, front foot no-ball pe wicket cancel aur free hit. **Red List wahi line hai:** kitna bhi achha article ho, ek Red List galti sab barbaad kar deti hai.

44.3 Warning signs

You hear or think…	It usually means…
"Just say it's the same as Ozempic, people understand"	Red List #1–2
"Round 14.9% to 'lose 15%'"	Red List #3–4

You hear or think…	It usually means…
"Let's write a sample patient story"	Red List #6
"Tell them to bump the dose if they plateau"	Red List #7
"It's a tiny change; no need to re-review"	Red List #8
"Update the date so it looks fresh"	Red List #11
"Stubborn belly fat" / "let yourself go"	Red List #12

44.4 The final gate (GATE + READY)

Check	Question
Goal confirmed	Is the purpose clear and beneficial?
Accuracy verified	Are facts current and correct (claim log complete)?
Trust protected	Is framing honest, cautious and transparent?
Ecosystem aligned	Does it match the rest of the site (prices, wording, links)?
Right purpose	Clear, beneficial, honest
Error-free facts	Verified against sources
Aligned intent	Matches what the searcher wanted
Defensible trust	Safe, transparent, responsible
YMYL-appropriate	Careful where the impact is high

Then run the full **Appendix L audit sheet**. Compare your piece with the annotated sample in **Appendix M**.

44.5 The final, honest question

> If a worried patient, a doctor, a regulator and a Google quality rater all read this page right now, would I be proud of it, or defensive about it?

If the answer is "defensive", the page isn't ready. Revise, then publish. 🔵

> 🟧 **Ongo Weight Loss example**
>
> The live-site items closest to the Red List in our October 2026 review: "Real Transformations, Backed by Science" (#3–4 risk), unsourced ratings (#9-style trust risk), and the compounded disclaimer sitting far from product cards (#1 risk by omission). None needs a rewrite of the whole site; each needs one careful sentence. 🔵
>
> 📝 **Note to remember:** the worst ZETA mistake is hiding a ZETA mistake. If a Red List item goes live, tell the team lead the same day.

44.6 Do's and Don'ts

✅ Do	❌ Don't
Run the final gate on every piece	Skip it "just this once"
Report Red List issues the same day	Fix quietly
Keep the Red List pinned beside your screen	Rely on memory
Ask the final honest question	Publish when you feel defensive

44.7 Pros and cons of zero tolerance

✅ Pros	❌ Cons
Protects patients, the brand and LegitScript certification	Slower publishing
Clear, teachable rules	Some catchy lines can't be used
Long-term trust	Needs discipline from everyone

44.8 Where freshers go wrong in Chapter 44 topics

Mistake	Why it happens	Avoid it by	Fix it by
Brand name slipped into an image caption	Not searching all text	Search list (Chapter 37)	Searching captions and alt text
"Sample" testimonial left in a draft that went live	Placeholder habit	Never write fake ones, even as placeholders	Removing and reporting
Not reporting own mistake	Fear	Same-day rule	Reporting now

44.9 Thumb rules

👍 Twelve lines never to cross, in every format.

👍 GATE and READY before every publish.

👍 Proud, not defensive.

44.10 Notes to remember

📝 Red List: FDA wording, brand names, outcomes, trial borrowing, pharmacy name, fake stories, medical advice, unreviewed content, wrong prices, personal data, fake dates/reviews, shame language.

📝 Report Red List issues the same day.

Chapter 44 Glossary

Term	Simple meaning
ZETA	Zero-Error Tolerance Approach
Red List	Mistakes that must never go live

Term	Simple meaning
GATE	Goal, Accuracy, Trust, Ecosystem check
READY	Right purpose, Error-free facts, Aligned intent, Defensible trust, YMYL-appropriate
Final honest question	The proud-or-defensive test before publishing

Chapter 44 Quiz

1. What does ZETA stand for?

- A) Zero Engagement Tracking App
- B) None
- C) Zone Editing Text Area
- D) Zero-Error Tolerance Approach

2. Which is a Red List item?

- A) A missing FAQ
- B) A weak heading
- C) Calling compounded medications FDA-approved
- D) A typo

3. Which is Amber, not Red?

- A) Naming the pharmacy
- B) Missing internal link
- C) Brand name in a CTA
- D) Invented testimonial

4. What's the "E" in GATE?

- A) Emojis added
- B) End
- C) Email sent
- D) Ecosystem aligned

5. What's the final honest question?

- A) Is the font nice?

- B) Would I be proud or defensive if a patient, doctor, regulator and rater read it?
- C) Does it have enough keywords?
- D) Is it long enough?

6. What does "Round 14.9% to 'lose 15%'" signal?

- A) Outcome promise and trial borrowing (Red List)
- B) A design issue
- C) Accessibility
- D) Good writing

7. Can fake testimonials be used as placeholders in drafts?

- A) No; never write them
- B) Only in emails
- C) Yes
- D) If deleted later

8. When should Red List issues be reported?

- A) Never
- B) Next quarter
- C) The same day
- D) Only if asked

9. What's the "D" in READY?

- A) Done fast
- B) Design
- C) Defensible trust
- D) Date changed

10. In the no-ball analogy, a Red List mistake...

- A) Is a boundary
- B) Earns extra runs
- C) Cancels the good work
- D) Doesn't matter

Sources

1. Google, Search Quality Rater Guidelines (September 11, 2025)
2. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025)
3. DWT, FTC finalizes rule banning fake consumer reviews (August 2024)
4. Ongo Weight Loss, homepage review, 30 October 2026 (internal)

Answer key, Chapter 44: 1-D · 2-C · 3-B · 4-D · 5-B · 6-A · 7-A · 8-C · 9-C · 10-C

Back Matter

Appendix A: Writing Models Cheat-Sheet

Model	Formula	Use for	Ongo Weight Loss example (one line)
Inverted pyramid	Most important → details → support → background	Every page	"Nausea is common early on and often eases. Here's why and what helps."
BLUF	Bottom line up front	FAQs, emails, intros	"Yes, most people can drink coffee; start small if nausea is an issue."
Answer block	40–60-word direct answer after each H2	All articles	"For many people, nausea is strongest early and after dose increases, then eases."
Hypophora	Reader's question → immediate answer	Worries, objections	"Do you need insurance? No. We're self-pay."
Breaking the fourth wall	Speak directly to the reader's situation	Emotional moments	"If you're reading this at 2 a.m. feeling sick..."
Why → What → How	Why it matters → what it is → how it applies	Planning any page	Nausea: why it matters → what causes it → what to do
What · Why · How · What next	Definition sequence	Glossary, definitions	Titration: low start → fewer side effects → provider schedule → don't self-adjust
PEEL	Point → Evidence → Explain → Link	Body paragraphs	"Protein matters → [source] → helps fullness → recipes"
SCQA	Situation → Complication → Question → Answer	Problem intros	Plateau article intro
Feynman	Explain to a 12-year-old; fix gaps	Complex science	"GLP-1 is a 'you're full' message from gut to brain"

Model	Formula	Use for	Ongo Weight Loss example (one line)
Analogy	Familiar comparison + real fact	Mechanisms	"Titration is like stepping into a cold pool"
Show, don't tell	Specific details instead of adjectives	Benefits copy	"Message your care team anytime" vs "very supportive"
Rule of three	Three items	Key messages	"Licensed providers, one all-in price, ongoing support"
5 Ws and H	Who, What, When, Where, Why, How	News, policy updates	FDA update summary
STAR	Situation → Task → Action → Result	Case studies (consented)	Plateau case
Story arc	Before → turning point → during → now	Experience stories	Consented patient story
AIDA	Attention → Interest → Desire → Action	Landing pages, emails	Self-pay landing page
PAS	Problem → Agitate (gently) → Solve	Cost, comparison	Affordability page
BAB	Before → After → Bridge (situations only)	How it works	Confused → clear answer → provider consult
FAB	Features → Advantages → Benefits	Service, pricing	Online visits → no waiting room → fits your schedule
4 Cs	Clear, Concise, Compelling, Credible	Checking any copy	Headline check
4 Us	Useful, Unique, Ultra-specific (+ Urgent only if real)	Headlines	"How to measure your compounded semaglutide dose"
Clinical neutrality	Remove hype, hedge, balance	Every medical sentence	"May reduce appetite; nausea is common at first"

Appendix B: The 38 Content Types at a Glance

#	Type	Funnel	Review	Suggested length (words)	Skeleton chapter
1	Service page	BOFU	Medical + legal	800–1,500	33
2	Medication page	BOFU	Medical + legal	1,000–1,800	33
3	Landing page	BOFU	Medical + legal	400–900	33
4	Cost / pricing page	BOFU	Legal	500–1,000	33
5	How-it-works page	BOFU	Legal (+ ops)	400–800	33
6	Eligibility / assessment copy	BOFU	Medical + legal	Per field: 5–25 words	32, 33
7	Comparison page (options)	MOFU/BOFU	Medical + legal	1,200–2,000	33, 34
8	Pillar page	TOFU/MOFU	Medical	2,500–4,000	34
9	Guide	TOFU/MOFU	Medical	1,200–2,000	34
10	Tutorial / how-to	MOFU/Retention	Medical	600–1,200	34
11	FAQ page	All	Medical	40–80 per answer; 8–20 questions	31
12	Comparison article	MOFU	Medical (+ legal if brands)	1,200–2,000	34
13	Listicle	TOFU	Medical if health claims	800–1,500	34
14	Myth vs fact	TOFU	Medical	900–1,500	34
15	Checklist	MOFU	Medical	400–900	34
16	Glossary / vocabulary	TOFU	Medical	15–40 per term	34
17	Definition page	TOFU	Medical	500–1,000	25, 34
18	Side-effect guide	MOFU/Retention	Medical + legal	1,000–1,600	34

#	Type	Funnel	Review	Suggested length (words)	Skeleton chapter
19	Timeline / what to expect	MOFU/Retention	Medical	1,000–1,800	34
20	Recipe / meal plan	TOFU	Dietitian/medical if claims	400–900 per recipe	34
21	Lifestyle article	TOFU	Medical if claims	900–1,500	34
22	News article	TOFU/MOFU	Legal (+ medical)	400–800	34
23	Report	MOFU	Medical + legal	1,500–3,000	35
24	Whitepaper	MOFU	Medical + legal	3,000–6,000	35
25	Interview	MOFU	Medical + legal	800–1,500	35
26	Expert opinion	MOFU	Medical	700–1,200	35
27	Case study	MOFU/BOFU	Medical + legal + consent	700–1,200	35
28	Success story	MOFU/BOFU	Medical + legal + consent	300–700	27
29	Testimonials page	BOFU	Legal	Varies (short entries)	35
30	Profile page (doctor/author)	Trust	Provider + HR	200–500	35
31	Editorial / medical review policy	Trust	Medical + legal	800–1,500	35
32	Policy page	Trust	Legal	As legal requires (+ 100–200-word summary)	35
33	Help article	Retention	Ops	300–700	10
34	Troubleshooting	Retention	Ops (+ medical)	300–700	10
35	Technical documentation	Trust	Legal/ops	600–1,500	10

#	Type	Funnel	Review	Suggested length (words)	Skeleton chapter
36	Video script	All	By topic	60–90 s ≈ 150–220 spoken words	36
37	Microcopy	BOFU	Legal/medical for health fields	1–25 words per item	32
38	Meta, alt text, email, social	All	By topic	Meta 50–60/140–160 chars; alt 1 sentence; email 150–300	36

Rule: every content type must have a unique primary keyword; if not, it should not be created. Lengths are starting estimates; write what fully answers the reader, then stop.

Appendix C: Word Swaps and Risky Words

C.1 Plain-word swaps

Instead of	Use	Instead of	Use
administer	give, take	individuals	people
adverse event	side effect	in order to	to
approximately	about	obtain	get
assist	help	optimal	right for you
commence	start	prior to	before
discontinue	stop	subsequently	later, then
due to the fact that	because	sufficient	enough
facilitate	help, make easier	utilize	use

C.2 Risky words → safer wording

❌ Risky	✅ Safer
guaranteed / guarantee	results vary; not guaranteed
cure / cures	helps manage; a treatment option
melts fat / burns fat	may reduce appetite and support weight loss
fast / rapid / in 30 days	over time; gradually
miracle / magic / breakthrough	a prescription medication; a new option
no side effects / 100% safe / risk-free	side effects can occur; talk to your provider
works for everyone	may not be right for everyone
best / #1 / top-rated (unsourced)	licensed; doctor-led; (sourced rating)
clinically proven (for compounded)	in [named trial] of [brand product]...; compounded versions weren't studied
FDA-approved semaglutide (for ours)	compounded semaglutide, not FDA-approved
generic Ozempic / same as Wegovy	compounded semaglutide
easy approval / instant prescription	a provider reviews your health history
obese person / the obese	person with obesity
stubborn fat / problem areas / bikini body	your health goals
guilt-free / cheat day	a lighter option / a treat
elderly	older adults
suffers from	has; lives with

C.3 Filler to cut

In today's world · It is important to note that · When it comes to · In this article, we will discuss · At the end of the day · In conclusion · Many people believe (unsourced) · Look no further · Without further ado · Let's dive in

Appendix D: Page Skeletons (Quick Reference)

Type	Skeleton
Service	H1 + subhead → answer block + disclaimer → CTA → how it works → what's included → price → who it's for/not for → safety → providers → trust → FAQs → final CTA + disclaimer
Medication	H1 (generic, "compounded") → what it is → how prescribed → how taken → side effects & safety → results context (brand trials named) → price → pharmacy line → FAQs → disclaimer → sources
Cost	Price first → included → not included → billing → why compounded can cost less (reviewed) → FAQs
How it works	Assessment → provider review → prescription if appropriate → delivery → ongoing care
Blog guide	H1 → trust box → answer-first intro → key points → question H2s with answer blocks → safety → bottom line → CTA by funnel → FAQs → disclaimer → sources
Comparison	Answer box → table → what each is → differences → who might consider each (provider decides) → bottom line
Side-effect guide	What → why → how common → what helps → how long → red flags → when to call → FAQs
Myth vs fact	Myth H2 → Fact answer with source (paired) → bottom line
Profile	Name & credentials → licences → role → experience → approved quote → reviewed content
Policy	Approved plain summary → sections → dates/version

Appendix E: Qualifier and Disclaimer Library

E.1 Qualifiers

may · can · might · often · usually · commonly · some people · many people · in some cases · depends on · varies · results vary · when appropriate · if a provider decides it's right for you · not guaranteed · talk to your provider · in the study · research suggests · is associated with

E.2 Disclaimers

Where	Line
Treatment mention	"Compounded medications are not FDA-approved. A licensed provider decides whether treatment is appropriate."
Brand trial results	"Results are from studies of [brand product]; compounded versions were not studied in these trials."
Educational article	"This article is for education and is not medical advice. Talk to your provider about your health."
Testimonial	"Shared with permission. Individual experiences vary."
Brand name in education	"Ongo Weight Loss does not offer [brand name]."
Eligibility	"A prescription is not guaranteed. A provider reviews your health history."
Pharmacy	"Prepared by a licensed U.S. compounding pharmacy."

Appendix F: Templates

F.1 Brief

Field	Entry
Working title	
Content type	
Primary keyword (unique)	
Secondary keywords	
Intent / funnel	
Persona	
Parent page / hub	
Internal links (to/from)	

Field	Entry
CTA (exact wording + event)	
Reviewers needed	
Deadline	

F.2 Purpose statement

"This page exists to ____ for ____, so they can ____."

F.3 Keyword table

Keyword	Type	SV	KD	Intent	Where used
	Primary				
	Secondary				

F.4 Claim log

#	Claim (as written)	Type	Source link	Source says	Date checked	Reviewer note

F.5 Approval log

Page	Version	Reviewer	Decision	Date	Notes

F.6 Submission pack checklist

☐ Purpose statement · ☐ Keyword table · ☐ Claim log · ☐ Information gain note · ☐ Internal links · ☐ Meta title/description/slug · ☐ Image suggestions + alt text · ☐ FAQs · ☐ Trust box details · ☐ Open questions

Appendix G: Source Whitelist and Blacklist

✅ Use (top to bottom)	❌ Never as evidence
FDA (labels, Drugs@FDA, DailyMed, safety communications)	Competitor blogs and product pages
CDC, NIH, HHS, MedlinePlus	Forums, Reddit, Quora
Peer-reviewed journals via PubMed (NEJM, JAMA, Lancet, Obesity)	Social media posts and influencers
Clinical guidelines and medical societies (Obesity Medicine Association, The Obesity Society, ADA, Endocrine Society)	AI tool answers (until verified at a source)
Established medical references (Mayo Clinic, Cleveland Clinic, StatPearls)	Content farms and "listicle" sites
Reputable surveys (KFF, Pew)	Press releases without data
Quality news and law-firm analyses (confirm with originals)	Wikipedia (use only to find original sources)

Appendix H: Checklist Tests and the Pre-Submit Sheet

H.1 The tests

Test	Letters
PURPOSE	Beneficial · Clear · Honest · Focused · Achieved
MASK	Match intent · Avoid deception · Serve one goal · Knowledge-first
HUMAN	Helpful · Understandable · Meaningful depth · Accurate · Not manipulative
SAFETY	Specific · Accurate & current · Framed with limits · Evidence-aligned · Transparent · You-first
CLEAR	Correct · Limited · Explicit · Accountable · Reader-safe
RIGHT	Relevant · Intent-matched · (Right audience) · Helpful next step · Timely
STOP	Safety risk · Truth distortion · Overconfidence · Purpose clarity

Test	Letters
FALSE	Fully guaranteed · Absolute language · Overreach · Selective facts · Emotional exploitation
NATURAL	Natural language · Audience clarity · Topic focus · Useful phrasing · Reader flow · Authentic voice · Logical structure
VERIFY	Validate · Examine scope · Recheck dates · Identify assumptions · Flag uncertainty · Your responsibility
GATE	Goal · Accuracy · Trust · Ecosystem
READY	Right purpose · Error-free facts · Aligned intent · Defensible trust · YMYL-appropriate

H.2 One-page pre-submit sheet

☐	Check
☐	Purpose statement written and served by every section
☐	Answer-first intro (<100 words) + key points
☐	Question H2s with 40–60-word answer blocks; one H1; no skipped levels
☐	Plain language; grade 6–8 for money pages; read aloud
☐	Every claim in the claim log with source and date
☐	"Compounded" + "not FDA-approved" + "provider decides" where treatment appears
☐	No brand drug names in commercial copy; "does not offer" line in reviewed education
☐	No outcome promises; trial results named and scoped
☐	Partner pharmacy not named; prices match the site
☐	People-first language; risky-word search done
☐	Safety section with red flags (medical topics)
☐	Information gain added and noted
☐	3–8 internal links with descriptive anchors; sources linked
☐	Meta title, description, slug; alt text; FAQs (visible)

☐	Check
☐	Trust box details; honest dates
☐	Submission pack complete; reviewers assigned
☐	Final honest question: proud, not defensive

Appendix I: Before and After Rewrites

#	❌ Before	✅ After	What changed
1	"Lose weight fast with our FDA-approved semaglutide!"	"Compounded semaglutide may help with weight loss when a provider decides it's right for you. It's not FDA-approved."	Removed speed and false approval; added provider
2	"Same active ingredient as Wegovy, for less."	"Compounded semaglutide, prepared by a licensed U.S. pharmacy. [Price as on site], including provider care."	No brand comparison; factual value
3	"In today's world, obesity is a huge problem for many obese people."	"About 2 in 5 U.S. adults have obesity (CDC)."	Cut filler; people-first; sourced
4	"Side effects are rare and mild."	"Side effects like nausea are common at first and often ease. Contact your provider if they're severe."	Honest, actionable
5	"You will lose 15% of your body weight."	"In the STEP 1 trial of brand semaglutide, participants lost 14.9% on average over 68 weeks. Compounded versions weren't studied, and results vary."	Trial named; scoped
6	"Guided. Tailored. Care."	"A licensed provider guides your plan and checks in as you go."	Meaning over slogan
7	"Real Transformations, Backed by Science"	"What patients say about their care (shared with permission)"	Honest framing
8	"Click here to get your prescription!"	"Start my free assessment"	Honest, specific CTA

#	❌ Before	✅ After	What changed
9	"Invalid input."	"Please enter your date of birth as MM/DD/YYYY."	Fixable error
10	"Melt stubborn belly fat with this miracle shot."	"GLP-1 medications can reduce appetite. A provider can tell you if one is right for you."	No hype or shame

Appendix J: Case Appendices

Cases J1–J3 are **real findings** from our 30 October 2026 review. Cases J4–J8 are **training scenarios**. 🔵

J1 · Real: the disclaimer far from the product

Situation: On our homepage, "Compounded semaglutide and compounded tirzepatide are not FDA-approved medications" appears in the FAQ and footer, not near the product cards. **Risk:** Omission near the claim; regulators and readers may miss it. **Action:** Add the disclaimer directly under each product card; keep the FAQ version. **Lesson:** Disclaimers travel with the claim.

J2 · Real: unsourced trust numbers

Situation: "4.9/5 Ratings" and "Trusted by 11945+ people" appear without a source. **Action:** Show the platform, review count and date (e.g., "4.x on [platform], [N] reviews, as of [date]"), or remove. **Lesson:** Every number on a money page has a source or goes.

J3 · Real: blog without review signals

Situation: Five posts with a doctor byline but no named reviewer or dates; listing page `noindex, nofollow`. **Action:** Add trust boxes (author, real reviewer, honest dates); SEO reviews indexing; plan missing content types (Chapter 34). **Lesson:** E-E-A-T must be visible and true.

J4 · Scenario: the SEO asks for "Wegovy" in the title

Situation: "Wegovy alternatives" has high search volume. **Action:** Explain the launch-phase rule; propose a reviewed educational page ("Compounded vs FDA-approved GLP-1 medications: what's different") with the "does not offer" line, approved by legal; commercial titles stay brand-free. **Lesson:** Search volume never overrides compliance.

J5 · Scenario: AI draft with an invented study

Situation: An AI brainstorm cites "a 2025 Harvard study showing 25% loss with compounded semaglutide." **Action:** Search PubMed; no such study; remove; log the incident. **Lesson:** Open every source.

J6 · Scenario: reviewer cuts your favourite paragraph

Situation: Medical reviewer removes a section on "speeding up results". **Action:** Ask why; learn the issue (implied outcome promise); offer a safer alternative focused on habits. **Lesson:** Reviewers protect readers; ask to learn.

J7 · Scenario: a patient story arrives by email

Situation: A happy patient emails a long story and photos. **Action:** Don't publish; forward to the team lead; consent and release process (Chapter 27); experience-focused edit; no body photos; legal and medical review. **Lesson:** Real and consented, or not at all.

J8 · Scenario: price changed after publishing

Situation: Pricing updated; 12 blog posts mention the old price. **Action:** Search all content for the old price; update; log; note in the refresh log; propose linking to the pricing page instead of repeating prices in blogs. **Lesson:** Prices live in one place; link to it.

Appendix K: Ongo Weight Loss Voice and Tone One-Pager

Item	Rule
Voice	Warm · Clear · Honest · Steady
One line	A knowledgeable, kind provider explaining things clearly over a cup of tea
Point of view	"You" for the reader; "we"/"Ongo Weight Loss" for us; third person for research
Tone dial	Education: warm/clinical, low sales · Money pages: clear, moderate sales · Safety: clinical, zero sales
Always	Remove shame; answer first; qualify uncertainty; name sources; one clear next step
Never	Hype, fear, shame, brand drug names commercially, outcome promises, emojis on web pages

Item	Rule
Spelling	U.S. English
Headings	Sentence case for blog H2/H3
Numbers	Numerals for doses, prices, percentages, 10+
Brand name	"Ongo Weight Loss" (never "Ongo" alone)
Medication	"compounded semaglutide", "compounded tirzepatide"; "not FDA-approved"; "a provider decides"

Appendix L: Content QA and SEO Audit Sheet

Use one sheet per piece. Copy it into the Google Doc (or a Sheet) and fill the ✅ column. **Owners:** W = writer · SEO = on-page SEO · MED = medical reviewer · LEG = legal reviewer · DES = graphic/UI-UX designer · DEV = developer · SOC = social team · OFF = off-page SEO · TL = team lead. **When:** B = before submitting · P = before publishing · A = after publishing.

L.1 Introduction (the brief)

#	Check	Pass rule	Owner	When	Ch	✅
1	Topic and purpose statement	"This page exists to..." written	W	B	9	☐
2	Content type	One of the 38 types	TL/W	B	10	☐
3	Search intent	Info / commercial / transactional / navigational, checked against the top 10	W/SEO	B	11	☐
4	Funnel stage	TOFU / MOFU / BOFU / retention	W	B	11	☐
5	Primary category and parent hub	From the Book Three content map	SEO	B	40	☐
6	Primary keyword (unique)	Not targeted by any other page	SEO/W	B	10, 40	☐
7	Suggested length	Within the Appendix B range or justified	W	B	9	☐

#	Check	Pass rule	Owner	When	Ch	✅
8	Publish date planned; document URL	Google Doc link added	TL	B	—	☐

L.2 Roles

#	Check	Pass rule	Owner	When	Ch	✅
9	Assignor, writer named	—	TL	B	39	☐
10	Reviewers named by type	SEO, medical (if health claims), legal (if prices/testimonials/policies)	TL	B	39	☐
11	Reviewer qualifications match the claims	Per 39.3	TL	B	39	☐

L.3 Head and meta

#	Check	Pass rule	Owner	When	Ch	✅
12	H1	One H1; contains primary keyword naturally; one clear promise	W	B	15, 28	☐
13	Meta title	50–60 characters, ≈580 px; keyword near start; "| Ongo Weight Loss" at end	W/SEO	B	36	☐
14	Meta description	140–160 characters (≈920–990 px); answer/benefit + qualifier + soft CTA	W/SEO	B	36	☐
15	No brand drug names or promises in meta	Commercial pages: none	W	B	4	☐
16	Open Graph title/description/image	Set (Book Four)	SEO/DES	P	36	☐

L.4 Keywords in body

#	Check	Pass rule	Owner	When	Ch	✅
17	Primary keyword in the first 100 words	Once, naturally	W	B	12	☐
18	Primary keyword or close variant in at least one H2/H3	Natural question form preferred	W	B	12, 15	☐
19	Natural use, no stuffing	Passes the read-aloud test; **no density target** (replaces "1–2%")	W/SEO	B	12	☐
20	Related terms and questions covered	People Also Ask / GSC questions answered (replaces "LSI")	W	B	12	☐

L.5 Content quality and compliance

#	Check	Pass rule	Owner	When	Ch	✅
21	Headings in order	One H1; H2 → H3; no skipped levels	W	B	15	☐
22	Answer-first intro + key points	Intro <100 words; 3–5 key points	W	B	20, 29	☐
23	Answer block under each H2	40–60 words	W	B	20	☐
24	Original, helpful, human-reviewed	Purpose served; AI help disclosed to editor (replaces "human written?")	W/TL	B	3, 17	☐
25	Clean, concise, error-free	3 editing passes done	W	B	37	☐
26	Plain language and readability	Grade 6–8 for money pages; read aloud	W	B	18, 19	☐
27	Scannable	Short paragraphs, lists, tables, 1–3 bold phrases per section	W	B	30	☐
28	Pain points and reader situation addressed	Persona's worry answered	W	B	6	☐
29	Information gain	At least one, noted in submission	W	B	26	☐

#	Check	Pass rule	Owner	When	Ch	✅
30	People-first, shame-free language	Risky-word search clean	W	B	7	☐
31	Claim types and qualifiers	No PROHIBITED claims; conditionals qualified	W/MED	B	4, 23	☐
32	Compounded wording	"Compounded" + "not FDA-approved" + "provider decides" where treatment appears	W/LEG	B	4	☐
33	Brand drug names	None in commercial copy; reviewed education only with "does not offer" line	W/LEG	B	4	☐
34	Trial results	Named, scoped, "compounded versions weren't studied"	W/MED	B	8	☐
35	Partner pharmacy not named; prices match the live site	—	W/LEG	B	4	☐
36	Safety section	Red flags + "contact your provider" for medical topics	W/MED	B	8	☐
37	Fact check	Claim log complete; VERIFY done	W/MED	B	16, 38	☐
38	Medical review	Approved; reviewer named in approval log	MED	P	39	☐
39	Legal review	Approved where required	LEG	P	39	☐
40	Trust box and honest dates	Author, reviewer, published, last medically reviewed	W/DEV	P	35, 38	☐
41	Disclaimers next to claims	—	W	B	4	☐
42	FAQs	4–8, visible, 40–80 words each	W	B	31	☐
43	CTA	Approved wording; matches GA4 event; strength fits funnel	W/SEO	B	32	☐

L.6 Links

#	Check	Pass rule	Owner	When	Ch	✅
44	Internal links	3–8 contextual; parent, sibling, next funnel step; descriptive anchors	W/SEO	B	40	☐
45	External links	To sources on the evidence ladder; no competitors	W	B	13	☐
46	External link attributes	New-tab links use `rel="noopener"`; `nofollow` / `sponsored` / `ugc` only for untrusted, paid or user-generated links (not needed for FDA, NIH, PubMed citations)	DEV/SEO	P	38	☐
47	Broken links	None	SEO	P / A	43	☐
48	No UTMs on internal links	—	W/SEO	B	40	☐

L.7 Media

#	Check	Pass rule	Owner	When	Ch	✅
49	Image briefs written	One per image (purpose, content, placement, alt, caption)	W	B	30	☐
50	Images created	By graphic or UI/UX designer; no before/after bodies; consent for real people	DES	P	30	☐
51	Image file optimised	Format/size/dimensions per Book Four (e.g., WebP, compressed)	DES/DEV	P	—	☐
52	Alt text	Describes the image; decorative images empty	W/DEV	B/P	36	☐

L.8 URL

#	Check	Pass rule	Owner	When	Ch	✅
53	Slug	Lowercase, hyphens (not underscores), primary keyword, short and readable	W/SEO	B	36	☐

#	Check	Pass rule	Owner	When	Ch	✅
54	Full URL length	Under ~100 characters (≈385 px display)	SEO	B	—	☐
55	URL matches the content map section	Correct folder (Book Three)	SEO	B	40	☐

L.9 Schema

#	Check	Pass rule	Owner	When	Ch	✅
56	Schema type chosen	e.g., Article/MedicalWebPage, FAQPage, BreadcrumbList (Book Four)	SEO	P	31	☐
57	Schema matches visible content	FAQs, author, dates identical	SEO/DEV	P	31	☐
58	Validated	Rich Results Test / Schema validator link saved	SEO	P	—	☐

L.10 Proofreading sign-off

#	Check	Pass rule	Owner	When	Ch	✅
59	Writer proofread	3 passes + search list	W	B	37	☐
60	Assignor proofread	Brief fulfilled	TL	P	—	☐
61	Reviewer proofread	Approval log row	SEO/MED/LEG	P	39	☐

L.11 After publishing: social, technical and off-page (others own these)

#	Check	Pass rule	Owner	When	Ch / Book	✅
62	Shared on social	Facebook, Instagram, Pinterest, YouTube (if video), LinkedIn as planned	SOC	A	Book Five	☐
63	Page indexed	URL Inspection in GSC	SEO	A	Book Four	☐
64	Crawlable content	Main text in HTML; not hidden behind scripts	DEV/SEO	P	Book Four	☐

#	Check	Pass rule	Owner	When	Ch / Book	✅
65	Page speed and mobile	Core Web Vitals pass; mobile layout checked	DEV	P	Book Four	☐
66	Canonical / redirects	Self-canonical; no chains	SEO/DEV	P	Book Four	☐
67	Analytics tracking	GA4 page view + CTA events firing; no personal data	SEO/DEV	P	Book Four	☐
68	No intrusive elements	No ads or pop-ups hurting the experience	DEV	P	Book Four	☐
69	Backlinks (later)	Quality, relevant, natural anchors; no redirected targets	OFF	A	Book Two	☐
70	Next refresh date set	Money pages monthly; medical every 6 months	TL/W	A	43	☐

Appendix M: Sample Article, Fully Annotated

This is a training sample showing what a finished submission looks like. Medical statements must be verified by the medical reviewer before any similar article is published.

M.1 The Google Doc header block

```
CONTENT TYPE:   Side-effect guide (blog), MOFU / retention
PRIMARY KW:     semaglutide nausea
SECONDARY:      how long does semaglutide nausea last
                what helps nausea on semaglutide
                foods to avoid on semaglutide
PURPOSE:        This page exists to explain why nausea happens on
                compounded semaglutide, how long it may last, what
                helps, and when to contact a provider.
META TITLE:     Semaglutide Nausea: Causes & What Helps
                | Ongo Weight Loss                    (58 chars)
META DESC:      Nausea is common in the first weeks of semaglutide.
                Learn why it happens, how long it may last, what
                helps, and when to call your provider for help.
                                                    (≈148 chars)
SLUG:           /blog/semaglutide-nausea/
PARENT HUB:     Compounded semaglutide hub
AUTHOR:         [assigned]
MED. REVIEWER:  [assigned, MD/DO]
CTA:            "Message my care team" (patients)
                + soft link to the medication page
SOURCES:        1. STEP 1 trial, NEJM 2021
                   doi.org/10.1056/NEJMoa2032183
                2. [FDA label / reviewer-provided source]
                3. [Other sources, all linked]
```

> 📝 **Why:** SEO and developers copy these fields exactly. The purpose line keeps every paragraph on track. Sources are listed before anyone reads the draft.

M.2 The article

H1: Semaglutide nausea: why it happens, how long it lasts, and what helps

Written by [Author] · Medically reviewed by [Reviewer], MD · Published [date] · Last medically reviewed [date]

> 📝 **Why:** One H1 with the primary keyword and one clear promise. The trust box shows Who and When (Chapters 3, 35).

Nausea is one of the most common side effects of semaglutide, especially in the first weeks and after a dose increase. For many people, it eases as the body adjusts. Here's why it happens, how

long it may last, what can help, and when to contact your provider.

> 📝 **Why:** Answer-first intro under 100 words; keyword in the first sentence; qualifiers ("many", "may") (Chapters 20, 29).

Key points

- Nausea is common when starting semaglutide or increasing the dose.
- It often improves over a few weeks, but everyone is different.
- Smaller meals, eating slowly and avoiding greasy foods can help.
- Contact your provider if nausea is severe, doesn't improve, or you can't keep fluids down.

> 📝 **Why:** Complete facts, including the safety point. No marketing bullet.

H2: Why does semaglutide cause nausea?

Semaglutide slows how quickly food leaves your stomach and affects the brain signals that tell you you're full. Food staying in the stomach longer can feel like fullness, or like nausea.

Think of it as a "you're full" message arriving sooner and louder than you're used to. Your body often needs time to adjust to it.

> 📝 **Why:** Answer block first (≈40 words), then an analogy followed by the real fact (Chapters 20, 25).

H2: How long does nausea last on semaglutide?

For many people, nausea is strongest in the first few weeks and for a short time after each dose increase, then eases. It varies from person to person. If it lasts or gets worse, talk to your provider; they may adjust your plan.

> 📝 **Why:** Hypophora-style question heading; hedged, specific answer; next step included (Chapters 21, 23).

H2: What helps with semaglutide nausea?

1. **Eat smaller meals.** A smaller plate can be easier on your stomach.
2. **Eat slowly and stop when you feel full.** Fullness comes sooner on semaglutide.
3. **Go easy on greasy, fried and very sweet foods.** These are common triggers.

4. **Sip water through the day.** Small sips can be easier than large glasses.
5. **Try bland foods when you feel queasy.** Crackers, toast, rice or bananas.
6. **Keep a simple symptom diary.** Note what you ate, when, and how you felt.

Day	What I ate	When nausea started	How bad (1–5)	What helped
Mon				

> 📝 **Why:** Numbered tips with bold leads (parallel structure). The diary template is the **information gain**: most competitor guides mention a diary; we give one ready to use (Chapters 26, 30).

H2: When should you contact your provider?

Contact your provider right away if you have:

- Severe stomach pain that doesn't go away
- Vomiting that stops you keeping fluids down
- Signs of dehydration, such as dizziness or very dark urine
- Nausea that gets worse instead of better

If you think you're having a medical emergency, call 911.

> 📝 **Why:** Safety section in plain words. The medical reviewer confirms the exact red-flag list (Chapter 8).

H2: Can nausea mean my dose is too high?

Nausea can be more noticeable after a dose increase. Only your provider should decide whether to change your dose. Please don't skip, split or change doses on your own.

> 📝 **Why:** Answers a real worry without giving personal medical advice (Chapter 44, Red List #7).

H2: The bottom line

Nausea is common on semaglutide, especially early on, and it often eases. Smaller, slower meals and gentle food choices help many people. If it's severe or doesn't improve, your provider can help.

Current patients: [Message my care team]. *Exploring treatment?* Read about [how compounded semaglutide works at Ongo Weight Loss].

> 📝 **Why:** Bottom line without new claims; support CTA for patients and a soft internal link for readers still exploring (Chapters 11, 29, 32).

H2: Frequently asked questions

Is nausea a sign the medication is working? Not necessarily. Nausea is a side effect, not a measure of how well the medication works. Some people have little or no nausea and still respond to treatment. Your provider tracks your progress.

Can I take anti-nausea medicine? Ask your provider first. They can tell you whether an over-the-counter or prescription option is safe with your other medications.

Does ginger help? Some people find ginger tea or chews soothing. Evidence is limited, so try it as a gentle option and talk to your provider if nausea continues.

Will nausea come back when my dose increases? It can. Many people notice a brief return after a dose change. The same tips usually help.

> 📝 **Why:** Real questions; answer in the first sentence; 30–60 words each; no sales FAQs (Chapter 31).

Compounded medications are not FDA-approved. A licensed provider decides whether treatment is appropriate. This article is for education and is not medical advice.

Sources: numbered, linked list (from the header).

Related articles: [Constipation on GLP-1 medications] · [Your first month on compounded semaglutide] · [High-protein breakfasts that are easy on your stomach]

> 📝 **Why:** Disclaimer near the treatment content; sources; three internal links to siblings (Chapter 40).

M.3 Image briefs (for the designer)

#	Purpose	Type	Placement	Alt text
1	Show the six tips at a glance	Infographic	After "What helps"	"Infographic: six ways to ease nausea on semaglutide"

#	Purpose	Type	Placement	Alt text
2	Downloadable symptom diary	Table graphic	After the diary table	"Printable symptom diary with columns for food, timing and how you felt"

M.4 Audit result (Appendix L, sample)

Area	Result
Brief and roles	✅ Complete; medical reviewer assigned
Head and meta	✅ Title 58 characters; description ≈148 characters; no brand drug names
Keywords	✅ Natural; in H1, intro, one H2
Quality and compliance	✅ Answer-first, qualifiers, safety, info gain, people-first; ✅ disclaimer
Links	✅ 4 internal, sources linked
Media	✅ 2 image briefs with alt text
URL	✅ /blog/semaglutide-nausea/
Next steps	Medical review → legal check of disclaimer → SEO schema → publish → social

Appendix N: Practice Assignments by Level

Do these in a Google Doc and share them with your team lead. "What good looks like" tells you how your work will be checked.

Level 1 · Foundation

#	Assignment	What good looks like
1	Sort 20 sentences (provided by your team lead) into FACTUAL, INTERPRETIVE, CONDITIONAL or PROHIBITED	Every PROHIBITED line caught; conditionals given a qualifier
2	Rewrite 10 lines from our homepage in the Ongo Weight Loss voice	Warm, clear, honest; no hype; compliance words where needed
3	Write a 150-word explanation of how GLP-1 medications work for Linda (58)	Grade 6–8; analogy + real fact; "provider decides"

#	Assignment	What good looks like
4	Find 5 non-people-first phrases on any weight-loss site and rewrite them	People-first; no shame words

Level 2 · Preparing to write

#	Assignment	What good looks like
5	Turn a one-line topic into a full brief (Appendix F.1)	Type, unique keyword, intent, funnel, links, CTA, reviewers
6	Research 15 real questions for "tirzepatide side effects"	From PAA, AlsoAsked, GSC; grouped Why/What/How
7	Audit the top 5 results for one keyword (Chapter 14 method)	Structure noted; gaps found; risky claims listed (not copied)
8	Build an outline + claim log for a "first month on semaglutide" timeline	Question H2s; safety section; every claim sourced on the evidence ladder

Level 3 · Writing techniques

#	Assignment	What good looks like
9	Rewrite 5 history-first intros as answer-first	Answer in sentence one; under 100 words
10	Write 5 hypophora passages for common objections (cost, injections, safety, compounded, insurance)	Real question; immediate, honest answer
11	Write one landing-page section with AIDA and one with FAB	Passes the 5-box ethics test
12	Write 10 headline options for one topic and pick the best	4 Cs pass; keyword early; no hype
13	Write 6 FAQs (40–80 words each)	Answer first; quotable sentences
14	Write microcopy for a 5-field form (labels, help text, errors)	Specific, kind, fixable errors

Level 4 · Page types, review and growth

#	Assignment	What good looks like
15	Write a full side-effect guide using Appendix M as the model	Passes Appendix L before submitting
16	Edit a deliberately bad draft (from your team lead) in three passes	All Red List items caught; search list run
17	Write a provider profile from a verified CV	Facts only; no subjective praise
18	Plan a refresh of one live page using GSC data	New questions added; real changes; honest dates
19	Write 3 image briefs and alt texts for one article	Clear purpose; compliant visuals; descriptive alt
20	Present your cluster plan (6–8 pages) to the team lead	Unique keywords; hub-and-spoke links; funnel path

Glossary (A to Z)

Term	Simple meaning	Ch.
Accessibility	Making content usable by people with disabilities	41
Active voice	The doer comes first ("A provider reviews...")	18
Aggregated	Combined into patterns without personal details	26
AIDA	Attention, Interest, Desire, Action	24
AI detector	A tool that guesses if text was AI-written (often unreliable)	17
AI Overview	Google's AI-generated summary at the top of some results	31
Alt text	Text describing an image for screen readers	36
Anchor text	The clickable words of a link	38
Answer block	The 1–2 sentences right after a heading that answer it	15

Term	Simple meaning	Ch.
Anticipatory heading	A heading phrased as the reader's question	15
Approval log	Record of who approved which version, when	39
BAB	Before, After, Bridge	24
Benchmark	A standard to compare against	14
BLUF	Bottom Line Up Front	20
BMI	Body mass index: a screening measure of weight for height	8
Bottom line	A short concluding summary	29
Breaking the fourth wall	Speaking directly and openly to the reader about their situation	22
Brief	Instructions for a piece of content	1
Byline	The line naming who wrote or reviewed a page	3
Cadence	How often something is done	43
Callout box	A highlighted box for tips or warnings	30
Cannibalisation	Two of our pages competing for the same keyword	10
Citation	A reference to the source of a fact	38
Claim	A statement about what something is or does	4
Claim log	A table of every claim with its source	16
Clear Communication Index	CDC's checklist for clear health materials	6
Clickbait	A misleading headline that overpromises	28
Clinical neutrality	Calm, balanced, evidence-based medical voice	23
Cluster	A group of linked pages around one topic	43
Cognitive accessibility	Making content easy to understand and remember	41
Commercial investigation	Comparing before deciding	11
Commodity content	Content that says the same as everyone else	26

Term	Simple meaning	Ch.
Competitor audit	A structured review of competitors' content	14
Composite persona	A made-up planning character based on research	27
Compounded medication	A medication prepared by a licensed pharmacy for a patient; not FDA-approved	4
Consultant mode	Using AI to suggest, not to write	17
Content optimiser	A tool suggesting topics/terms based on top results	42
Content refresh	Updating a page with real improvements	43
Content type	The kind of content (guide, FAQ, comparison...)	10
Content writer	The person who writes the words on pages, posts and emails	1
Conversational tone	Writing like a natural, respectful conversation	22
Copy	The words on a page, especially marketing words	1
Correction note	A visible note explaining a fixed error	38
Correlation	Two things happening together, not necessarily cause	23
Counter-point	A careful correction of a common belief	26
Credentials	Verified qualifications and licences	35
CTA	Call to action: what you ask the reader to do	32
CTR	Click-through rate: clicks ÷ impressions	43
Dark pattern	A design or copy trick that pressures or misleads users	24
Descriptive link text	Link words that say where the link goes	41
Disclaimer	A sentence that clarifies limits or status	4
DOI	A permanent link ID for research papers	38
Editorial policy	Public rules for how content is created	35
E-E-A-T	Experience, Expertise, Authoritativeness, Trust	3

Term	Simple meaning	Ch.
Empty state	What a screen shows when there's no content yet	32
End stack	The ordered elements at the end of an article	29
Engaged session	A GA4 visit with meaningful activity	43
Error message	Text explaining what went wrong and how to fix it	32
FAB	Features, Advantages, Benefits	24
Fact sheet	A reusable one-page summary of key facts on a topic	16
FAQ schema	Code describing FAQs for search engines	31
Featured snippet	A highlighted answer at the top of Google results	20
Feynman technique	Explaining something simply to find gaps in understanding	25
Filler	Words that add length but no meaning	7
Final honest question	The proud-or-defensive test before publishing	44
Flesch Reading Ease	A 0–100 score; higher is easier	19
Fourth wall	The invisible line between a work and its audience	22
F-pattern	A common eye-scanning shape on web pages	30
Fragment	An incomplete sentence used for effect	19
Freemium	Free basic version with paid upgrades	42
Gap	Something useful that no competitor covers well	14
GATE	Goal, Accuracy, Trust, Ecosystem check	44
GLP-1 receptor agonist	A medicine that acts like the GLP-1 hormone	8
Goldilocks rule	Not too short, not too long: just enough	9
H1, H2, H3	Heading levels from main title down	15
Hallucination	Confident but false AI output	17
Handoff	Passing finished copy to SEO and developers	40

Term	Simple meaning	Ch.
Headline	The main title readers see	28
Health literacy	How easily someone can find, understand and use health information	6
Hedge	A word that limits certainty ("may", "often")	23
Help text	A short explanation under a form field	32
Hierarchy	The order and nesting of headings	15
Hook	The line that makes a reader keep reading	29
House style	Agreed rules for spelling, numbers and formatting	5
Hub (pillar)	Main page for a topic that links to related pages	40
Hype word	An exaggerated word that overpromises	7
Hypophora	Asking a question and answering it immediately	21
Inclusions	What the price covers	33
Inclusive language	Words that respect all people	41
Infographic	A visual that explains information	30
Information gain	New, useful information not found in other top results	26
Intensifier	A word like "very" that adds emphasis but little meaning	19
Internal link	A link to another page on our site	40
Intro	The opening paragraph(s)	29
Inverted pyramid	Most important information first, least important last	20
Jargon	Specialist words most readers don't know	18
Key points box	3–5 bullets summarising the article	29
Key takeaways box	A short summary near the top of an article	14
Keyword difficulty (KD)	A tool's estimate of ranking difficulty	12

Term	Simple meaning	Ch.
Keyword stuffing	Overusing keywords unnaturally	12
Landing page	A focused page with one goal, often for campaigns	33
Last medically reviewed	The date a medical reviewer last checked the page	38
Leading question	A question that pushes one answer	21
Legal review	Legal checks compliance, claims and policies	39
Line edit	Improving sentences for clarity and flow	37
Listicle	An article built around a numbered list	34
LLM	Large language model, the technology behind ChatGPT, Claude, Gemini	17
Loaded word	A word that pushes emotion ("miracle", "shocking")	23
Long-tail keyword	A longer, more specific search phrase	12
Medical review	A licensed provider checks health content	39
Medical review policy	How and by whom medical content is checked	35
Message match	The page headline matches the ad or email promise	33
Meta description	The summary under a title in search results	36
Meta title	The title shown in search results (title tag)	28
Microcopy	Tiny bits of interface text (buttons, errors)	10
Money page	A page that leads directly to a service or sign-up	1
Myth vs fact	An article correcting common false beliefs	34
Needs Met (NM)	How well a page answers the searcher's need	2
North Star	The guiding goal for every decision	9
Objection	A reason a reader hesitates	21
Omission	Leaving out important information	23

Term	Simple meaning	Ch.
Outline	The planned structure of headings and points	15
Padding	Extra words that add length but not value	9
Page Quality (PQ)	How trustworthy and well made a page is	2
Parallel structure	List items written in the same grammatical form	30
PAS	Problem, Agitate, Solve	24
Pass	One read-through focused on one kind of problem	37
Passive voice	The action comes first ("...is reviewed by...")	18
PEEL	Point, Evidence, Explain, Link	25
Peer-reviewed	Checked by independent experts before publishing	13
People Also Ask (PAA)	Google's box of related questions	12
People-first content	Content made mainly to help readers	3
People-first language	Putting the person before the condition ("person with obesity")	7
Persona	A made-up but research-based example reader	6
Pillar page	A central hub page for a big topic	10
Placebo	Inactive comparison treatment	8
Plagiarism checker	A tool that finds copied text	42
Plain language	Writing the audience understands the first time	18
Plain Writing Act	U.S. law requiring clear government writing (2010)	18
Point of view (POV)	Who the text speaks as or to (I, we, you, they)	5
Prescribing label	Official FDA-approved drug information	13
Preview text	The line shown after an email subject	36
Primary keyword	The main search phrase a page targets	10
Primary source	The original study, label or agency statement	13

Term	Simple meaning	Ch.
Prompt	Instructions you give an AI tool	17
Proofreading	Fixing spelling, grammar and typos	37
PubMed	NIH's free database of medical research	13
Purpose statement	One sentence explaining why the page exists	9
Qualifier	A word that limits a claim ("may", "some")	4
Quality rater	A person hired by Google to rate search results	2
Quotable sentence	A sentence that makes sense on its own	31
RCT	Randomised controlled trial: strongest study type for cause and effect	8
Readability	How easy text is to read	19
Readability formula	A score based on word and sentence length (Flesch, SMOG)	6
Reading level	How many years of school a text roughly needs	6
READY	Right purpose, Error-free facts, Aligned intent, Defensible trust, YMYL-appropriate	44
Red List	Mistakes that must never go live	44
Reference manager	A tool for saving and citing sources (e.g., Zotero)	42
Register	How formal or informal language is	22
Release form	Signed permission to use someone's story	27
Reviewer	An expert who checks content before it goes live	1
Rhetorical question	A question asked for effect, not answered	21
Rhythm / burstiness	Variation in sentence length and shape	19
Scaled content abuse	Mass-producing pages to manipulate rankings	3
Scannability	How easily readers can find information by skimming	30

Term	Simple meaning	Ch.
Scope	Who and what a fact applies to	16
SCQA	Situation, Complication, Question, Answer	25
Screen reader	Software that reads screen content aloud	41
Search-engine-first content	Content made mainly to attract search traffic	3
Search intent	The reason behind a search	11
Search Quality Rater Guidelines	Google's public rulebook for raters	2
Search volume (SV)	How many times a phrase is searched per month	12
Secondary source	A summary of a primary source	13
Second person	Writing with "you"	22
Self-editing	Reviewing and improving your own draft	37
Sentence case	Only the first word and names capitalised	5
SERP	Search engine results page	11
Service page	A page describing a service and how to start	33
Show, don't tell	Using specific details instead of adjectives	25
Sign-off	Final approval to publish	39
Silent question	What the reader is thinking but hasn't asked	21
Skeleton	A standard structure for a content type	34
Slug	The end part of a URL	36
Spoke	A supporting page linked to the hub	40
Stand-alone sentence	A sentence that needs no surrounding context	31
STAR	Situation, Task, Action, Result	25
Story arc	The shape of a story: before, turning point, during, now	27

Term	Simple meaning	Ch.
Structural edit	Checking order, purpose and completeness	37
Submission pack	Everything you send with a draft	16
Substantiation	Evidence that supports a claim	4
Testimonial	A statement about someone's experience with us	27
Timeline article	A "what to expect" week-by-week guide	34
Title tag	The HTML element that sets the meta title	28
Titration	Slowly increasing a dose	8
TOFU / MOFU / BOFU	Top, middle and bottom of the funnel	11
Tone	How the voice adjusts to a situation	5
Transactional intent	Wanting to do or start something	11
Trust box	Author, reviewer and date information	14
Trust page	A page that shows who we are and how we work	35
Trust signal	Evidence that builds confidence (certifications, reviews)	33
Typicality disclosure	A note that results or experiences vary	27
VERIFY	Our six-step fact-check routine	16
Version	A numbered draft (v1, v2...)	39
Visual translation	Turning text into a table, list or chart	26
Voice	The brand's consistent personality in writing	5
Warm-up	Unnecessary lead-in before the point	20
WAVE	A free web accessibility checker	42
Weight bias	Negative attitudes toward people because of their weight	7
Weight stigma	Shame or discrimination linked to weight	7
Whitepaper	A deep, evidence-based document	35

Term	Simple meaning	Ch.
WHY → WHAT → HOW	The order people learn in	9
YMYL	Topics that can affect health, money, safety or society	2
ZETA	Zero-Error Tolerance Approach	44

MCQ Answer Keys (all chapters)

Try every quiz first, then turn the page upside down to check.

Chapter	Answers
1	1-A · 2-B · 3-C · 4-A · 5-D · 6-B · 7-B · 8-A · 9-D · 10-A
2	1-A · 2-C · 3-C · 4-B · 5-B · 6-C · 7-A · 8-C · 9-C · 10-A
3	1-D · 2-A · 3-A · 4-A · 5-C · 6-A · 7-A · 8-C · 9-D · 10-B
4	1-B · 2-A · 3-A · 4-B · 5-C · 6-C · 7-D · 8-A · 9-B · 10-C
5	1-D · 2-A · 3-A · 4-B · 5-C · 6-D · 7-B · 8-D · 9-A · 10-D
6	1-B · 2-B · 3-A · 4-C · 5-B · 6-D · 7-C · 8-D · 9-B · 10-A
7	1-B · 2-B · 3-D · 4-A · 5-B · 6-D · 7-C · 8-B · 9-D · 10-D
8	1-B · 2-B · 3-D · 4-D · 5-B · 6-D · 7-B · 8-D · 9-B · 10-A
9	1-B · 2-C · 3-D · 4-B · 5-B · 6-D · 7-A · 8-B · 9-D · 10-B
10	1-C · 2-C · 3-D · 4-C · 5-A · 6-D · 7-A · 8-A · 9-C · 10-A
11	1-D · 2-C · 3-D · 4-A · 5-A · 6-A · 7-B · 8-A · 9-A · 10-A
12	1-B · 2-B · 3-C · 4-A · 5-B · 6-C · 7-A · 8-D · 9-B · 10-D
13	1-D · 2-C · 3-A · 4-A · 5-A · 6-C · 7-C · 8-A · 9-A · 10-A
14	1-B · 2-C · 3-B · 4-A · 5-D · 6-B · 7-D · 8-A · 9-B · 10-C
15	1-A · 2-C · 3-A · 4-B · 5-C · 6-C · 7-A · 8-B · 9-C · 10-B
16	1-B · 2-A · 3-A · 4-B · 5-A · 6-C · 7-A · 8-B · 9-D · 10-D

Chapter	Answers
17	1-C · 2-C · 3-C · 4-B · 5-A · 6-C · 7-C · 8-D · 9-C · 10-A
18	1-B · 2-D · 3-B · 4-A · 5-D · 6-B · 7-C · 8-C · 9-D · 10-D
19	1-D · 2-D · 3-B · 4-D · 5-C · 6-B · 7-D · 8-A · 9-D · 10-B
20	1-D · 2-D · 3-A · 4-A · 5-A · 6-A · 7-C · 8-D · 9-B · 10-B
21	1-D · 2-D · 3-A · 4-A · 5-C · 6-C · 7-C · 8-D · 9-B · 10-C
22	1-B · 2-D · 3-D · 4-C · 5-C · 6-C · 7-C · 8-C · 9-B · 10-B
23	1-D · 2-B · 3-A · 4-C · 5-A · 6-C · 7-B · 8-A · 9-A · 10-A
24	1-C · 2-B · 3-D · 4-C · 5-C · 6-C · 7-C · 8-D · 9-D · 10-C
25	1-C · 2-B · 3-B · 4-A · 5-D · 6-D · 7-D · 8-D · 9-C · 10-D
26	1-A · 2-A · 3-A · 4-C · 5-D · 6-D · 7-A · 8-A · 9-C · 10-D
27	1-B · 2-D · 3-D · 4-D · 5-D · 6-B · 7-C · 8-B · 9-B · 10-C
28	1-C · 2-C · 3-B · 4-C · 5-C · 6-C · 7-A · 8-A · 9-A · 10-D
29	1-A · 2-C · 3-B · 4-C · 5-C · 6-B · 7-B · 8-A · 9-C · 10-D
30	1-A · 2-B · 3-B · 4-D · 5-B · 6-D · 7-A · 8-C · 9-C · 10-A
31	1-B · 2-D · 3-B · 4-B · 5-A · 6-B · 7-B · 8-A · 9-C · 10-D
32	1-B · 2-A · 3-A · 4-C · 5-C · 6-D · 7-C · 8-D · 9-D · 10-B

Chapter	Answers
33	1-D · 2-C · 3-D · 4-B · 5-D · 6-D · 7-C · 8-D · 9-C · 10-A
34	1-B · 2-D · 3-C · 4-A · 5-C · 6-B · 7-D · 8-B · 9-D · 10-A
35	1-B · 2-A · 3-B · 4-C · 5-B · 6-A · 7-D · 8-C · 9-A · 10-C
36	1-C · 2-B · 3-D · 4-B · 5-D · 6-C · 7-C · 8-D · 9-C · 10-C
37	1-B · 2-C · 3-B · 4-C · 5-B · 6-D · 7-D · 8-D · 9-C · 10-A
38	1-A · 2-B · 3-C · 4-C · 5-D · 6-A · 7-D · 8-A · 9-B · 10-D
39	1-C · 2-D · 3-B · 4-D · 5-D · 6-C · 7-C · 8-A · 9-A · 10-A
40	1-C · 2-D · 3-A · 4-B · 5-A · 6-D · 7-D · 8-D · 9-C · 10-B
41	1-D · 2-B · 3-A · 4-A · 5-D · 6-A · 7-D · 8-A · 9-B · 10-C
42	1-B · 2-C · 3-B · 4-A · 5-B · 6-B · 7-D · 8-D · 9-A · 10-D
43	1-B · 2-B · 3-C · 4-C · 5-A · 6-A · 7-B · 8-C · 9-B · 10-C
44	1-D · 2-C · 3-B · 4-D · 5-B · 6-A · 7-A · 8-C · 9-C · 10-C

Sources Index (by chapter)

Chapter 1: What a Writer Does at Ongo Weight Loss

1. CDC, Adult Obesity Facts ●
2. Ongo Weight Loss, homepage copy review, 30 October 2026 (internal) ●

Chapter 2: YMYL: Why Google Judges Health Pages Harder

1. Google, Search Quality Rater Guidelines (September 11, 2025) ●
2. Ongo Weight Loss, blog listing review, 30 October 2026 ●

Chapter 3: E-E-A-T and Helpful Content

1. Google Search Central, Creating helpful, reliable, people-first content ●

2. Google Search Central, Spam policies for Google web search
3. Found, Weight loss news blog (editorial standards), viewed 30 October 2026

Chapter 4: Compliance Rules and the Four Types of Claims

1. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025)
2. Mochi Health, homepage, viewed 30 October 2026
3. Ongo Weight Loss, Book One (standing rules) and homepage review, 30 October 2026 (internal)

Chapter 5: The Ongo Weight Loss Voice and Tone Guide

1. Hims, Weight loss, viewed 30 October 2026
2. Calibrate, homepage, viewed 30 October 2026
3. Ro, Weight loss, viewed 30 October 2026
4. Mochi Health, homepage, viewed 30 October 2026
5. IVIM Health, homepage, viewed 30 October 2026
6. Henry Meds, Weight management, viewed 30 October 2026
7. Ongo Weight Loss, voice and tone guide v1, October 2026 (internal)

Chapter 6: Who We Write For: Readers, Personas and Reading Level

1. JMIR, Assessing the readability of online patient education materials in obstetrics and gynecology (2023)
2. KFF, Poll: 1 in 8 adults say they are currently taking a GLP-1 drug (November 14, 2025)
3. CDC, Clear Communication Index
4. NIH, Clear & Simple

Chapter 7: People-First, Weight-Respectful Language

1. Obesity Action Coalition, People-First Language
2. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025)
3. Ongo Weight Loss, homepage copy review, 30 October 2026 (internal)

Chapter 8: Medical Basics Every Writer Must Know

1. Wilding et al., Once-Weekly Semaglutide in Adults with Overweight or Obesity (STEP 1), *New England Journal of Medicine* (2021)

2. Jastreboff et al., Tirzepatide Once Weekly for the Treatment of Obesity (SURMOUNT-1), *New England Journal of Medicine* (2022)
3. CDC, Adult BMI Categories
4. IVIM Health, homepage, viewed 30 October 2026

Chapter 9: The Writer's Mindset: Purpose First

1. Google Search Central, Creating helpful, reliable, people-first content
2. Ongo Weight Loss, writing guidelines and blog review, October 2026 (internal)

Chapter 10: Content Briefs and the 38 Content Types

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026)
2. Ongo Weight Loss, blog listing, viewed 30 October 2026
3. Ongo Weight Loss, content-type list (team guidelines), October 2026 (internal)

Chapter 11: Search Intent and the Funnel

1. Google, Search Quality Rater Guidelines (September 11, 2025)
2. Ongo Weight Loss, Book Three (content map) and Book Four (events), internal

Chapter 12: Keyword and Question Research for Writers

1. Ro, Health Guide, viewed 30 October 2026
2. Ongo Weight Loss, blog listing, viewed 30 October 2026
3. Ongo Weight Loss, writing guidelines (keyword table), internal

Chapter 13: Researching a Health Topic: The Evidence Ladder

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026)
2. CDC, Adult Obesity Facts
3. Wilding et al., STEP 1, *NEJM* (2021)

Chapter 14: How Competitors Write: Best Pages, Blogs and Why They Work

1. Found, Weight loss news and Zepbound vs Ozempic, viewed 30 October 2026
2. Ro, Weight loss and Semaglutide nausea guide, viewed 30 October 2026
3. Hims, Weight loss, viewed 30 October 2026
4. WeightWatchers, Blog, viewed 30 October 2026
5. Noom, Blog, viewed 30 October 2026
6. Mochi Health, homepage, viewed 30 October 2026

7. Fridays, homepage, viewed 30 October 2026
8. Calibrate, homepage, viewed 30 October 2026
9. IVIM Health, homepage, viewed 30 October 2026
10. Henry Meds, Weight management, viewed 30 October 2026
11. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026

Chapter 15: Building the Outline and Writing Headings

1. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026
2. Ongo Weight Loss, Book Four, Chapter 7 (headings), internal

Chapter 16: Fact Sheets and Claim Logs

1. Found, Zepbound vs Ozempic, viewed 30 October 2026
2. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (VERIFY), internal

Chapter 17: Using AI Safely: Consultant, Not Co-Writer

1. Google Search Central Blog, Google Search's guidance about AI-generated content (February 2023)
2. Google Search Central, Creating helpful, reliable, people-first content
3. Ongo Weight Loss, AI use rules (writing guidelines), internal

Chapter 18: Plain Language: The Foundation Technique

1. digital.gov, Plain language guides
2. NIH, Clear & Simple
3. CDC, Clear Communication Index
4. Ongo Weight Loss, homepage review, 30 October 2026 (internal)

Chapter 19: Readability, Rhythm and Sounding Human

1. NIH, Clear & Simple
2. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (rhythm), internal

Chapter 20: The Inverted Pyramid and BLUF: Answer First

1. Ro, Semaglutide nausea: causes, duration, and tips for relief (updated August 24, 2026)
2. NIH, Clear & Simple (most important points first)

3. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Chapter 21: Hypophora: Ask the Reader's Question, Then Answer It

1. Ongo Weight Loss, homepage review, 30 October 2026, and writing guidelines (hypophora), internal 🔵

Chapter 22: Breaking the Fourth Wall and the Conversational "You"

1. Ongo Weight Loss, homepage review, 30 October 2026, and voice guide (Chapter 5), internal 🔵

Chapter 23: Clinical Neutrality and Hedging

1. Mayo Clinic, Semaglutide (subcutaneous route), viewed 30 October 2026 🟢
2. Wilding et al., STEP 1, *NEJM* (2021) 🟢
3. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Chapter 24: Persuasion Models, Used Ethically

1. Ro, Weight loss, viewed 30 October 2026 🟠
2. Fridays, homepage, viewed 30 October 2026 🟠
3. KFF, GLP-1 poll (November 14, 2025) 🟢
4. FTC, Disclosures 101 for Social Media Influencers 🟢

Chapter 25: Explanation Models: Making Hard Things Easy

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Chapter 26: Information Gain: Be 1% More Useful

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ro, Semaglutide nausea guide, viewed 30 October 2026 🟠
3. Ongo Weight Loss, writing guidelines (information gain), internal 🔵

Chapter 27: Storytelling and Patient Stories

1. DWT, FTC finalizes rule banning fake consumer reviews (August 2024) 🟠
2. FTC, Disclosures 101 for Social Media Influencers 🟢
3. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Chapter 28: Headlines and Titles That Are Honest and Still Get Clicks

1. Noom, Blog, viewed 30 October 2026
2. Ongo Weight Loss, blog listing and homepage, viewed 30 October 2026
3. Henry Meds, Weight management, viewed 30 October 2026

Chapter 29: Introductions, Key Points and Conclusions

1. Ro, Semaglutide nausea guide (updated August 24, 2026)
2. CDC, Adult Obesity Facts
3. Ongo Weight Loss, writing guidelines and homepage review, October 2026 (internal)

Chapter 30: Scannability: Headings, Lists, Tables and Bold

1. NIH, Clear & Simple
2. Ongo Weight Loss, blog listing, viewed 30 October 2026

Chapter 31: Writing for AI Answers, Snippets and FAQs

1. Google Search Central, Creating helpful, reliable, people-first content
2. Ongo Weight Loss, blog listing, viewed 30 October 2026, and Book Four (schema), internal

Chapter 32: CTAs and Microcopy: Buttons, Forms and Errors

1. Ro, Weight loss, viewed 30 October 2026
2. Hims, Weight loss, viewed 30 October 2026
3. Mochi Health, homepage, viewed 30 October 2026
4. Fridays, homepage, viewed 30 October 2026
5. Ongo Weight Loss, homepage review and Book Four event dictionary, internal

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1. Mochi Health, homepage, viewed 30 October 2026
2. Ongo Weight Loss, homepage review, 30 October 2026, and Books Three–Four (page templates), internal

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1. WeightWatchers, Blog, viewed 30 October 2026
2. Noom, Blog, viewed 30 October 2026
3. Ongo Weight Loss, blog listing, viewed 30 October 2026

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1. Google, Search Quality Rater Guidelines (September 11, 2025) 🟢
2. Google Search Central, Creating helpful, reliable, people-first content 🟢
3. Ongo Weight Loss, homepage review, 30 October 2026, and Book One (Medical Director), internal 🔵

Chapter 36: Short Formats: Meta, Alt Text, Emails, Captions and Video Scripts

1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, Book Four (meta, alt text) and Book Five (captions), internal 🔵

Chapter 37: Self-Editing: Three Passes and the Checklist Tests

1. NIH, Clear & Simple 🟢
2. Ongo Weight Loss, writing guidelines (checklist tests) and homepage review, October 2026 (internal) 🔵

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1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, blog listing, viewed 30 October 2026 🔵

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1. Ro, Semaglutide nausea guide, viewed 30 October 2026 🟠
2. Found, Zepbound vs Ozempic, viewed 30 October 2026 🟠
3. Ongo Weight Loss, review workflow (Book Five, Chapter 38), internal 🔵

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1. Google Search Central, Spam policies 🟢
2. Ongo Weight Loss, Book Three (content map) and Book Four (on-page SEO, events), internal 🔵

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1. CDC, Clear Communication Index 🟢
2. Ongo Weight Loss, homepage review, 30 October 2026, and Book Four (accessibility), internal 🔵

Chapter 42: The Writer's Toolkit: Free, Freemium and Paid

1. NIH, Clear & Simple 🟢

2. Ongo Weight Loss, writing guidelines (tools) and team workflow, October 2026 (internal) 🔵

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1. Google Search Central, Creating helpful, reliable, people-first content 🟢
2. Ongo Weight Loss, Book Four (GA4 events) and Book Three (content map), internal 🔵

Chapter 44: The ZETA Model for Writers and the Final Gate

1. Google, Search Quality Rater Guidelines (September 11, 2025) 🟢
2. Holland & Knight, FDA and HHS action against telehealth compounded drug advertising (September 29, 2025) 🟠
3. DWT, FTC finalizes rule banning fake consumer reviews (August 2024) 🟠
4. Ongo Weight Loss, homepage review, 30 October 2026 (internal) 🔵

Clear, honest, kind.

Answer first. Say only what you can prove. Name the limits. Remove the shame. Show the next step. Then check it once more, because someone worried is going to read it.

Edition: October 2026. Approvals, rules and prices change; every source is dated so you can recheck it.

Prepared by **Ashish Osten**, Team Lead, Ongo Weight Loss.

Ongo Weight Loss